Answer Key & Rationales · Form 4

Quick key. **P** = pretest (unscored; do not count). Bank 2026.10.23-0423.

#	Key	Area	Sub-area	P
1	**B**	II	II.C	
2	**D**	III	III.A	
3	**B**	II	II.B	
4	**B**	II	II.B	
5	**D**	I	I.C	
6	**A**	III	III.A	
7	**A**	I	I.B	
8	**A**	I	I.B	
9	**C**	III	III.C	
10	**C**	I	I.A	
11	**A**	I	I.C	
12	**A**	I	I.B	
13	**C**	II	II.B	
14	**A**	III	III.B	
15	**B**	III	III.B	
16	**C**	II	II.B	
17	**B**	I	I.A	
18	**A**	I	I.A	
19	**B**	I	I.C	
20	**B**	II	II.B	**P**
21	**B**	I	I.A	
22	**C**	I	I.B	
23	**B**	I	I.A	
24	**B**	I	I.B	
25	**A**	III	III.B	
26	**A**	I	I.B	
27	**C**	II	II.B	
28	**B**	II	II.A	
29	**C**	III	III.B	
30	**C**	III	III.B	
31	**D**	I	I.A	**P**
32	**A**	III	III.C	
33	**B**	II	II.B	
34	**C**	III	III.B	
35	**A**	I	I.B	
36	**D**	III	III.B	
37	**C**	II	II.A	
38	**A**	III	III.B	
39	**C**	III	III.A	
40	**A**	III	III.B	
41	**A**	II	II.A	
42	**B**	III	III.B	**P**
43	**C**	III	III.B	**P**
44	**C**	II	II.C	
45	**A**	II	II.A	
46	**A**	I	I.B	
47	**C**	I	I.C	
48	**A**	III	III.D	
49	**A**	II	II.A	
50	**C**	III	III.D	
51	**D**	II	II.A	
52	**A**	I	I.C	
53	**C**	I	I.B	
54	**A**	II	II.A	
55	**B**	III	III.B	**P**
56	**B**	I	I.B	**P**
57	**B**	II	II.A	
58	**D**	II	II.A	
59	**D**	I	I.A	
60	**C**	II	II.C	
61	**A**	III	III.B	
62	**B**	III	III.B	
63	**A**	I	I.B	
64	**B**	I	I.C	
65	**B**	III	III.B	
66	**C**	II	II.B	
67	**A**	III	III.A	
68	**D**	II	II.B	
69	**C**	I	I.B	
70	**C**	III	III.B	
71	**C**	II	II.A	
72	**A**	I	I.A	
73	**C**	II	II.B	
74	**C**	III	III.A	
75	**D**	II	II.A	
76	**D**	II	II.A	
77	**C**	II	II.B	
78	**A**	I	I.C	
79	**A**	III	III.A	
80	**C**	III	III.C	
81	**B**	I	I.B	
82	**B**	III	III.D	
83	**A**	II	II.A	
84	**A**	I	I.B	**P**
85	**B**	III	III.B	
86	**B**	I	I.A	
87	**C**	II	II.C	**P**
88	**A**	II	II.A	
89	**A**	II	II.B	
90	**B**	III	III.B	
91	**C**	III	III.B	**P**
92	**D**	II	II.A	**P**
93	**B**	I	I.B	
94	**C**	II	II.A	
95	**B**	I	I.A	
96	**A**	I	I.A	
97	**B**	I	I.B	**P**
98	**B**	II	II.C	
99	**A**	I	I.B	
100	**C**	III	III.B	
101	**A**	I	I.A	
102	**C**	I	I.B	
103	**C**	III	III.B	
104	**B**	II	II.A	
105	**C**	I	I.C	
106	**B**	III	III.C	
107	**C**	I	I.A	
108	**C**	III	III.B	
109	**A**	II	II.B	
110	**A**	I	I.B	
111	**A**	III	III.B	
112	**C**	III	III.B	
113	**C**	III	III.D	
114	**B**	II	II.A	**P**
115	**C**	III	III.C	
116	**B**	II	II.C	
117	**A**	I	I.B	
118	**C**	I	I.C	
119	**C**	I	I.C	
120	**D**	II	II.A	
121	**A**	I	I.C	
122	**D**	II	II.A	

Scoring worksheet

Count correct answers on **scored items only**. Item numbers for each sub-area are listed; pretest items are left out.

Area / sub-area	Scored item numbers	Items	Correct
I. Values and Ethics		**40**	**___ / 40**
I.A Ethical Principles and Responsibilities	10, 17, 18, 21, 23, 59, 72, 86, 95, 96, 101, 107	12	____
I.B Ethical Service Delivery	7, 8, 12, 22, 24, 26, 35, 46, 53, 63, 69, 81, 93, 99, 102, 110, 117	17	____
I.C Diversity and Social Justice	5, 11, 19, 47, 52, 64, 78, 105, 118, 119, 121	11	____
II. Assessment and Planning		**35**	**___ / 35**
II.A Assessment Concepts	28, 37, 41, 45, 49, 51, 54, 57, 58, 71, 75, 76, 83, 88, 94, 104, 120, 122	18	____
II.B Assessment Methods and Techniques	3, 4, 13, 16, 27, 33, 66, 68, 73, 77, 89, 109	12	____
II.C Assessment Practices	1, 44, 60, 98, 116	5	____
III. Intervention and Practice		**35**	**___ / 35**
III.A Practice Concepts	2, 6, 39, 67, 74, 79	6	____
III.B Intervention Methods and Techniques	14, 15, 25, 29, 30, 34, 36, 38, 40, 61, 62, 65, 70, 85, 90, 100, 103, 108, 111, 112	20	____
III.C Practice Evaluation and Research	9, 32, 80, 106, 115	5	____
III.D Supervision and Administration	48, 50, 82, 113	4	____

Pretest items (skip when scoring): 20, 31, 42, 43, 55, 56, 84, 87, 91, 92, 97, 114

Total correct (scored items)	_____ / 110
Practice pass line	77 / 110 (70%)
Correct needed to reach the line	77 − your total = _____

The real exam sets its passing score separately for each form (it is reported as a scaled score). 77 of 110 is a deliberately cautious practice target that builds a margin; it is not a prediction of your real result.

Score conversion (raw out of 110 → percent)

The boxed row is the practice pass line: **77 / 110 = 70%**. Any score at or above it meets the line.

Raw	%	Raw	%	Raw	%	Raw	%	Raw	%	Raw	%
110	100%	91	83%	72	65%	53	48%	34	31%	15	14%
109	99%	90	82%	71	65%	52	47%	33	30%	14	13%
108	98%	89	81%	70	64%	51	46%	32	29%	13	12%
107	97%	88	80%	69	63%	50	45%	31	28%	12	11%
106	96%	87	79%	68	62%	49	45%	30	27%	11	10%
105	95%	86	78%	67	61%	48	44%	29	26%	10	9%
104	95%	85	77%	66	60%	47	43%	28	25%	9	8%
103	94%	84	76%	65	59%	46	42%	27	25%	8	7%
102	93%	83	75%	64	58%	45	41%	26	24%	7	6%
101	92%	82	75%	63	57%	44	40%	25	23%	6	5%
100	91%	81	74%	62	56%	43	39%	24	22%	5	5%
99	90%	80	73%	61	55%	42	38%	23	21%	4	4%
98	89%	79	72%	60	55%	41	37%	22	20%	3	3%
97	88%	78	71%	59	54%	40	36%	21	19%	2	2%
96	87%	**77**	**70%**	58	53%	39	35%	20	18%	1	1%
95	86%	76	69%	57	52%	38	35%	19	17%	0	0%
94	85%	75	68%	56	51%	37	34%	18	16%		
93	85%	74	67%	55	50%	36	33%	17	15%		
92	84%	73	66%	54	49%	35	32%	16	15%		

Per-area practice guide: aim for at least 70% in each area as well as overall (Area I 28 of 40; Areas II and III 25 of 35 each).

Rationales for every item

Section 1

1. ASM-1076 (rev 1) · Best answer: B

An 81-year-old client with mild memory problems lives with an adult child. During a home visit, the adult child reports that over the past 4 days the client has become confused at night, has trouble passing urine and complains of a dry mouth. Last week the client began taking an over-the-counter nighttime pain reliever that contains diphenhydramine. What is the **MOST** likely cause of the change?

A. Progression of a neurocognitive disorder
B. Anticholinergic effects of the new product
C. Onset of a late-life major depressive episode
D. Normal age-related cognitive decline

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Neurocognitive disorders usually progress gradually over months. A change over 4 days, with urinary and mouth symptoms that began after a new product, points to a reversible drug effect that needs medical review.

B **Correct.** Diphenhydramine, common in over-the-counter sleep and nighttime pain products, is strongly anticholinergic. In older adults it can cause confusion or delirium, urinary retention and dry mouth, and the AGS Beers Criteria advise avoiding it in this age group (American Geriatrics Society, 2023). The timing and the bodily signs fit. It beats A because neurocognitive disorder progresses slowly and would not explain urinary retention; an abrupt change in attention and awareness suggests delirium, which DSM-5-TR lists as a possible medication effect.

C **Incorrect** (trap: Near-miss diagnosis). Depression in later life can impair concentration and memory. It does not explain trouble passing urine or a dry mouth, and these changes started within days of a new medicine.

D **Incorrect** (trap: Near-miss diagnosis). Normal aging brings mild, slow changes in memory and processing speed. It does not produce nighttime confusion over a few days or urinary retention.

Takeaway: Sudden confusion in an older adult who recently started an anticholinergic product such as diphenhydramine suggests a drug effect needing medical review.

Sources: American Geriatrics Society Beers Criteria Update Expert Panel (2023). American Geriatrics Society 2023 updated AGS Beers Criteria for potentially inappropriate medication use in older adults. Journal of the American Geriatrics Society, 71(7), 2052-2081, First-generation antihistamines (diphenhydramine); DSM-5-TR (APA, 2022), Neurocognitive Disorders: Delirium (medication-induced)
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.5 Prescription, OTC, alternative medications: uses and side effects · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

2. INT-1503 (rev 1) · Best answer: D

A social worker in a court-ordered treatment program feels intense disgust toward a client convicted of harming a child. For 6 weeks, the worker has discussed the reaction in weekly supervision and in personal therapy. The disgust persists, the worker dreads each session, and the client says, 'You barely look at me anymore.' What is the **MOST** appropriate next step?

A. Keep meeting while continuing to process the reaction in supervision
B. Tell the client about the reaction so the work feels honest
C. Ask the client to describe the change they have noticed
D. Arrange a planned transfer to another qualified social worker

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Processing countertransference in supervision is the correct first response, and it has been tried. After 6 weeks without change, and with the client feeling the withdrawal, staying on keeps the client in a relationship that is not working.

B **Incorrect** (trap: Boundary slip or collusion). Disclosing disgust would burden and shame the client and serve the worker's need to unload. Workers usually do not share countertransference feelings with clients; they work through them in supervision or therapy.

C **Incorrect** (trap: Good but not best). Inviting the client's view is a good repair move when the cause of a strain is unclear. Here the cause is known, the worker's persisting reaction, and exploring it with the client cannot restore the worker's capacity to stay engaged.

D **Correct.** When a worker's reaction keeps impairing the relationship after supervision and personal work, a planned transfer protects the client. The Code directs referral when a social worker is not being effective and other services are needed (NASW Code 1.16(a)). The client already senses the withdrawal. It beats A because supervision has been tried for 6 weeks without change, so more of the same prolongs ineffective care. Supervision remains the right first step in most cases (Hepworth et al.).

Takeaway: Take countertransference to supervision first; if it still impairs the work, arrange a planned transfer rather than continue ineffective care.

Sources: NASW Code of Ethics (2021), 1.16(a); Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Relational reactions: countertransference
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.2 Building and sustaining the helping relationship (empathy, rapport, acceptance) · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

3. ASM-1139 (rev 1) · Best answer: B

At a pediatric clinic, a parent asks the social worker to start an ADHD diagnosis for their 7-year-old. For 8 months, the child has struggled to finish chores, loses things and interrupts siblings. So far, no information about the child's behavior at school has been gathered. What should the social worker do **FIRST**?

A. Refer the child to the pediatrician to discuss medication
B. Obtain rating scales from the parent and the teacher
C. Start behavioral parent training for the problems at home

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). A medication discussion follows an established diagnosis. Referring now skips the multi-setting assessment that ADHD requires and may lead to treating a problem that occurs only at home.

B **Correct.** ADHD requires several symptoms in two or more settings, such as home and school, so information from more than one setting is needed before a diagnosis. The AAP guideline directs clinicians to gather reports from parents and from teachers or other school staff (Wolraich et al., 2019). All information so far comes from home. It beats C because parent training may become part of the plan, but the assessment must first show whether the problem is ADHD, another condition or something limited to home (DSM-5-TR).

C **Incorrect** (trap: Right action, wrong time). Behavioral parent training is an evidence-based treatment for ADHD symptoms and disruptive behavior. Starting it before the assessment is complete puts treatment ahead of knowing what is being treated.

Takeaway: ADHD diagnosis requires symptoms in at least two settings; gather parent and teacher reports before diagnosing or treating.

Sources: DSM-5-TR (APA, 2022), Neurodevelopmental Disorders: Attention-Deficit/Hyperactivity Disorder; Wolraich, M. L., et al. (2019). Clinical practice guideline for the diagnosis, evaluation, and treatment of attention-deficit/hyperactivity disorder in children and adolescents. Pediatrics, 144(4), e20192528, Key Action Statement 2
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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4. ASM-1588 (rev 3) · Best answer: B

A client referred by drug court after a third arrest for driving under the influence arrives for an outpatient intake. The client sits with arms crossed and says, 'I'm only here because the judge made me. My drinking is nobody's business.' What should the social worker do **FIRST**?

A. Ask the client to rate readiness to change on a 0-10 scale
B. Acknowledge the client's anger about being sent to treatment
C. Point out that three arrests suggest drinking is a problem

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A readiness ruler is a good way to gauge motivation, and the social worker may use one later in this intake. Asked of an angry client in the first minute, it is likely to feel like a test and draw a defensive '0' rather than useful information.

B **Correct.** Anger at being sent to treatment is common among mandated clients. Meeting it with empathy rather than argument lowers defensiveness and opens the way to honest talk about drinking, and the client's response starts to show where they stand, likely precontemplation here (SAMHSA TIP 35; Prochaska, DiClemente & Norcross, 1992). Acknowledging the frustration does not mean agreeing that drinking is not a problem. It beats A because a readiness ruler works once the client is engaged enough to answer it honestly; asked now, it yields little.

C **Incorrect** (trap: Too directive). The arrest history matters, but confronting a resentful client with it at intake invites argument and more defense of drinking. Motivational approaches avoid labeling and arguing for change, especially before engagement.

Takeaway: With an angry mandated client, acknowledge the frustration first; readiness tools and talk about consequences work better once the client is engaged.

Sources: Prochaska, J. O., DiClemente, C. C., & Norcross, J. C. (1992). In search of how people change: Applications to addictive behaviors. American Psychologist, 47(9), 1102-1114, whole article; SAMHSA TIP 35 (2019), Ch. 4: From Precontemplation to Contemplation: Building Readiness

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.4 Assessing motivation, barriers, and readiness for change · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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5. ETH-1637 (rev 2) · Best answer: D

A city plans a new diesel bus depot next to a low-income neighborhood. The only public hearing is held at 10 a.m. on a weekday, in English, with notice posted only on the city website. Residents say they learned of the plan after the vote. Which aspect of environmental justice does the hearing arrangement fail?

A. Distributive justice
B. Restorative justice
C. Intergenerational justice
D. Procedural justice

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Distributive justice concerns how environmental burdens and benefits are shared. Placing a diesel depot beside a low-income neighborhood may raise that concern too, but the hearing's timing, language and notice are about who could take part.

B **Incorrect** (trap: Wrong model or concept). Restorative justice repairs harm after it occurs by bringing affected people together around needs and obligations. Nothing here concerns repairing a past harm; the issue is exclusion from a decision.

C **Incorrect** (trap: Wrong model or concept). Intergenerational justice concerns duties to future generations, such as keeping air and land healthy for people not yet born. The hearing problem is about which of today's residents could take part in the decision, not about obligations to the future.

D **Correct.** Environmental justice has a distributive side, who bears environmental burdens, and a participatory side, who has a meaningful voice in decisions (Schlosberg, 2007). The 1991 Principles of Environmental Justice call for affected communities to take part as equal partners at every stage of decision-making (Principle 7). A weekday-morning, English-only hearing announced only online shut residents out, which is a failure of procedural justice. It beats A, the strongest competitor, because the depot's location may also be unfair, but the question asks about the hearing, which concerns participation rather than the share of burden.

Takeaway: Environmental justice includes fair sharing of burdens and meaningful participation; inaccessible hearings violate the participation, or procedural, side.

Sources: Schlosberg, D. (2007). Defining Environmental Justice: Theories, Movements, and Nature. Oxford University Press, distribution, recognition and participation; Principles of Environmental Justice (1991). First National People of Color Environmental Leadership Summit, Principle 7

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.2 Principles of social, racial, environmental, economic justice · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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6. INT-1566 (rev 1) · Best answer: A

A school social worker is writing a foundation grant proposal for an anxiety skills group for middle school students. The funder's guidelines ask for a statement of need. Which content would **BEST** support this section?

A. District data on anxiety rates and current waitlists for services
B. National statistics showing rising anxiety among adolescents
C. A history of the agency's 30 years serving local youth
D. A session-by-session outline of the planned group curriculum

Why each option is right or wrong

A **Correct.** A need statement documents the problem in the specific population and place the program will serve, using credible data that show the gap the program will fill (Coley, Scheinberg & Levites Strekalova, 2021). District anxiety data and waitlists show both the local need and the shortage of current help. It beats B because national figures show a broad trend but do not prove that students in this district need this group.

B **Incorrect** (trap: Good but not best). National data can add context in a sentence or two, but funders want evidence that the problem exists locally and is unmet. Relying on national trends leaves the local need unproven.

C **Incorrect** (trap: Wrong model or concept). Agency history shows capacity and credibility, which belongs in the organizational background section. It does not describe the problem the program will address.

D **Incorrect** (trap: Wrong model or concept). A curriculum outline describes how the program will work, which belongs in the program description or methods section, not in the statement of need.

Takeaway: A grant's need statement uses local, credible data to show the problem and service gap in the population the program will serve.

Sources: Coley, S. M., Scheinberg, C. A., & Levites Strekalova, Y. A. (2021). Proposal Writing: Effective Grantsmanship for Funding (6th ed.). SAGE, Ch. 8: Writing the Need or Problem Statement
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.8 Writing formal documents: grants, proposals, reports, evaluations · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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7. ETH-1540 (rev 2) · Best answer: A

Which term describes overriding a competent client's choice, or withholding information from the client, on the grounds that doing so is for the client's own good?

A. Paternalism
B. Beneficence
C. Nonmaleficence

Why each option is right or wrong

A **Correct.** Paternalism is interfering with a person's choices or information, against their wishes, for their own benefit (Reamer, 2018). It is the central tension with self-determination, which the Code allows a social worker to limit when a client's actions pose a serious, foreseeable and imminent risk (NASW Code 1.02). It beats B because beneficence is the general duty to act for the client's benefit; paternalism is one way of pursuing benefit, by overriding the client's choice (Beauchamp & Childress, 2019).

B **Incorrect** (trap: Wrong model or concept). Beneficence is the general duty to promote the client's welfare. It motivates paternalism but does not describe overriding a competent person's choice; most beneficent acts respect the client's decisions.

C **Incorrect** (trap: Wrong model or concept). Nonmaleficence is the duty to avoid causing harm. It concerns what the social worker does to the client, not overriding the client's own decisions for the client's benefit.

Takeaway: Paternalism means overriding a competent client's choice, or withholding information, for the client's own good; it is the classic threat to self-determination.

Sources: Reamer, F. G. (2018). Social Work Values and Ethics (5th ed.). Columbia University Press, client self-determination and paternalism; NASW Code of Ethics (2021), 1.02; Beauchamp, T. L., & Childress, J. F. (2019). Principles of Biomedical Ethics (8th ed.). Oxford University Press, Ch. 6: paternalism
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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8. ETH-0006 (rev 1) · Best answer: A

A 9-year-old client in a school-based program tells the social worker that a parent hit them with a belt last night. The client shows the social worker red marks on the back of their legs. What should the social worker do **FIRST**?

A. Make a report to child protective services
B. Call the parent to ask about the marks
C. Interview the child in detail about other incidents

Why each option is right or wrong

A **Correct.** A child's disclosure of being hit with an object, with visible marks, gives reasonable suspicion of physical abuse. That triggers the legal duty to report promptly to child protective services. Reporters need suspicion, not proof. It beats C because investigating belongs to child protective services and law enforcement, and detailed questioning can distress the child and taint a later forensic interview. Every state must have mandatory reporting provisions (CAPTA), and reporting is a recognized limit of confidentiality (NASW Code 1.07(c)). CA aside: California requires a phone report immediately or as soon as practicable and a written report within 36 hours (Penal Code 11166).

B **Incorrect** (trap: Ignores risk). Contacting the alleged abuser first can put the child at greater risk and lead to pressure on the child to take back the disclosure. Getting the parent's account is part of the protective services investigation, not the reporter's task.

C **Incorrect** (trap: Outside the social work role). A detailed interview goes beyond what reporting requires and can contaminate a later forensic interview. The social worker gathers only enough information to make the report, then lets trained investigators take over.

Takeaway: Reasonable suspicion of child abuse triggers a prompt report. The social worker reports; investigators investigate.

Sources: Child Abuse Prevention and Treatment Act, 42 U.S.C. 5106a(b)(2)(B)(i); NASW Code of Ethics (2021), 1.07(c)

Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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9. INT-1588 (rev 2) · Best answer: C

An after-school program serves 40 middle school students in a city of 300,000. Its activities are tutoring, mentoring and family outreach. The new board chair wants the program's main objective to be a 50% drop in citywide juvenile arrests within 1 year. What is the **MOST** appropriate response from the program's social worker?

A. Keep the arrest objective but extend the deadline to 5 years

B. Report students served and hours of tutoring and mentoring

C. Propose student outcomes that its tutoring and mentoring can change

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). A longer timeline sounds more realistic, but 40 students cannot measurably change arrests across a city of 300,000, and any change could not be traced to the program. Citywide arrests fit as a long-term aim, not as the program's main objective.

B **Incorrect** (trap: Wrong model or concept). Counts of students and hours of tutoring and mentoring are outputs. They document activity and belong in reports, but they do not show whether students changed, so they cannot replace outcome objectives.

C **Correct.** Objectives should be achievable and logically linked to program activities. A logic model connects activities to short- and intermediate-term outcomes the program can influence, such as grades, school attendance or family engagement, with broader community change as a long-term aim (W.K. Kellogg Foundation, 2004; Kettner,

Moroney & Martin, 2022). A 40-student program cannot plausibly halve citywide arrests in a year. This beats A because more time does not make citywide arrests something this program can change or be credited with, so results would mislead.

Takeaway: Set objectives the program can plausibly achieve and attribute to its activities; keep broad community change as a long-term goal.

Sources: W.K. Kellogg Foundation (2004). Logic Model Development Guide, Ch. 1: outcomes and impact; Kettner, P. M., Moroney, R. M., & Martin, L. L. (2022). Designing and Managing Programs: An Effectiveness-Based Approach (6th ed.). SAGE, Ch. 7: Organizational Mission and Program Goals, Objectives, and Activities
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.2 Setting program objectives and outcomes · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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10. ETH-1131 (rev 1) · Best answer: C

Under HIPAA, what type of permitted disclosure is it when a practice sends a former client's outstanding bill to a collection agency?

A. Health care operations
B. Treatment
C. Payment
D. Required by law

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Health care operations covers activities such as quality review, auditing, training and business management. Collection activities are named in the payment definition, so this disclosure is classified as payment.

B **Incorrect** (trap: Wrong model or concept). Treatment means providing, coordinating or managing care among providers. Sending a balance to a collection agency involves no care, so it is not a treatment disclosure.

C **Correct.** HIPAA's definition of payment includes billing and collection activities (45 CFR 164.501), and a covered entity may disclose information for its own payment without authorization (164.506(c)(1)). The minimum necessary rule still applies, so only identifying, contact and balance details go to the agency, never clinical content (164.502(b); NASW Code 1.07(c)). C beats A because, although health care operations includes general business management, HIPAA names collection activities in the payment definition.

D **Incorrect** (trap: Wrong model or concept). Required-by-law disclosures are those a statute or court compels, such as certain abuse reports. No law requires sending a debt to collections; the practice chooses to do it under the payment provision.

Takeaway: HIPAA classes collection activity as payment; disclose only identifying, contact and balance details, never clinical information.

Sources: HIPAA Privacy Rule, 45 CFR 164.501 (definition: payment); HIPAA Privacy Rule, 45 CFR 164.506(c)(1); HIPAA Privacy Rule, 45 CFR 164.502(b); NASW Code of Ethics (2021), 1.07(c)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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11. ETH-1714 (rev 2) · Best answer: A

In a youth diversion conference, a 16-year-old who vandalized a community garden hears from gardeners about the damage. Elders and the youth's coach say the act was wrong and hurtful, then describe the youth's strengths and invite the youth to help replant. The gardeners then thank the youth and ask them to join the next community workday. Which concept **MOST** likely describes this approach?

A. Reintegrative shaming
B. Stigmatizing shaming
C. Labeling
D. Deterrence

Why each option is right or wrong

A **Correct.** Braithwaite (1989) described reintegrative shaming as disapproval of the act from people the offender respects, followed by gestures of forgiveness and reacceptance. Here elders and the coach name the harm, affirm the youth's worth, offer a way to repair it and close by inviting the youth into the group's work, which separates the act from the person. It beats B because stigmatizing shaming condemns the person as well as the act and offers no path back, which tends to push young people toward others who reject the rules.

B **Incorrect** (trap: Wrong model or concept). Stigmatizing shaming marks the person as bad and leaves them outcast, and Braithwaite linked it to more offending. This conference condemns the act while affirming the youth and ends with an invitation to join in, the opposite pattern.

C **Incorrect** (trap: Wrong model or concept). Labeling theory explains how official labels such as 'delinquent' can push a young person toward a deviant identity. The conference avoids attaching a lasting label, so labeling describes a risk the process is designed to prevent.

D **Incorrect** (trap: Wrong model or concept). Deterrence aims to prevent offending through the threat of punishment. This conference relies on relationships, repair and acceptance rather than fear of penalties.

Takeaway: Reintegrative shaming disapproves of the harmful act while affirming the person and offering reacceptance; stigmatizing shaming rejects the person.

Sources: Braithwaite, J. (1989). Crime, Shame and Reintegration. Cambridge University Press, reintegrative versus stigmatizing shaming
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.8 Restorative practice, truth and reconciliation with historically oppressed groups · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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12. ETH-1058 (rev 2) · Best answer: A

A client in an intensive outpatient program for alcohol use sends a friend request to the social worker's personal social media account. The program gave each client written social media rules at intake. What is the **MOST** appropriate response?

A. Leave it unaccepted and review the policy at the next session
B. Send the client a private message explaining the policy
C. Delete the request without mentioning it to the client

Why each option is right or wrong

A **Correct.** Social workers should not accept friend requests from clients on personal social media (NASW Code 1.06(h)), and a social media policy shared at intake is reviewed with clients as needed (Tech Standards 2.10). Raising it at the next session treats the request as clinical material and keeps the relationship clear. A beats C because silently deleting the request avoids the topic; the client may feel rejected without knowing why, and the boundary is never explained.

B **Incorrect** (trap: Boundary slip or collusion). Explaining the policy promptly is the right idea, which makes this tempting. Doing it by message from the personal account opens the very channel the policy closes and starts personal electronic contact (NASW Code 1.06(e)); the conversation belongs in session.

C **Incorrect** (trap: Good but not best). Not accepting is right, but deleting the request in silence misses the chance to explain the boundary. The client may read it as rejection, which can harm engagement.

Takeaway: Decline client friend requests on personal accounts and discuss the social media policy openly in session.

Sources: NASW Code of Ethics (2021), 1.06(h); NASW Code of Ethics (2021), 1.06(e); NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.10
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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13. ASM-1003 (rev 3) · Best answer: C

At a college counseling center, a 20-year-old client seeks help for 6 weeks of depression. The client mentions that last spring they barely slept for 5 days, felt unstoppable and started several ventures, and roommates said they seemed like a different person. The campus prescriber plans to start an antidepressant at an appointment this afternoon. What should the social worker do **FIRST**?

A. Explore how the energetic period affected the client's friendships
B. Advise the client to delay the prescriber's antidepressant plan
C. Tell the prescriber about the earlier high-energy period

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Learning more about the episode's effects is good assessment and should happen. It is not the first step when a medication decision is hours away and the prescriber does not yet know about a possible hypomanic episode.

B **Incorrect** (trap: Outside the social work role). Advising a client whether to take a medication is outside the social work role. Medication decisions belong to the prescriber, who needs the new history to make them well.

C **Correct.** A bipolar-spectrum illness often first appears as depression, so past hypomania must be identified before an antidepressant is started; treatment with an antidepressant alone can trigger a switch into mania (DSM-5-TR). Five days of little sleep, high energy and grandiosity, noticed by others, suggests a hypomanic episode. The prescriber makes the medication decision, so the social worker shares the finding before this afternoon's visit, as the Code expects of members of an interdisciplinary

team (NASW Code 2.03(a)). It beats A because exploring the episode further is useful but can follow; the time-sensitive step is getting the history to the prescriber.

Takeaway: Possible past hypomania in a depressed client is a bipolar-spectrum warning sign; get that history to the prescriber before an antidepressant starts.

Sources: DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Bipolar II Disorder; NASW Code of Ethics (2021), 2.03(a)
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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14. INT-1090 (rev 1) · Best answer: A

A college student whose parent died 3 months ago describes a typical day to a campus social worker. Mornings are spent looking at old photos and crying. Afternoons go to classes, estate paperwork, and learning to cook the parent's recipes for younger siblings. The student worries that this back-and-forth means something is wrong. Which framework **MOST** likely explains this pattern?

A. Dual process model
B. Worden's tasks of mourning
C. Kübler-Ross stages of dying
D. Continuing bonds

Why each option is right or wrong

A **Correct.** The dual process model describes healthy grieving as oscillation between loss-oriented coping, such as looking at photos and crying, and restoration-oriented coping, such as taking on new roles and tasks; time away from grief is part of adjustment (Stroebe & Schut, 1999). The student's daily alternation fits this closely, and the model frames the shifting as adaptive. It beats B because Worden's tasks describe what mourning must accomplish, such as processing pain and adjusting to a changed world, but they do not center on moving between the two kinds of coping.

B **Incorrect** (trap: Wrong model or concept). Worden's tasks (accept the reality, process the pain, adjust to a world without the person, find an enduring connection) fit parts of this picture (Worden, 2018). The defining feature here, though, is the daily alternation between grieving and restoring, the core of the dual process model.

C **Incorrect** (trap: Wrong model or concept). Kübler-Ross described stages such as denial, anger, bargaining, depression and acceptance, first in dying patients. The student is not moving through a sequence of emotional stages but alternating between two kinds of coping each day.

D **Incorrect** (trap: Wrong model or concept). Continuing bonds describes keeping an ongoing inner connection with the person who died, as cooking the parent's recipes may show. It explains that connection, not the alternation between grieving and getting on with new tasks.

Takeaway: The dual process model sees healthy grieving as oscillation between loss-oriented and restoration-oriented coping; time away from grief is adaptive.

Sources: Stroebe, M., & Schut, H. (1999). The dual process model of coping with bereavement: Rationale and description. Death Studies, 23(3), 197-224, whole article; Worden, J. W. (2018). Grief Counseling and Grief Therapy (5th ed.). Springer Publishing, four tasks of mourning
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.10 Working with loss, separation, and grief · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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15. INT-1060 (rev 1) · Best answer: B

At a drop-in center for adults experiencing homelessness, two clients begin shouting about a missing backpack. One stands inches from the other with fists clenched, and other clients have stopped to watch. What should the social worker do **FIRST**?

A. Bring both to a table to give their sides
B. Separate them calmly and speak with each apart
C. Warn both that more shouting will get them barred

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Hearing both sides is how the worker will settle the dispute, but it belongs after tempers cool. Bringing two aroused people face to face, with one already standing over the other, invites escalation.

B **Correct.** A conflict cannot be resolved while the parties are highly aroused, and safety comes before problem solving. Calmly creating physical space, moving away from the audience, and speaking with each person separately lowers the risk of a fight and lets emotions settle; respecting personal space and avoiding provocation are core de-escalation steps (Richmond et al., 2012). Once both are calmer, the worker can bring them together about the backpack. It beats A because a joint conversation is the right later step, but holding it now keeps the face-to-face standoff going.

C **Incorrect** (trap: Too directive). Clear limits may be needed later, but a threat delivered first, in front of onlookers, can feel humiliating and provoking. It does nothing to lower arousal and may push one client to act.

Takeaway: With parties in heated conflict, create space and calm each person separately before any joint problem solving.

Sources: Richmond, J. S., et al. (2012). Verbal de-escalation of the agitated patient: Consensus statement of the American Association for Emergency Psychiatry Project BETA De-escalation Workgroup. Western Journal of Emergency Medicine, 13(1), 17-25, ten domains of verbal de-escalation: respect personal space; do not be provocative
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.19 Resolving conflict · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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16. ASM-1064 (rev 1) · Best answer: C

During a telehealth intake, an adult client smiles steadily while describing a recent job loss and says, 'Honestly, I feel hollow most days.' The social worker is completing the mood section of the mental status exam. What is the **BEST** way to document the client's mood?

A. Record mood as euthymic, consistent with the steady smile
B. Record mood as depressed, based on the worker's impression
C. Record mood using the client's own description, in quotes

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). The smile is observed affect, not mood. Recording euthymic mood from outward expression confuses the two domains and contradicts what the client reported.

B **Incorrect** (trap: Misuses the technique). The worker's impression belongs in the affect section and the clinical formulation. Mood should reflect the client's own report, which here is more specific than a general label.

C **Correct.** In the MSE, mood is the client's subjective emotional state and is best recorded in the client's own words, while affect is the worker's observation of outward expression. Quoting the client keeps the record accurate and lets it show the mismatch between the smile (affect) and the reported hollowness (mood), which is itself clinically meaningful. It beats B because substituting the worker's label for the client's report blurs the line between subjective mood and observed affect (Kaplan & Sadock).

Takeaway: Document mood as the client's own report, quoted where possible; outward expression is documented separately under affect.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Mental status examination: mood and affect
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.10 Mental status exam: components and purpose · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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17. ETH-1122 (rev 1) · Best answer: B

A community mental health social worker learns in the morning that a long-term client died by suicide overnight. The supervisor offers to have colleagues cover the worker's afternoon, which includes two new crisis evaluations, and mentions a support meeting for staff. The worker feels numb and would rather stay busy. What is the **MOST** appropriate response for the worker?

A. Keep the afternoon's crisis evaluations to stay in routine
B. Accept coverage for this afternoon's crisis evaluations
C. Ask a colleague to sit in on the crisis evaluations

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Routine comforts some people after a loss, and lower-stakes tasks may be fine. Crisis evaluations, though, require full judgment, and numbness on the same morning signals that judgment may be affected.

B **Correct.** Self-care is part of competent, ethical practice (NASW Code, Purpose), and social workers must not let personal distress impair judgment in ways that could harm the people they serve (4.05(a)). The worker reports feeling numb only hours after the loss, and new crisis evaluations demand full attention to risk. Accepting coverage for those evaluations protects the new clients and gives the worker room to use staff support. B beats A because staying busy may feel better, but high-stakes risk assessments are the wrong place to test one's functioning on the day of a client's death.

C **Incorrect** (trap: Good but not best). A second clinician adds a check, but the worker would still lead high-stakes risk assessments hours after the loss while feeling numb. Full coverage protects the new clients better and frees the worker to use support.

Takeaway: After a client's death or other acute loss, accept support and coverage for high-stakes work rather than proving resilience.

Sources: NASW Code of Ethics (2021), Purpose (self-care paragraph); NASW Code of Ethics (2021), 4.05(a)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.7 Principles of self-care for the social worker · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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18. ETH-1001 (rev 1) · Best answer: A

Which term names a client's legal right to block a social worker's testimony about their sessions in court?

A. Privileged communication
B. Confidentiality
C. Privacy
D. Informed consent

Why each option is right or wrong

A **Correct.** Privileged communication is a legal rule of evidence that protects what a client says in treatment from compelled disclosure in legal proceedings. The privilege belongs to the client, who may assert or waive it, and federal courts recognize it for licensed clinical social workers (Jaffee v. Redmond, 1996). It beats B because confidentiality is the professional and ethical duty not to disclose, which by itself does not stop a court from compelling testimony (NASW Code 1.07(j)).

B **Incorrect** (trap: Wrong model or concept). Confidentiality is the social worker's ethical duty to keep client information private. It governs everyday disclosures, but a court can still compel testimony unless a legal privilege applies.

C **Incorrect** (trap: Wrong model or concept). Privacy is the client's broader right to control access to personal information, including what they share at all. It is a different idea from the rule of evidence that applies in court.

D **Incorrect** (trap: Wrong model or concept). Informed consent concerns agreeing to services after understanding their purpose, risks and alternatives. It does not decide whether session content can be compelled in court.

Takeaway: Privilege is the client's legal right to keep session content out of court; confidentiality is the worker's ethical duty.

Sources: Jaffee v. Redmond, 518 U.S. 1 (1996); NASW Code of Ethics (2021), 1.07(j)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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19. ETH-1601 (rev 2) · Best answer: B

In a hospital emergency department, a social worker is asked to assess a patient with limited English who took extra pills. The patient's adult sibling, who is fluent in English, offers to interpret. A bilingual nurse whose language skills have not been assessed is on shift. A telephone interpreter is available at once, and an in-person interpreter can arrive in 2 hours. What should the social worker do **FIRST**?

A. Let the sibling interpret so the assessment can begin
B. Start the assessment with the phone interpreter
C. Wait for the in-person interpreter before assessing
D. Ask the bilingual nurse to interpret the interview

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). A relative may filter the patient's words, and the patient may hide suicidal thoughts or family conflict in front of a sibling. Federal rules allow an accompanying adult to interpret only as an emergency stopgap until a qualified interpreter arrives, or at the patient's own private request. Here the sibling offered, and a qualified interpreter is ready.

B **Correct.** Health programs must offer a qualified interpreter and may not rely on an accompanying adult or untested bilingual staff except in narrow cases, such as an emergency stopgap or the patient's own private request (45 CFR 92.201(c), (e)). A qualified telephone interpreter is ready now, so the risk assessment after a possible overdose can start at once without depending on family. It beats C because an in-person interpreter adds nuance, but a 2-hour delay in assessing suicide risk is unsafe when a qualified option exists now. Clear communication is also needed for the patient to understand and take part (NASW Code 1.03(b)).

C **Incorrect** (trap: Ignores risk). In-person interpreting captures tone and gesture and suits long or complex sessions. Waiting 2 hours to assess a patient after a possible overdose delays safety decisions when a qualified telephone interpreter can start now.

D **Incorrect** (trap: Misuses the technique). Bilingual staff can interpret when their skills have been assessed and they are trained for the role. An untested speaker may miss or soften key risk statements, so this is the right kind of resource used the wrong way.

Takeaway: When interpretation is urgent, use a qualified remote interpreter now rather than family, untested staff or a delayed in-person interpreter.

Sources: Section 1557 of the Affordable Care Act regulations (45 CFR Part 92), 45 CFR 92.201(c), (e); NASW Code of Ethics (2021), 1.03(b)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

20. ASM-1135 (rev 2) · Best answer: B

PRETEST — unscored on this form; do not count it

A jail social worker interviews a detainee awaiting sentencing who reports hearing voices 'all day, every day' and asks to move to the mental health unit. The detainee describes the voices vaguely and denies thoughts of harming self or others. Officers' logs describe the detainee sleeping well and playing cards calmly with others. After completing the interview, what should the social worker do **NEXT**?

A. Recommend transfer to the mental health unit for observation
B. Request prior treatment records and reassess over several contacts
C. Document malingering based on the inconsistencies found

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). Transfer may be right if corroboration supports the symptoms or if risk emerges. The detainee denies thoughts of harm and appears settled in daily life, so moving now acts on the claim before checking it.

B **Correct.** DSM-5-TR advises strong suspicion of malingering when a legal context and a gap between reported and observed functioning appear together, but suspicion is not a conclusion. With the interview done, the next step is corroboration: prior records, observation by mental health staff and repeat contact. It beats C because labeling malingering now could deny care to someone with a real illness, and the evidence so far is suggestive, not conclusive. The findings then guide the level-of-care decision (DSM-5-TR).

C **Incorrect** (trap: Right action, wrong time). Malingering is recorded only when evidence shows intentional false or exaggerated symptoms for an external gain. Writing it now, after one interview, risks mislabeling a real illness and coloring every later decision.

Takeaway: Suspected malingering calls for corroboration from records and observation before any label or level-of-care change.

Sources: DSM-5-TR (APA, 2022), Other Conditions That May Be a Focus of Clinical Attention: Malingering
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 2 of 3

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21. ETH-1119 (rev 2) · Best answer: B

A social worker conducts forensic interviews at a child advocacy center. In recent weeks, the worker has begun cutting interviews short when children start giving details, startles at small noises, and lies awake replaying children's statements. Two interviews this month had to be repeated. What should the social worker do **FIRST**?

A. Complete a self-screening measure to gauge the symptoms
B. Tell the supervisor how the reactions affect interviews
C. Request a transfer out of forensic interviewing
D. Begin a daily mindfulness practice to ease symptoms

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Screening tools can track symptoms over time, but the impact is already clear: interviews are being repeated. Measuring first delays the consultation the Code calls for once work is affected.

B **Correct.** When personal distress impairs professional performance, social workers seek consultation immediately and take remedial action, such as getting help or adjusting workload (NASW Code 4.05(b)). Cutting interviews short and needing repeat interviews show the secondary traumatic stress symptoms are already affecting children's cases. Supervision is a core response to secondary traumatic stress and can address caseload and next steps (NCTSN, 2011). B beats D because self-care helps but does not address the immediate effect on interviews; it belongs in the plan made with the supervisor.

C **Incorrect** (trap: Skips assessment). A role change may become part of the plan, but requesting a transfer before consulting skips options such as caseload adjustment, supervision and treatment. That decision is best made with the supervisor.

D **Incorrect** (trap: Right action, wrong time). Mindfulness and rest support recovery, but they work slowly and privately. With interviews already compromised, the supervisor needs to know now so cases can be protected.

Takeaway: Once secondary traumatic stress affects job performance, consult a supervisor promptly; self-care and screening belong inside that plan.

Sources: NASW Code of Ethics (2021), 4.05(b); National Child Traumatic Stress Network, Secondary Traumatic Stress Committee (2011). Secondary Traumatic Stress: A Fact Sheet for Child-Serving Professionals, Sections Four and Five: Strategies for Prevention; Strategies for Intervention
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.6 Burnout, compassion fatigue, and secondary traumatic stress · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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22. ETH-1564 (rev 3) · Best answer: C

A field instructor at a middle school will complete an MSW intern's final evaluation in 3 weeks. For several months, the intern's case notes have been late and incomplete, but the field instructor has never raised the issue. The evaluation form includes documentation as a rated skill. What should the field instructor do **FIRST**?

A. Rate the intern's documentation as unsatisfactory on the evaluation
B. Email the intern a list of the late and incomplete notes
C. Meet with the intern now to review documentation against the criteria

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). An accurate rating matters and may still be low after the discussion. Issuing it with no prior feedback denies the intern the chance to understand and improve, which makes the evaluation unfair.

B **Incorrect** (trap: Misuses the technique). Flagging the missing notes is a start, and it is quick. A task list by email, though, does not review documentation against the evaluation criteria or give the intern a chance to discuss the skill and improve before it is rated.

C **Correct.** Evaluations should be fair, based on clearly stated criteria, and supported by planned, continuous feedback (NASW Code 3.02(b), 3.03; Supervision Standards,

Evaluation and Outcomes). The field instructor has let a known problem go unmentioned for months. Meeting now to review the notes against the stated criteria gives the intern specific feedback and time to improve before the rating. It beats A because a low rating may still be warranted, but giving it first, with no prior feedback, turns the evaluation into a surprise rather than a fair judgment.

Takeaway: Fair evaluation means feedback throughout against stated criteria; raise concerns before the final rating, not for the first time in it.

Sources: NASW Code of Ethics (2021), 3.02(b); NASW Code of Ethics (2021), 3.03; NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Evaluation and Outcomes
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.7 Ethics of supervising and managing · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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23. ETH-1112 (rev 1) · Best answer: B

The only social worker at a rural health clinic has a new client with gambling disorder who wants treatment. The nearest gambling specialist is 3 hours away, and the client cannot travel or use video. The social worker has general addiction training but has never treated gambling disorder. What should the social worker do **FIRST**?

A. Make a referral to the distant specialist
B. Arrange consultation with a gambling expert
C. Begin weekly gambling-focused sessions with the client

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). Referral is right when a qualified provider is reachable (1.16(a)). This client cannot travel or use video, so the referral would leave the client without treatment.

B **Correct.** Social workers practice within their competence and, before offering an intervention new to them, obtain appropriate study, training, consultation and supervision (NASW Code 1.04(a), 1.04(b)). This client cannot reach the only specialist, so a referral would leave no care at all. Consulting a gambling specialist by phone or email lets the social worker build a sound plan that draws on existing addiction skills (2.05(a), 2.05(b)). B beats C because starting gambling-specific treatment with no expert input risks ineffective care; sessions begin once consultation shapes the plan.

C **Incorrect** (trap: Right action, wrong time). Treating the client is the goal, but beginning a specialized treatment with no training or consultation risks harm. Sessions follow once a specialist's guidance shapes the plan.

Takeaway: When no qualified provider is reachable, build competence through expert consultation before and while serving the client.

Sources: NASW Code of Ethics (2021), 1.04(a); NASW Code of Ethics (2021), 1.04(b); NASW Code of Ethics (2021), 2.05(a); NASW Code of Ethics (2021), 2.05(b); NASW Code of Ethics (2021), 1.16(a)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.8 Keeping knowledge current: training, licensing CE, supervision, consultation · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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24. ETH-1156 (rev 2) · Best answer: B

Members of a hospital patient's family disagree about whether to continue the patient's dialysis. The social worker consults the hospital's ethics consultant, who suggests a family meeting focused on the patient's previously stated wishes. The social worker schedules the meeting for later in the week. What is **MOST** important to document about the consultation now?

A. A brief entry noting that a consultation occurred
B. When it occurred, the advice given and the action taken
C. A summary in a separate file kept out of the chart
D. The family meeting's outcome, once the meeting has been held

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). A brief entry that a consultation happened seems discreet, but it leaves out the advice and the action taken, so the team cannot see how the plan was reached. Documentation should be sufficient for care and continuity while staying relevant to services (3.04(b), 3.04(c)).

B **Correct.** Records should be sufficient and timely enough to support care and continuity (NASW Code 3.04(b)). A consultation about an ethically difficult decision is a key event to record: who was consulted, when, what was advised and what the worker did in response. This lets the team see how the plan was reached and shows that the decision followed recognized steps, including consultation (Reamer, 2018). It beats D because waiting to record only the meeting's outcome leaves out why the meeting was arranged and the advice behind it.

C **Incorrect** (trap: Misuses the technique). Keeping the consultation in a side file leaves the chart without information the care team needs to understand the plan. Consultation that shapes care belongs in the record, written with the same attention to relevance as any other entry.

D **Incorrect** (trap: Right action, wrong time). The meeting's outcome will need documenting too, but that comes later. Waiting until then delays the entry and drops the reasoning and advice that led to the meeting, which other team members may need before it is held.

Takeaway: Document consultations promptly: who was consulted, when, what was advised and what action followed.

Sources: NASW Code of Ethics (2021), 3.04(b); NASW Code of Ethics (2021), 3.04(c); Reamer, F. G. (2018). Social Work Values and Ethics (5th ed.). Columbia University Press, ethical decision-making framework: consult and document

Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** MOST · **Cognitive** application · **Difficulty** 1 of 3

25. INT-1637 (rev 1) · Best answer: A

By telehealth, a parent says the main goal is to stop yelling at the children, which happens most evenings. The parent and the social worker are writing a measurable objective for the next 6 weeks. Which objective follows goal-setting guidelines?

A. Use a calm voice for evening directions on 5 of 7 days
B. Stop yelling at the children completely within 6 weeks
C. Yell less often at the children over the next 6 weeks
D. Become a calmer, more patient parent by the end of 6 weeks

Why each option is right or wrong

A **Correct.** Well-formed objectives are explicit and measurable, feasible, and stated in positive terms: they name what the client will do rather than what the client will stop doing (Hepworth et al.). A calm voice for evening directions on 5 of 7 days gives the parent a behavior to practice and a count to track. It beats B because B can be measured but names only the absence of a behavior and sets a perfection target for a habit that occurs most evenings, which invites early failure.

B **Incorrect** (trap: Misuses the technique). This objective can be measured, but it tells the parent only what not to do and expects perfection within 6 weeks from a nearly nightly pattern. Positive, attainable targets are easier to practice and sustain.

C **Incorrect** (trap: Misuses the technique). 'Less often' gives no amount, so neither the parent nor the social worker can tell whether the objective was met. A baseline and a target number would make it measurable.

D **Incorrect** (trap: Misuses the technique). Calm and patient are worthy aims, but they describe traits rather than observable behaviors. Without a specified action and amount, progress cannot be tracked.

Takeaway: Write objectives as positive, observable behaviors with a measurable, attainable target, naming what the client will do rather than stop doing.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Guidelines for selecting and defining goals: stated in positive terms
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.15 Contracting and goal setting: partializing, measurable objectives · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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26. ETH-1045 (rev 1) · Best answer: A

A staff member at an after-school program tells the program's social worker that an 8-year-old has lingered at pickup each Friday this month and once asked to stay late. The staff member describes the child as otherwise cheerful and engaged. What should the social worker do **FIRST**?

A. Talk with the child using open, non-leading questions
B. Report the staff member's concern to child protective services
C. Ask the parent at pickup whether anything is wrong at home

Why each option is right or wrong

A **Correct.** Reporting laws cover known or suspected abuse (CAPTA, 42 U.S.C. 5106a(b)(2)(B)(i)), and lingering at pickup has many explanations, from enjoying the program to a change at home that is not harmful. A brief, open, non-leading conversation within the worker's role clarifies whether suspicion exists without shaping any later disclosure. A beats B because reporting a vague behavior acts before there is cause; if the conversation raises suspicion, the worker reports right away.

B **Incorrect** (trap: Skips assessment). A report is required once there is reasonable suspicion, and the worker should not hesitate if one develops. Lingering at pickup alone does not reach that point, so a report now discloses information without a compelling reason (NASW Code 1.07(c)).

C **Incorrect** (trap: Ignores risk). Raising concerns with a parent before understanding them could alert a caregiver who may be harming the child and put the child at risk. Parent contact may fit later if the concern proves harmless.

Takeaway: When a concern falls short of reasonable suspicion, clarify it briefly and without leading questions; report as soon as suspicion forms.

Sources: Child Abuse Prevention and Treatment Act, 42 U.S.C. 5106a(b)(2)(B)(i); NASW Code of Ethics (2021), 1.07(c)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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27. ASM-0110 (rev 1) · Best answer: C

A social worker at a family services agency holds an intake with a client who has limited English proficiency. A trained interpreter is present in the room. The client is seeking help with a child's attendance problems at school. How should the social worker **BEST** conduct the interview?

A. Ask the interpreter to summarize the client's answers
B. Pose questions to the interpreter to relay to the client
C. Speak directly to the client in the first person

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Summaries filter the client's words through the interpreter's judgment, so details, emotion and meaning can be lost. Interpreters should render everything said, completely and accurately, rather than condense it.

B **Incorrect** (trap: Good but not best). Talking about the client in the third person ('Ask what brings them in') treats the interpreter as the person being interviewed. It weakens rapport and can leave the client feeling talked about rather than talked to.

C **Correct.** When working through an interpreter, the social worker speaks directly to the client in the first person ('What brings you in today?'), and the interpreter renders everything said in both directions (NCIHC, 2024). This keeps the helping relationship between the social worker and the client and preserves the client's own words. It beats B because addressing the interpreter in the third person sidelines the client and makes the interpreter the focus of the interview. Using a qualified interpreter also supports the client's understanding (NASW Code 1.03(b)).

Takeaway: With an interpreter, talk directly to the client in the first person and expect everything to be interpreted, not summarized.

Sources: National Council on Interpreting in Health Care (2024). Guide for Partnering with an Interpreter, speak directly to the patient; first-person interpreting; NASW Code of Ethics (2021), 1.03(b)
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.2 Interviewing skills: clarifying, validating, confronting, reflecting, interpreters · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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28. ASM-0100 (rev 2) · Best answer: B

Two days after a tornado destroyed several homes in a small town, a social worker staffs a community shelter. Many residents report poor sleep, jumpiness at loud sounds and repeated images of the storm. Most are still caring for their families and helping with cleanup. What **MOST** likely explains these responses?

A. Posttraumatic stress disorder
B. Expected acute stress reactions
C. Acute stress disorder
D. Adjustment disorder with anxiety

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). PTSD requires symptoms lasting more than 1 month after the event, along with distress or impairment. At 2 days, no one can meet the duration requirement. It becomes a consideration if symptoms persist and impair functioning beyond a month.

B **Correct.** Most people exposed to a disaster have short-term stress reactions such as poor sleep, startle and intrusive images, and most recover without developing a disorder (Brymer et al., 2006). These residents are 2 days out and still functioning, caring for family and helping with cleanup. It beats C because acute stress disorder cannot be diagnosed until symptoms have lasted at least 3 days, and it requires significant distress or impairment (DSM-5-TR). The fitting response now is psychological first aid and monitoring, not a diagnosis.

C **Incorrect** (trap: Near-miss diagnosis). Acute stress disorder applies from 3 days to 1 month after trauma, when many symptoms cause significant distress or impairment. These residents are only 2 days out and functioning. It is worth screening for if symptoms continue and worsen.

D **Incorrect** (trap: Near-miss diagnosis). Adjustment disorder fits a reaction to a stressor that is out of proportion or impairs functioning and does not meet criteria for another disorder. Common, short-lived responses in people who are coping well do not reach that bar.

Takeaway: Early stress reactions after a disaster are expected; diagnose only when duration and impairment thresholds are met, and offer psychological first aid meanwhile.

Sources: DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Acute Stress Disorder; Brymer, M., Jacobs, A., Layne, C., Pynoos, R., Ruzek, J., Steinberg, A., Vernberg, E., & Watson, P. (2006). Psychological First Aid: Field Operations Guide (2nd ed.). National Child Traumatic Stress Network and National Center for PTSD, introduction and overview
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.1 Trauma, stress, violence, crisis: signs and impact (historical, complex) · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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29. INT-1103 (rev 1) · Best answer: C

In a primary care clinic, a client who has drunk about a fifth of liquor daily for years says they will quit alone this weekend. Last year, the client had a seizure two days after stopping suddenly. The client asks the social worker for tips to get through the weekend. What should the social worker do **FIRST**?

A. Plan coping strategies and supports to use over the weekend
B. Explore what is motivating the client to quit drinking now
C. Arrange medically supervised withdrawal before the client stops

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). The client asked for coping tips, and they will help, but they do not address the medical danger of stopping suddenly after a prior withdrawal seizure. Coping planning fits alongside or after medically managed withdrawal.

B **Incorrect** (trap: Good but not best). The client has already decided to quit this weekend, so exploring motivation is not the priority. Exploring reasons fits a client who is unsure about change; here the urgent issue is a safe way to stop.

C **Correct.** Safety comes first. A past withdrawal seizure is a major warning sign for complicated alcohol withdrawal, so stopping heavy daily drinking without medical care could again cause seizures or delirium (ASAM, 2020). The client's readiness to quit is a strength; the task is to make quitting safe by linking the client to medically supervised withdrawal management. It beats A because coping strategies matter but cannot prevent a medical complication; they belong in the plan once withdrawal care is in place.

Takeaway: A history of withdrawal seizures means stopping heavy drinking needs medical withdrawal management first; coping plans come after safety.

Sources: ASAM (2020). The ASAM Clinical Practice Guideline on Alcohol Withdrawal Management. Journal of Addiction Medicine, 14(3S), 1-72, Assessment and level of care: risk factors for complicated withdrawal
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.17 Intervening with addiction · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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30. INT-1129 (rev 1) · Best answer: C

A client in standard outpatient DBT reports at the start of an individual session that they cut their forearm 2 days ago after an argument. The social worker checks the wound, which needed no medical care, and confirms the client has no current urge or intent to self-harm. What should the social worker do **NEXT**?

A. Teach a new distress tolerance skill to use after arguments
B. Turn to the argument and explore what made it so painful
C. Do a chain analysis of the events that led to the cutting

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Teaching a skill is part of the response, but in DBT it follows the chain analysis, which shows where in the sequence a skill would help. Choosing one now skips the analysis that makes the skill a good fit.

B **Incorrect** (trap: Misuses the technique). The argument is one link in the chain, but an open talk about feelings misses the structured, step-by-step review of vulnerabilities, links, and consequences that DBT uses to understand and change a target behavior.

C **Correct.** In DBT, self-harm since the last session is the top target in individual therapy, and the standard response is a behavioral chain analysis: a moment-by-moment review of vulnerabilities, the prompting event, thoughts, feelings, urges, the behavior, and its consequences (Linehan, 1993). Current safety has been checked, so the chain analysis comes next. It shows the links where skills could replace cutting. It beats A because picking a skill before the chain is mapped guesses at which link needs to change; solutions follow the analysis.

Takeaway: In DBT, review self-harm since the last session with a detailed chain analysis, then match skills to the links it reveals.

Sources: Linehan, M. M. (1993). Cognitive-Behavioral Treatment of Borderline Personality Disorder. Guilford Press, behavioral analysis: chain analysis; target hierarchy
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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31. ETH-1717 (rev 2) · Best answer: D

PRETEST — unscored on this form; do not count it

A social worker has treated an asylum seeker for PTSD for 4 months. In earlier sessions the client described being beaten in detention in the home country. The client's immigration attorney asks the social worker to write a forensic evaluation stating whether the client's symptoms are consistent with torture, for the asylum hearing. The social worker has never conducted forensic evaluations. What is the **BEST** response?

A. Write the asylum evaluation from the 4 months of session notes
B. Write the evaluation after consulting a forensic evaluator
C. Ask the client to describe the abuse again in full detail
D. Refer the client to an evaluator trained in asylum cases

Why each option is right or wrong

A **Incorrect** (trap: Outside the social work role). Treatment notes record history and symptoms, but a forensic opinion on consistency with torture calls for structured methods and training the worker lacks. An unsound evaluation could weaken the asylum claim and blur the treating role.

B **Incorrect** (trap: Boundary slip or collusion). Consultation can support work in a new area (NASW Code 1.04(b)), but one consultation does not supply forensic training. The worker would also be evaluating their own client, a treating-forensic role conflict that can weaken both the therapy and the evaluation's credibility (Greenberg & Shuman, 1997).

C **Incorrect** (trap: Ignores risk). The client has already described the abuse, and repeating it in detail can be retraumatizing. More detail would not give the worker the forensic skills the request requires.

D **Correct.** Social workers practice within the limits of their training (NASW Code 1.04(a)). Asylum evaluations are forensic work with specialized standards for

documenting torture (Istanbul Protocol; OHCHR, 2022), and acting as both treating clinician and forensic evaluator creates role conflict (Greenberg & Shuman, 1997). Referring to a trained evaluator protects the client's claim and the treatment relationship; with consent, the worker can still share a factual treatment summary. It beats A because notes gathered for treatment do not meet forensic standards, and an untrained opinion could hurt the claim.

Takeaway: Forensic asylum evaluations need specialized training; treating clinicians without it should refer, and may share factual treatment summaries with consent.

Sources: NASW Code of Ethics (2021), 1.04(a); NASW Code of Ethics (2021), 1.04(b); OHCHR (2022). Istanbul Protocol: Manual on the Effective Investigation and Documentation of Torture and Other Cruel, Inhuman or Degrading Treatment or Punishment (Professional Training Series No. 8/Rev.2), whole manual; Greenberg, S. A., & Shuman, D. W. (1997). Irreconcilable conflict between therapeutic and forensic roles. Professional Psychology: Research and Practice, 28(1), 50-57, whole article

Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.5 Core values: competence, social justice, integrity, dignity and worth · **Qualifier** BEST · **Cognitive** application · **Difficulty** 3 of 3

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32. INT-1570 (rev 1) · Best answer: A

A social worker providing telehealth services plans to evaluate exposure work for a client with social anxiety. During a 2-week baseline, the client records how many conversations they start each day. The counts rise steadily across the 14 days, from 1 a day to 5 a day. The client is not in crisis. What should the social worker do **NEXT**?

A. Keep collecting daily counts until they level off

B. Begin the planned exposure sessions on schedule

C. Replace the baseline counts with a standardized anxiety scale

Why each option is right or wrong

A **Correct.** Change during an intervention can be credited to it only if the baseline is stable. When baseline data already trend in the desired direction, the usual step is to extend the baseline until it levels off, or to reconsider whether intervention is needed (Bloom, Fischer & Orme, 2009). The client is improving without exposure and is not in crisis, so waiting costs little. It beats B because starting now would make further gains impossible to separate from the existing upward trend.

B **Incorrect** (trap: Misuses the technique). Keeping to the schedule seems efficient, but an improving baseline means later gains could simply continue the trend. Starting now is reasonable when delay would harm the client, which is not the case here.

C **Incorrect** (trap: Misuses the technique). A standardized scale can add useful data, but switching measures does not solve the problem of a trending baseline and breaks the continuity needed to compare phases.

Takeaway: Begin an intervention phase only after a stable baseline; if the baseline trends toward improvement, extend it or reconsider the need.

Sources: Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, single-system designs: baselines, AB, withdrawal and multiple-baseline designs

Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.1 Evaluating progress and effectiveness of intervention and service plans · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 3 of 3

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33. ASM-1004 (rev 1) · Best answer: B

In a hospice bereavement program, a client meets with the social worker 18 months after an adult child died. Nearly every day the client longs intensely for the child and says, 'Part of me died too.' The client avoids the child's neighborhood, has stopped seeing friends and feels life has no purpose. The distress far exceeds what the client's faith community expects. What is the **MOST** likely diagnosis?

A. Major depressive disorder
B. Prolonged grief disorder
C. Uncomplicated bereavement

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Major depressive disorder centers on pervasive low mood or loss of interest, with symptoms such as worthlessness and sleep or appetite change. The features here focus on longing for the child and loss of identity, which fit a loss-specific diagnosis better.

B **Correct.** Prolonged grief disorder, added in DSM-5-TR, applies in adults when at least 12 months have passed since the death and, for at least the past month, intense yearning or preoccupation occurs nearly daily with at least three further features, such as identity disruption, avoidance of reminders, trouble re-engaging and a sense of meaninglessness (DSM-5-TR). This client shows all of these, beyond cultural norms. It beats A because the distress centers on the lost child rather than a pervasive low mood with worthlessness; major depression can co-occur but is not the best fit here.

C **Incorrect** (trap: Near-miss diagnosis). Uncomplicated bereavement is an expected response to a death, recorded as a condition for clinical attention rather than a disorder. Daily intense longing 18 months later, with impairment beyond cultural norms, goes beyond it.

Takeaway: Adult yearning for the deceased nearly daily, 12 or more months after a death, with impairment beyond cultural norms, suggests prolonged grief disorder.

Sources: DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Prolonged Grief Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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34. INT-1081 (rev 1) · Best answer: C

Late at night, an 8-year-old waits at a child welfare office after being removed from home. The required investigative interview is already done. The child is crying and keeps asking, 'Where am I going to sleep? Who's going to feed my dog?' What would be **MOST** helpful for the social worker to do?

A. Teach the child a slow-breathing exercise to calm down
B. Promise the child they will go home within a few days
C. Tell the child plainly what will happen tonight, step by step

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Brief calming skills are part of stabilization and may help later tonight. This child, however, is asking specific questions; teaching a skill instead of answering them leaves the main source of fear in place.

B **Incorrect** (trap: Misreads law or ethics). A promise to return home soon may calm the child for a moment, but the worker cannot know the outcome. If the promise fails, trust is damaged when the child most needs reliable adults. Honest, simple information is safer.

C **Correct.** Psychological First Aid stresses safety, comfort and simple, accurate information about what will happen next, which restores some predictability (Brymer et al., 2006). This child's distress centers on concrete unknowns: where to sleep and who will care for the dog. Answering those plainly, including what the worker can find out about the pet, targets the actual source of fear. It beats A because calming skills are useful, but a breathing exercise leaves the child's urgent questions unanswered, and the worry returns when the exercise ends.

Takeaway: For a child in acute crisis, give simple, honest information about immediate next steps; uncertainty is often the core fear.

Sources: Brymer, M., et al. (2006). Psychological First Aid: Field Operations Guide (2nd ed.). National Child Traumatic Stress Network and National Center for PTSD, Core Action 2: Safety and Comfort (providing information; attending to children)

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.5 Crisis intervention and crisis treatment approaches · **Qualifier** MOST · **Cognitive** reasoning · **Difficulty** 2 of 3

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35. ETH-1167 (rev 2) · Best answer: A

A social worker leads the evaluation of a parenting program that runs at four sites. Outcomes improved at three sites but not at the fourth, which had heavy staff turnover during the year. Worried about renewal, the agency director asks the social worker to leave the fourth site out of the report to the funder. What is the **BEST** response?

A. Report every site and explain the fourth site's context

B. Drop the fourth site from the report as asked

C. Combine the four sites into one overall average

Why each option is right or wrong

A **Correct.** Evaluators must report findings accurately and must not fabricate or falsify results (NASW Code 5.02(o)); leaving out an unfavorable site misrepresents the program's results. Accurate reporting can still be fair to the agency: describing the staff turnover at the fourth site gives the funder a plausible explanation and a lesson for improvement. Social workers monitor and evaluate programs (5.02(a)), and that work can guide decisions only if the funder sees the full picture. It beats C because pooling the sites technically includes the data but hides the site-level difference the funder most needs to see.

B **Incorrect** (trap: Misreads law or ethics). Dropping the site because its results are disappointing is selective reporting, which falsifies the overall findings. Funding pressure does not change the duty to report results accurately.

C **Incorrect** (trap: Misuses the technique). Pooling all sites into a single average includes the data but hides the site-level difference that matters most. Reporting choices made to conceal an unfavorable result still misrepresent the findings.

Takeaway: Funding pressure never justifies dropping or masking unfavorable evaluation results; report them with honest context.

Sources: NASW Code of Ethics (2021), 5.02(o); NASW Code of Ethics (2021), 5.02(a)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.11 Research ethics: IRBs, human subjects protections, consent · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 1 of 3

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36. INT-0108 (rev 1) · Best answer: D

During a couples session about household spending, one partner rolls their eyes and says in a mocking tone, 'Oh, here comes the financial genius again.' Which of Gottman's Four Horsemen does this behavior show?

A. Criticism
B. Defensiveness
C. Stonewalling
D. Contempt

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Criticism attacks the partner's character instead of naming a specific complaint, as in 'You're so careless with money.' The sarcasm and eye-rolling here go further, signaling scorn and superiority.

B **Incorrect** (trap: Wrong model or concept). Defensiveness is self-protection through excuses, counterattacks or playing the innocent victim in response to a complaint. This partner is mocking the other, not defending against a complaint.

C **Incorrect** (trap: Wrong model or concept). Stonewalling is withdrawing from the interaction by going silent, looking away or shutting down, often when flooded. This partner stays engaged, but scornfully.

D **Correct.** Contempt communicates disrespect and superiority, often through sarcasm, mockery, name-calling or eye-rolling (Gottman, 1994). The eye roll and the mocking label 'financial genius' are classic signs. Gottman found contempt to be the strongest single predictor of divorce, so it is a priority target in couples work, often countered by nurturing fondness and admiration (Gottman & Silver, 1999). It beats A because criticism attacks the partner's character with a complaint, such as 'You never think about money,' while contempt adds open scorn.

Takeaway: In Gottman's model, sarcasm, mockery and eye-rolling signal contempt, the strongest predictor of divorce; criticism attacks character without open scorn.

Sources: Gottman, J. M. (1994). What Predicts Divorce? The Relationship Between Marital Processes and Marital Outcomes. Lawrence Erlbaum, Four Horsemen: contempt; Gottman, J. M., & Silver, N. (1999). The Seven Principles for Making Marriage Work. Crown, Four Horsemen; nurturing fondness and admiration
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.27 Couples treatment approaches · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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37. ASM-1116 (rev 1) · Best answer: C

On a medical unit, a 78-year-old patient treated for a urinary tract infection is confused each evening and refuses the planned move to a rehabilitation facility. Each morning the patient is alert, oriented and asking about home. The transfer is planned for 4 days from now. What should the social worker do **FIRST**?

A. Ask the adult child to make the transfer decision
B. Record the evening refusal as the patient's decision
C. Revisit the decision with the patient early in the day

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A family member may act as surrogate if the patient lacks capacity for the decision. That has not been shown, and the patient is clear each morning, so turning to the adult child now sidelines a patient who may decide for themselves.

B **Incorrect** (trap: Skips assessment). A refusal given while confused may not reflect the patient's real wishes. Treating it as final skips an assessment during the patient's clearer hours.

C **Correct.** Capacity can fluctuate, so it is best assessed when the person is at their clearest, especially when the decision can wait (Appelbaum, 2007). The confusion appears in the evening, a pattern typical of delirium, which DSM-5-TR describes as fluctuating over the day, and the transfer is 4 days away. Revisiting the move early in the day lets the patient make their own decision before anyone else steps in. It beats A because a surrogate is a later step, used only if the patient lacks capacity for this choice even when most alert.

Takeaway: When capacity fluctuates and the decision can wait, assess it during the person's clearest periods before involving a surrogate.

Sources: Appelbaum, P. S. (2007). Assessment of patients' competence to consent to treatment. New England Journal of Medicine, 357(18), 1834-1840, Legally relevant criteria for decision-making capacity; DSM-5-TR (APA, 2022), Neurocognitive Disorders: Delirium
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.23 Client capacity, self-determination, self-monitoring (consent age, emancipation, permanency) · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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38. INT-1158 (rev 2) · Best answer: A

In a caregiver support group at a senior center, one member has attended 5 sessions, listens closely, and speaks only when asked a direct question. During a break, the member explains that in their family, talking about relatives' problems with outsiders brings shame, but hearing the others 'helps a lot.' What is the **MOST** appropriate response by the social worker?

A. Use go-around turns in which any member may pass
B. Invite the member to share one struggle at the next session
C. Suggest individual counseling as a better fit for the member

Why each option is right or wrong

A **Correct.** Group leaders respect silence, consider its cultural meaning, and invite participation without forcing disclosure (Corey, Corey & Corey, 2018). The member has explained that sharing family problems with outsiders brings shame, and says the group is helping. Structured rounds with a clear option to pass keep the door open without singling the member out, which honors the member's cultural values (NASW Cultural Competence Standard 4). It beats B because a direct invitation puts the member on the spot and pushes against the norm the member just described.

B **Incorrect** (trap: Too directive). A personal invitation can help a shy member who wants to speak but needs an opening. This member has described a cultural norm against such sharing, so being asked to disclose a struggle in the group risks shame and withdrawal.

C **Incorrect** (trap: Hands off too soon). The member is benefiting from listening and wants to stay. Moving the member to individual counseling treats a cultural style as a poor fit and removes a support the member values.

Takeaway: Respect culturally rooted silence in groups: offer low-pressure ways to take part, such as rounds with the option to pass, instead of pressing for disclosure.

Sources: Corey, M. S., Corey, G., & Corey, C. (2018). Groups: Process and Practice (10th ed.). Cengage, problem behaviors: silence and lack of participation; NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 4: Cross-Cultural Skills

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.23 Group work: managing process and building cohesion · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 3 of 3

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39. INT-1553 (rev 2) · Best answer: C

A patient with metastatic colon cancer has received home hospice care for 6 weeks. The oncologist mentions a clinical trial of a new treatment, and the patient wants to try it. The patient's adult child tells the social worker they fear that leaving hospice means the patient can never return to it. How should the social worker respond?

A. Confirm that revoking hospice ends the patient's future eligibility

B. Explain that a return is possible once the current benefit period ends

C. Explain that hospice can be revoked now and elected again later

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). This repeats the family's fear as fact. Revocation ends hospice coverage only for the current election period, and the patient can elect hospice again later if still eligible. Misinformation could push the patient into a choice they do not want.

B **Incorrect** (trap: Misreads law or ethics). This mixes up two rules. Revoking does give up the days left in the current election period, but the patient may elect hospice again at any time for a later period they qualify for; there is no wait until the old period would have ended. A false waiting period could weigh on the patient's choice.

C **Correct.** Under the Medicare hospice benefit, a patient may revoke the hospice election at any time and may elect hospice again for later periods they are eligible for (42 CFR 418.28). Hospice election waives coverage for treatment aimed at the terminal

illness, so pursuing the trial generally means revoking first. Accurate information lets the patient make an informed choice without fearing the loss of comfort care (NASW Code 1.02). It beats B because revocation forfeits only the rest of the current period; re-election does not have to wait for that period to run out.

Takeaway: Give patients accurate hospice information: the election can be revoked to pursue treatment and elected again later if eligible.

Sources: Medicare hospice regulations, 42 CFR Part 418, 42 CFR 418.28 (revoking the election of hospice care); NASW Code of Ethics (2021), 1.02

Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.5 End-of-life practice: hospice, palliative care, continuity, dying stages · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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40. INT-1064 (rev 1) · Best answer: A

In month 3 of an outpatient process group, a member often makes jokes when others share painful stories. Two members tell the member that the jokes leave them feeling brushed off. Over the next weeks, the member says a partner and coworkers have complained of the same habit, and starts responding seriously in group. Which therapeutic factor is **MOST** likely at work?

A. Interpersonal learning
B. Catharsis
C. Universality
D. Imitative behavior

Why each option is right or wrong

A **Correct.** Yalom describes interpersonal learning as discovering, through members' honest feedback in the here and now, how one's behavior affects others, then trying new behavior in the group. The group works as a social microcosm: the member hears how the jokes land, links the pattern to life outside, and changes in session (Yalom & Leszcz, 2020). It beats D because imitative behavior means modeling oneself on other members or the leader; here the change follows direct feedback about the member's own impact.

B **Incorrect** (trap: Wrong model or concept). Catharsis is the release of strong feeling that brings relief. The member's change came from hearing feedback and recognizing a pattern, not from expressing pent-up emotion.

C **Incorrect** (trap: Wrong model or concept). Universality is the relief of finding that others share one's problems. The member is not discovering shared struggles; they are hearing how their own behavior affects others and changing it.

D **Incorrect** (trap: Wrong model or concept). Imitative behavior means copying another member's or the leader's way of acting. Nothing suggests the member is modeling anyone; the change follows feedback about their own habit.

Takeaway: Interpersonal learning: honest member feedback in the here and now shows a person their impact, and the group becomes a place to try new behavior.

Sources: Yalom, I. D., & Leszcz, M. (2020). The Theory and Practice of Group Psychotherapy (6th ed.). Basic Books, therapeutic factors: interpersonal learning; the group as social microcosm

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.23 Group work: managing process and building cohesion · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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41. ASM-0105 (rev 1) · Best answer: A

A client in an outpatient program arrives restless and irritable after missing 2 days of treatment. The social worker notices yawning, a runny nose, watery eyes, dilated pupils and goosebumps. The client reports muscle aches and diarrhea. Which condition is the client **MOST** likely experiencing?

A. Opioid withdrawal
B. Opioid intoxication
C. Stimulant intoxication
D. Alcohol withdrawal

Why each option is right or wrong

A **Correct.** Opioid withdrawal produces flu-like signs: yawning, runny nose, tearing, dilated pupils, goosebumps, muscle aches and diarrhea, along with restlessness and low mood (DSM-5-TR). This client shows nearly the full cluster. It beats D because alcohol withdrawal centers on tremor, sweating, rapid pulse and possible seizures or hallucinations, without the tearing, runny nose, yawning and goosebumps typical of opioids. Opioid withdrawal is rarely life-threatening in healthy adults, but it raises overdose risk if the client returns to use with lower tolerance.

B **Incorrect** (trap: Near-miss diagnosis). Opioid intoxication looks nearly opposite: constricted (pinpoint) pupils, drowsiness, slurred speech and slowed breathing. Dilated pupils and restlessness point away from intoxication.

C **Incorrect** (trap: Near-miss diagnosis). Stimulant intoxication can cause dilated pupils and restlessness, which makes it tempting. It does not explain yawning, tearing, runny nose, goosebumps and diarrhea, which together point to opioid withdrawal.

D **Incorrect** (trap: Near-miss diagnosis). Alcohol withdrawal brings tremor, sweating, rapid heartbeat, insomnia and, in severe cases, seizures or hallucinations. It lacks the runny nose, tearing, yawning and goosebumps seen here.

Takeaway: Opioid withdrawal: yawning, tearing, runny nose, dilated pupils, goosebumps, aches and diarrhea. Opioid intoxication: pinpoint pupils and sedation.

Sources: DSM-5-TR (APA, 2022), Substance-Related and Addictive Disorders: Opioid Withdrawal; DSM-5-TR (APA, 2022), Substance-Related and Addictive Disorders: Opioid Intoxication
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.8 Addiction and substance use: signs and impact on systems · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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42. INT-1082 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

A client with a history of repeated childhood abuse starts outpatient therapy and asks to 'go through the memories' right away. In the past month, the client has cut their arms several times, has been sleeping in their car, and often loses track of time during sessions. The client says reading about trauma therapy gave them hope. What should the social worker focus on **FIRST**?

A. Begin detailed processing of the abuse memories as requested
B. Work with the client on safety and stabilization
C. Refer the client to an inpatient trauma program

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Trauma processing is the client's goal and a valid later step. Starting it while the client is cutting, without stable housing and dissociating risks pushing the client beyond what they can tolerate. Processing fits once safety and coping are established.

B **Correct.** Phase-oriented treatment of complex trauma begins with establishing safety and stabilization before remembrance and mourning (Herman, 1992). Current cutting, unstable housing and frequent dissociation in session show the client cannot yet tolerate intense memory work. The client's hope is honored by framing stabilization as the first part of trauma treatment, not a refusal. It beats A because following the request respects choice, but processing memories while the client is self-harming and dissociating risks overwhelming them; the goal is pursued in sequence.

C **Incorrect** (trap: Hands off too soon). Inpatient care fits imminent danger or an inability to stay safe with outpatient support. This client is engaged and seeking help, so the first step is stabilization within the current relationship, with a higher level of care considered if safety worsens.

Takeaway: With complex trauma, establish safety and stabilization before trauma processing; current self-harm, instability or dissociation mark the first phase.

Sources: Herman, J. L. (1992). Trauma and Recovery. Basic Books, stages of recovery: establishing safety
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.1 Trauma-informed care across trauma types (complex, vicarious) · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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43. INT-0111 (rev 1) · Best answer: C

PRETEST — unscored on this form; do not count it

At intake, the social worker explains the court's attendance requirement and what will be reported to the probation officer. The client says, 'I don't have an anger problem. I'm only here so the judge stays off my back.' Which response by the social worker is **BEST**?

A. Review the police report with the client to address the denial
B. Repeat the court's requirements and the reporting rules
C. Ask what the client would want to gain from finishing the program

Why each option is right or wrong

A **Incorrect** (trap: Too directive). Confronting denial with the police report this early tends to increase defensiveness and discord. Discrepancies can be explored later, gently, once rapport exists.

B **Incorrect** (trap: Good but not best). Clarifying requirements and reporting is essential, but it was just done. Repeating it in response to the client's frustration can feel like pressure and does not invite the client into the work.

C **Correct.** With mandated clients, engagement grows when the social worker looks for a goal the client actually wants within the mandate's limits, such as getting the court out of their life (Rooney, 2009). The requirements and reporting rules have already been explained, so the useful move now is to invite the client's own reasons for completing the program. Offering real choice reduces reactance. It beats B because repeating

rules the client just heard adds nothing new and may sound like a threat, which tends to deepen resistance rather than build engagement.

Takeaway: With a mandated client, after clarifying the mandate, look for a goal the client owns, such as ending outside involvement.

Sources: Rooney, R. H. (Ed.). (2009). Strategies for Work with Involuntary Clients (2nd ed.). Columbia University Press, engaging involuntary clients; reactance; Miller & Rollnick (2023). Motivational Interviewing (4th ed.). Guilford Press, engaging; discord

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.11 Engaging and motivating voluntary and mandated clients · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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44. ASM-1692 (rev 1) · Best answer: C

A couple began counseling 5 months ago for frequent arguments. For the past 2 months, they report using time-outs and calm check-ins on their own, and they recently settled a disagreement about moving without help. They canceled the last two sessions. When the social worker asks about the cancellations, both say things are going well and they feel less need to come. What should the social worker do **NEXT**?

A. Explore whether the cancellations reflect unspoken dissatisfaction

B. Continue biweekly sessions to guard against slipping back

C. Propose planning an ending with a final review session

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Checking whether cancellations signal dissatisfaction is wise, and the worker has already asked. Both partners tied the cancellations to progress, so probing again would second-guess a clear answer.

B **Incorrect** (trap: Good but not best). Ongoing sessions may seem protective, but the couple has shown they keep their gains without help. Extending services past need does not serve them; a return option can be offered at the ending.

C **Correct.** Signs of readiness to end include goals met, skills used independently over time, and gains that hold under real stress (Hepworth et al.). This couple has used their skills alone for 2 months, handled a major decision without help, and, when asked, linked the cancellations to doing well. Proposing a planned ending with a final review consolidates gains and prevents an unplanned drift out of services. It beats B because continuing sessions the couple no longer needs does not serve them, and the Code calls for ending services when they are no longer required (NASW Code 1.17(a)).

Takeaway: When clients use skills independently and gains hold under stress, propose a planned ending rather than continuing or letting services drift.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Termination: indicators of readiness and planned endings; NASW Code of Ethics (2021), 1.17(a)

Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.6 Signs a client is ready for termination · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 2 of 3

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45. ASM-0003 (rev 1) · Best answer: A

A week after a parent's death, a client spends the session describing research on the stages of grief and how bereavement affects the immune system. When the social worker asks how the client is feeling, the client returns to discussing studies. Which defense mechanism is the client **MOST** likely using?

A. Intellectualization
B. Denial
C. Rationalization
D. Reaction formation

Why each option is right or wrong

A **Correct.** Intellectualization uses abstract thinking, facts and analysis to keep painful emotions at a distance (Vaillant, 1992). This client acknowledges the loss but talks about research instead of feelings, and returns to it when asked about emotions. It beats B because denial refuses to accept the reality of an event, and this client openly discusses the parent's death. Intellectualization is not pathological in itself. Recognizing it helps the social worker gently invite feelings at the client's pace.

B **Incorrect** (trap: Wrong model or concept). Denial means refusing to acknowledge an external reality, such as insisting the parent is still alive. This client accepts the death and talks about bereavement, which rules denial out.

C **Incorrect** (trap: Wrong model or concept). Rationalization offers plausible-sounding excuses to justify a behavior or decision. The client is not explaining away an action; they are using facts to avoid feelings.

D **Incorrect** (trap: Wrong model or concept). Reaction formation is acting the opposite of an unacceptable feeling, such as being overly cheerful about a resented event. Academic discussion of grief is avoidance through thinking, not opposite behavior.

Takeaway: Intellectualization is retreating into facts and analysis to avoid feelings; denial is refusing to accept that the event happened.

Sources: Vaillant, G. E. (1992). Ego Mechanisms of Defense: A Guide for Clinicians and Researchers. American Psychiatric Press, intellectualization

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.11 Defense mechanisms and how they shape behavior and relationships · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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46. ETH-1562 (rev 2) · Best answer: A

A clinical supervisor in a group practice is asked to sign a former supervisee's licensure verification form. The form lists 24 months of supervision, but no supervision took place during 3 of those months while the supervisor was on medical leave. The supervisee needs the full total to apply this year. What is the **BEST** action by the supervisor?

A. Sign for the months in which supervision occurred
B. Sign for the 24 months of supervision and attach a note about the leave
C. Decline to sign any part of the verification form

Why each option is right or wrong

A **Correct.** Social workers do not take part in dishonesty, fraud or deception (NASW Code 4.04), and those who evaluate others do so fairly and on stated criteria (3.03). Licensing boards rely on supervisors' verification to protect the public. Certifying the 21 months that occurred is accurate and lets the supervisee complete the remaining hours. It beats C because refusing to verify supervision that did happen is unfair to the supervisee and protects no one.

B **Incorrect** (trap: Misreads law or ethics). A note seems transparent, but the signed form still certifies 24 months, 3 of which never happened. A verification to a licensing board must be accurate on its face; a side note does not cure a false certification and still exposes the supervisor to discipline.

C **Incorrect** (trap: Good but not best). Refusing to sign anything penalizes the supervisee for the supervisor's leave and withholds accurate verification of 21 real months. Declining fits only if the supervisor cannot confirm any of the supervision.

Takeaway: Verify supervision accurately: certify the supervision that occurred, no more, and do not withhold verification of what did occur.

Sources: NASW Code of Ethics (2021), 4.04; NASW Code of Ethics (2021), 3.03
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.7 Ethics of supervising and managing · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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47. ETH-1631 (rev 1) · Best answer: C

A school district gives every student the same laptop. It also gives home hotspots and extra tech support to students whose families lack internet service. Which justice principle does the added support for some students reflect?

A. Equality
B. Universal design
C. Equity

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Equality describes the first step, an identical laptop for every student. The hotspots go only to students who lack internet, so the added support departs from equal shares in order to meet need.

B **Incorrect** (trap: Wrong model or concept). Universal design builds products or settings that everyone can use from the start, such as captioned videos for all. Hotspots given only to families without service are a targeted addition, not a universal design feature.

C **Correct.** Equity means distributing resources according to need so that people can reach comparable outcomes; equality means giving everyone the same thing. The identical laptop is equality, and the hotspots and support for students without home internet are matched to need, which is equity. It beats A because identical laptops alone would leave unconnected students unable to use them. The Code calls for expanding opportunity with special regard for disadvantaged groups (NASW Code 6.04(b)), and social work education names equity as a core competency (CSWE, 2022).

Takeaway: Equality gives everyone the same; equity distributes resources by need so that people can reach comparable outcomes.

Sources: NASW Code of Ethics (2021), 6.04(b); Council on Social Work Education (2022). Educational Policy and Accreditation Standards, Competency 3: Engage Anti-Racism, Diversity, Equity, and Inclusion in Practice
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.2 Principles of social, racial, environmental, economic justice · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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48. INT-1608 (rev 1) · Best answer: A

A hospital social worker covering two medical units falls 3 days behind on discharge assessments and progress notes. Two patients on the list have unsafe home situations, and several others need only routine paperwork. The social worker has been staying late, which has not closed the gap. What is the **MOST** appropriate action?

A. Rank the caseload by risk and review it with the supervisor
B. Keep working late until every assessment is current
C. Shorten notes on routine cases to free up time
D. Ask a colleague on another unit to take the two complex patients

Why each option is right or wrong

A **Correct.** Supervisees are responsible for recognizing when workload exceeds capacity and bringing it to supervision with their own view of priorities. Workload and case assignment fall within the supervisor's administrative function (Kadushin & Harkness, 2014). Staying late has failed, and two patients face unsafe discharges, so the worker needs help deciding what comes first and possibly redistributing cases. It beats D because an informal hand-off bypasses the person responsible for assignments and may leave the riskiest patients with someone unfamiliar with them.

B **Incorrect** (trap: Good but not best). Extra hours have already failed to close the gap, and continued overwork raises the risk of errors and burnout. Persistence makes sense for a brief backlog that is shrinking.

C **Incorrect** (trap: Misreads law or ethics). Records should be sufficient and timely enough to support continuity of care (NASW Code 3.04(b)). Cutting notes trades a workload problem for a documentation problem and does nothing for the unsafe discharges.

D **Incorrect** (trap: Hands off too soon). Colleagues may be able to help, but reassigning the two highest-risk patients informally bypasses the supervisor who controls assignments. The colleague may lack context, and the coverage gap stays unresolved.

Takeaway: When workload exceeds capacity, supervisees rank cases by risk and take the problem to supervision rather than overworking, cutting records or handing off informally.

Sources: Kadushin & Harkness (2014). Supervision in Social Work (5th ed.). Columbia University Press, Ch. 2: Administrative Supervision; NASW Code of Ethics (2021), 3.04(b)
Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.1 Supervisee's role: learning needs, self-assessment, prioritizing · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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49. ASM-1526 (rev 1) · Best answer: A

The adult children of an 81-year-old who lives alone ask a senior center social worker for help. In 4 months, the parent has had two minor crashes, and new scrapes keep appearing on the car. The parent says both crashes were other drivers' fault and refuses to stop driving. One adult child wants to hide the car keys. Which recommendation is **BEST**?

A. Arrange a professional evaluation of the parent's driving ability
B. Suggest the parent limit driving to daylight hours near home
C. Support the adult children in removing access to the car
D. Ask the parent's physician to report the parent to the licensing agency

Why each option is right or wrong

A **Correct.** Driving often stands for independence in late life, so the response must protect both safety and self-determination (NASW Code 1.02). Repeated crashes and unexplained damage are red flags that call for an objective assessment, which can include clinical screening and a driver rehabilitation specialist's on-road evaluation (Pomidor, 2019). It beats B because self-imposed limits make sense once the parent's abilities are known; two crashes in 4 months make an unassessed compromise risky, and an evaluation gives the family a neutral basis for any next step.

B **Incorrect** (trap: Good but not best). Limiting driving to daylight and familiar roads is a common, useful adjustment for older drivers whose skills are known. Recommending it now guesses at the problem, may not address whatever is causing the crashes, and leaves the family's dispute unresolved.

C **Incorrect** (trap: Overrides the client's choice). Taking away access to the car overrides a competent adult's choice before anyone has assessed whether the parent can drive safely, and it can damage trust. Restricting driving may become necessary if an evaluation shows the parent is unsafe.

D **Incorrect** (trap: Skips assessment). A report to the licensing agency can be appropriate after an evaluation shows unsafe driving and the person keeps driving; reporting rules vary by state. Requesting one now skips the assessment and casts the physician as an enforcer before the facts are known.

Takeaway: When an older adult's driving safety is in question, seek an objective driving evaluation before restricting a competent person's independence.

Sources: NASW Code of Ethics (2021), 1.02; Pomidor, A. (Ed.) (2019). Clinician's Guide to Assessing and Counseling Older Drivers (4th ed.). American Geriatrics Society and National Highway Traffic Safety Administration, Ch. 2 red flags for unsafe driving; Ch. 5 driver rehabilitation
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.18 Aging's impact on individuals and family systems · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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50. INT-1615 (rev 1) · Best answer: C

An agency committee lists risk areas such as consent and boundaries. It compares daily practice with written rules, rates each risk and plans fixes. What is this process called?

A. Utilization review
B. Root cause analysis
C. Ethics audit
D. Peer review

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Utilization review checks whether services are necessary and appropriate, often for payers. It examines service use, not risks across areas such as confidentiality, consent and boundaries.

B **Incorrect** (trap: Wrong model or concept). Root cause analysis examines one adverse event to find the system failures behind it. The process described is a broad, proactive review across practice areas rather than an investigation of a single event.

C **Correct.** Reamer's ethics audit is a risk-management tool. A committee identifies ethics-related risk areas such as confidentiality, informed consent and boundaries, gathers data on actual practice, rates each area from no risk to high risk, and writes action plans starting with the highest risks (Reamer, 2001). It beats B because root cause analysis investigates one adverse event after it happens, while an audit reviews practices across areas before harm occurs.

D **Incorrect** (trap: Wrong model or concept). Peer review has colleagues assess the quality of individual practitioners' work or records. It does not rate agency-wide risk areas or produce an agency action plan.

Takeaway: An ethics audit proactively compares practice with policy across risk areas, rates each risk, and sets action plans, starting with high-risk areas.

Sources: Reamer, F. G. (2001). The Social Work Ethics Audit: A Risk Management Tool. NASW Press, Audit steps and risk ratings

Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.4 Developing and evaluating risk-reducing policies and procedures · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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51. ASM-1052 (rev 1) · Best answer: D

A client in private practice has major depressive disorder. After 12 weeks of treatment, the client's mood, energy and interest have largely returned. However, the client still takes over an hour to fall asleep most nights, as for the past 5 months, and feels exhausted at work. The prescriber expects sleep to recover as the depression lifts. What is the **MOST** appropriate recommendation?

A. Wait for sleep to improve as the depression resolves
B. Ask the prescriber about adding a sleep medication
C. Give the client a handout on sleep hygiene
D. Add CBT for insomnia to the client's care plan

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Waiting is reasonable early in depression treatment, when poor sleep is part of the episode. Here mood and interest have largely recovered while sleep has not, so the insomnia now needs direct attention.

B **Incorrect** (trap: Right action, wrong time). Medication is an option the prescriber may weigh, but guidelines place CBT-I first for chronic insomnia because its gains last after treatment ends. Medication fits if CBT-I is unavailable or does not help.

C **Incorrect** (trap: Good but not best). Sleep hygiene advice is useful, but used alone it is not an effective treatment for chronic insomnia (Edinger et al., 2021). It is one piece of CBT-I, which adds stimulus control, sleep restriction and cognitive work.

D **Correct.** Insomnia often persists after depression improves, and it can be diagnosed as a co-occurring insomnia disorder when the other condition no longer adequately explains it (DSM-5-TR). Here mood and interest have recovered, yet trouble falling asleep has lasted 5 months and impairs work, so it warrants its own treatment. Cognitive behavioral therapy for insomnia (CBT-I) is the recommended first-line treatment (Qaseem et al., 2016). It beats A because waiting assumes the depression explains a problem that has outlasted it.

Takeaway: Insomnia that outlasts improvement in depression is a co-occurring disorder that needs its own treatment, with CBT-I first.

Sources: DSM-5-TR (APA, 2022), Sleep-Wake Disorders: Insomnia Disorder; Qaseem, A., et al. (2016). Management of chronic insomnia disorder in adults: A clinical practice guideline from the American College of Physicians. Annals of Internal Medicine, 165(2), 125-133, Recommendation 1; Edinger, J. D., et al. (2021). Behavioral and psychological treatments for chronic insomnia disorder in adults: An American Academy of Sleep Medicine clinical practice guideline. Journal of Clinical Sleep Medicine, 17(2), 255-262, Recommendations: sleep hygiene

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.6 Comorbid (co-occurring) disorders and conditions · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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52. ETH-1594 (rev 2) · Best answer: A

Which statement about implicit bias is **MOST** accurate?

A. It can shape judgments even in people with egalitarian beliefs

B. It is prejudice that people knowingly hide from others

C. It can be lastingly reduced by a brief training session

Why each option is right or wrong

A **Correct.** Implicit bias refers to associations that operate outside conscious awareness and can influence judgment and behavior, even in people who sincerely hold egalitarian views (Greenwald & Banaji, 1995). This is why good intentions alone do not prevent biased decisions and why structured, reviewed processes matter. It beats C because a large test of brief interventions found that shifts in implicit preferences faded within hours to days (Lai et al., 2016).

B **Incorrect** (trap: Wrong model or concept). Prejudice that a person is aware of and conceals is explicit bias. Implicit bias operates outside awareness, so people may not know they hold it.

C **Incorrect** (trap: Wrong model or concept). Brief interventions can shift measured implicit preferences right away, but research found the effects faded within days. Lasting change relies on sustained effort and structured decision processes.

Takeaway: Implicit bias operates outside awareness and can affect decisions despite good intentions; brief training does not durably remove it.

Sources: Greenwald, A. G., & Banaji, M. R. (1995). Implicit social cognition: Attitudes, self-esteem, and stereotypes. Psychological Review, 102(1), 4-27, whole article; Lai, C. K., et al. (2016). Reducing implicit racial preferences: II. Intervention effectiveness across time. Journal of Experimental Psychology: General, 145(8), 1001-1016, whole article

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** MOST · **Cognitive** recall · **Difficulty** 1 of 3

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53. ETH-1536 (rev 2) · Best answer: C

At a community mental health agency, a client became enraged when a housing letter was delayed and told the social worker, 'I know where you park. You'll regret this.' The next day the client left a similar voicemail. When the supervisor called the client to discuss the threats, the client repeated them. The agency has reported the threats under its safety policy. What is the **BEST** way to handle the clinical relationship?

A. Continue sessions in a room close to security staff
B. Address the threats with the client at the next session
C. End services and arrange care with another provider
D. Close the case by letter and decline further contact

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Security precautions are reasonable while a transition is arranged. Continuing the relationship as before, though, keeps the worker exposed to a client who has repeated the threats even after the supervisor raised them, and the relationship itself remains damaged.

B **Incorrect** (trap: Ignores risk). Talking through anger can repair minor ruptures, and a safe closing contact may acknowledge what happened. Here the supervisor already tried to discuss the threats and the client repeated them, so another attempt in session is unlikely to help and puts the worker at risk.

C **Correct.** Social workers avoid abandoning clients, withdraw services abruptly only in unusual circumstances while minimizing harm, and help arrange continued services (NASW Code 1.17(b)). Specific threats against the worker, repeated even after the supervisor addressed them, compromise both the worker's safety and the working relationship. Ending this relationship while arranging care with another provider protects the worker and keeps the client served. It beats A and B because added security or another talk leaves the client with a worker the client keeps threatening, after a direct attempt to address the threats has failed.

D **Incorrect** (trap: Misreads law or ethics). Ending contact by letter alone, with no referral, leaves a client in need without services, which is abandonment. Even when termination is justified, the social worker helps arrange continued care.

Takeaway: Threats against the worker can justify ending services, but termination still includes arranging care elsewhere to avoid abandonment.

Sources: NASW Code of Ethics (2021), 1.17(b)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

54. ASM-1095 (rev 1) · Best answer: A

A 10-year-old client in a child advocacy center program disclosed last month that a parent's partner sexually abused them. The partner then moved out, and the family lost much of its income. Today the client says, 'I made it up,' while looking at the floor, and adds that the parent cries every night. What is the **MOST** appropriate action for the social worker?

A. Tell the investigating caseworker about the recantation
B. Accept the recantation and note the report was likely false
C. Question the client in detail to learn which account is true

Why each option is right or wrong

A **Correct.** Recantation occurs in a meaningful share of substantiated child sexual abuse cases, especially when the child is young, the alleged abuser is a parent figure and the nonoffending caregiver is unsupportive or distressed. It does not by itself mean the first report was false (Malloy, Lyon & Quas, 2007). The partner's removal, lost income and the parent's distress are typical pressures. The caseworker needs this to protect the child. It beats C because probing for the 'true' account belongs to trained forensic interviewers, and repeated questioning can distress the child and taint later statements.

B **Incorrect** (trap: Ignores risk). Taking a child's words seriously is important, but research on recantation shows it often follows family pressure rather than a false report. Treating it as proof of fabrication could leave the child unprotected.

C **Incorrect** (trap: Outside the social work role). Wanting clarity is understandable. Detailed questioning by the social worker risks leading the child, adds pressure and can compromise the investigation; forensic interviews belong to trained staff.

Takeaway: A child's recantation under family pressure does not mean abuse did not occur; share it with investigators rather than settling the question yourself.

Sources: Malloy, L. C., Lyon, T. D., & Quas, J. A. (2007). Filial dependency and recantation of child sexual abuse allegations. Journal of the American Academy of Child and Adolescent Psychiatry, 46(2), 162-170, whole article
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.4 Physical, sexual, psychological abuse and neglect: signs, dynamics, impact · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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55. INT-1025 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

At a senior center, a 74-year-old client tells the social worker that most nights they lie awake in bed for an hour or more, watching TV and checking the news on a tablet until sleep comes. A physician found no medical or medication cause. Which self-care skill is **BEST** for the social worker to teach?

A. Swap the TV and tablet for reading in bed until drowsy
B. Get up when unable to sleep and return when sleepy
C. Go to bed earlier to allow more total time for sleep

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Reading is calmer than screens, so this feels like progress. Stimulus control, though, asks that the bed be kept for sleep (and sex) only. Reading in bed still pairs the bed with being awake.

B **Correct.** Stimulus control is a recommended behavioral treatment for chronic insomnia: go to bed only when sleepy, get out of bed when unable to sleep, and keep the bed for sleep rather than screens or reading (Edinger et al., 2021). Lying awake with the TV and a tablet has taught this client that bed is a place to be awake. Getting up and returning only when sleepy rebuilds the link between bed and sleep. It beats A because reading in bed is gentler than screens but still pairs the bed with wakefulness.

C **Incorrect** (trap: Misuses the technique). More time in bed usually means more time lying awake, which deepens insomnia. Stimulus control asks people to go to bed only when sleepy, not earlier in the hope of banking extra hours.

Takeaway: For chronic insomnia, teach stimulus control: go to bed only when sleepy, keep the bed for sleep, and get up when unable to sleep.

Sources: Edinger, J. D., et al. (2021). Behavioral and psychological treatments for chronic insomnia disorder in adults: an American Academy of Sleep Medicine clinical practice guideline. Journal of Clinical Sleep Medicine, 17(2), 255-262, Recommendation 3: stimulus control

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.8 Teaching clients coping and self-care skills · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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56. ETH-1077 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

A social worker conducts interviews for an IRB-approved study of job-training outcomes. A question about past jobs leads one participant to describe a workplace assault and become very distressed. The social worker stops the interview, stays with the participant until calm, and, with consent, links them to a counseling service. What should the social worker do **NEXT**?

A. Call the participant's emergency contact about the distress
B. Report the event to the IRB promptly
C. Describe the event at the IRB's annual continuing review

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). The participant is a calm adult who has accepted a referral. Calling an emergency contact would reveal study participation and private details without consent, breaking the confidentiality promised under 5.02(m).

B **Correct.** Participant protection comes first, and that step is done: the interview stopped and support was arranged (NASW Code 5.02(j), 5.02(k)). An unexpected, study-related harm that suggests more risk than the protocol anticipated must be reported promptly, so the IRB can decide whether the questions, consent form or procedures need to change (45 CFR 46.108(a)(4)(i)). B beats C because waiting months for continuing review leaves other participants exposed to the same question in the meantime.

C **Incorrect** (trap: Right action, wrong time). Continuing review is the place for routine updates, not for an unanticipated problem involving risk. IRBs must have procedures for prompt reporting, so this timing is wrong.

Takeaway: After protecting a research participant, report unanticipated study-related harm to the IRB promptly, not at the next scheduled review.

Sources: NASW Code of Ethics (2021), 5.02(j); NASW Code of Ethics (2021), 5.02(k); NASW Code of Ethics (2021), 5.02(m); 45 CFR Part 46 (Common Rule), 45 CFR 46.108(a)(4)(i)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.11 Research ethics: IRBs, human subjects protections, consent · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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57. ASM-1639 (rev 3) · Best answer: B

A couple with a 4-month-old, their first child, comes for a first counseling session. In the opening minutes, they say they argue about chores and sleep and fear they are 'falling out of love.' Then both turn to the social worker, waiting for a verdict. What should the social worker do **FIRST**?

A. Help the couple make a plan for sharing chores and night feedings
B. Ask each partner how life together has changed since the birth
C. Explain that satisfaction often dips with a new baby
D. Refer the couple to a specialized couples therapy program

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A fair plan for chores and night feedings is a sensible later step. Building it now assumes the arguments are only about tasks before the social worker knows how each partner sees the problem.

B **Correct.** At a first session, the first task is to hear how each partner experiences the change: sleep, roles, closeness and support since the birth. Exploring both views engages each partner and gives the social worker the facts needed to judge whether this is the common strain of new parenthood or something more. Relationship quality often declines after a first child (Doss et al., 2009), but saying so before hearing them can sound like a verdict that sidesteps their fear. It beats C because that reassurance fits only once the social worker knows the strain matches the common pattern.

C **Incorrect** (trap: Right action, wrong time). This is often true and can bring relief, and it may well fit this couple. Offered in the opening minutes, before either partner has described what is happening, it can sound like a verdict that brushes past their fear.

D **Incorrect** (trap: Hands off too soon). Specialized couples therapy may help if problems prove severe, involve violence or exceed the social worker's skills. Referring at a first session hands off concerns that have not yet been assessed and may reflect a common transition.

Takeaway: Hear each partner's view of a family transition before teaching what is typical or planning changes.

Sources: Doss, B. D., Rhoades, G. K., Stanley, S. M., & Markman, H. J. (2009). The effect of the transition to parenthood on relationship quality: An 8-year prospective study. Journal of Personality and Social Psychology, 96(3), 601-619, whole article
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.3 Relationship dynamics and development: families, couples, groups, polyamory · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 3 of 3

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58. ASM-1657 (rev 1) · Best answer: D

Two days after an ice storm knocked out power, a social worker makes a scheduled home visit. A gasoline generator is running in the attached garage. The parent and two children report headaches and nausea, and the infant seems unusually sleepy. The parent says everyone feels better after time outside. What should the social worker do **FIRST**?

A. Ask each family member about fever, cough and other flu signs
B. Urge the parent to take the infant to the pediatrician today
C. Have the parent move the generator outdoors, away from windows
D. Get the family out of the house and call 911

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). Asking about flu signs would fit if the illness had no clear pattern. Here several people share symptoms that ease outdoors while a generator runs in the garage. More questions delay leaving a possibly toxic home.

B **Incorrect** (trap: Ignores risk). Same-day care for the infant is too slow and leaves everyone else in the house. Possible carbon monoxide poisoning with a drowsy infant needs immediate removal from the source and emergency care.

C **Incorrect** (trap: Right action, wrong time). Moving the generator outside is necessary, but it is a later step. Gas already in the house keeps harming the family, and the drowsy infant needs emergency care now. Fire or emergency crews can then clear the home.

D **Correct.** Symptoms shared by several people in one home, easing away from the house, while a generator runs indoors point to carbon monoxide poisoning. The Centers for Disease Control and Prevention (CDC) warns never to run a generator inside a home or garage, even with doors open, and notes that infants are at higher risk; poisoning can cause fainting and death, and treatment calls for oxygen. Leaving the house stops the exposure, and emergency services can treat the infant and clear the home. It beats C because moving the generator does not remove gas already inside or treat people who are already ill.

Takeaway: Shared headaches, nausea or drowsiness in a home with fuel-burning equipment suggest carbon monoxide; get everyone out and call emergency services.

Sources: Centers for Disease Control and Prevention (CDC). Carbon Monoxide Poisoning Basics (2024), symptoms; higher-risk groups; generator safety; Centers for Disease Control and Prevention (CDC). Clinical Guidance for Carbon Monoxide Poisoning Following Disasters and Severe Weather (2024), recognition and oxygen treatment

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.22 Urbanization, globalization, environmental hazards, climate change impacts · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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59. ETH-1121 (rev 1) · Best answer: D

In 2021, the NASW Code of Ethics added a sentence on caring for one's own well-being. Which ethical principle now holds it?

A. Competence
B. Service
C. Dignity and worth of the person
D. Integrity

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Competence covers practicing within one's areas of expertise and building skill. Self-care supports competent work, but the 2021 sentence was added to the Integrity principle.

B **Incorrect** (trap: Wrong model or concept). Service concerns putting helping others above self-interest and offering some pro bono work. The self-care sentence does not appear under this principle.

C **Incorrect** (trap: Wrong model or concept). Dignity and worth concerns respect for each person and support for clients' self-determination. The 2021 self-care language was added elsewhere.

D **Correct.** The 2021 revision added self-care language in two places: a Purpose paragraph calling professional self-care "paramount for competent and ethical social work practice," and a sentence in the Integrity principle saying social workers should care for themselves professionally and personally (NASW Code, Ethical Principles; Purpose). Placing it under integrity frames self-care as part of trustworthy conduct. D beats A because competence concerns practicing within one's expertise and enhancing it; self-care supports competence, but the added principle sentence sits under integrity.

Takeaway: Since 2021, the Code treats self-care as an ethical matter, in the Purpose section and under the Integrity principle.

Sources: NASW Code of Ethics (2021), Ethical Principles: Integrity; NASW Code of Ethics (2021), Purpose (self-care paragraph)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.7 Principles of self-care for the social worker · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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60. ASM-1165 (rev 1) · Best answer: C

An 82-year-old client has taken lorazepam every night for 10 years. After two falls this month, the client's adult child tells the home-visit social worker they plan to throw the pills away tonight. The client says they would like to stop the pills but fear they will not sleep. What is the **BEST** action?

A. Support removing the pills tonight to prevent another fall
B. Help the family make the home safer against falls
C. Arrange a prescriber review to plan a gradual taper
D. Respect the client's fear and keep the routine as is

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). The goal is right, but stopping a nightly benzodiazepine abruptly after 10 years can cause severe withdrawal, including seizures. Removal should happen through a supervised taper, not overnight.

B **Incorrect** (trap: Good but not best). Home safety changes reduce fall risk and can be part of the plan. They do not address the sedating medication that is a likely cause of the falls.

C **Correct.** Benzodiazepines are on the Beers list of drugs to avoid in older adults because they raise the risk of falls, confusion and fractures (American Geriatrics Society, 2023). After long daily use, stopping suddenly can cause withdrawal, including seizures, so the FDA advises a gradual, individualized taper (FDA, 2020). A

prescriber-led taper supports the client's own wish to stop while addressing sleep worries. It beats B because fall-proofing helps but leaves in place the drug most likely driving the falls.

D **Incorrect** (trap: Good but not best). The client's fear deserves attention, but the client has said they want to stop. Keeping the routine leaves a high fall risk in place when a safe taper could honor the client's goal.

Takeaway: Older adults on long-term benzodiazepines who fall need prescriber-guided tapering; abrupt stopping risks dangerous withdrawal.

Sources: American Geriatrics Society Beers Criteria Update Expert Panel (2023). American Geriatrics Society 2023 updated AGS Beers Criteria for potentially inappropriate medication use in older adults. Journal of the American Geriatrics Society, 71(7), 2052-2081, Benzodiazepines; FDA (U.S. Food and Drug Administration) (2020). FDA requiring Boxed Warning updated to improve safe use of benzodiazepine drug class. Drug Safety Communication, September 23, 2020: withdrawal reactions and tapering
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.5 Prescription, OTC, alternative medications: uses and side effects · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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61. INT-1101 (rev 1) · Best answer: A

An outpatient client stopped drinking 4 months ago. In session, the client reports having three beers at a cousin's wedding last weekend and none since. The client says, 'I blew it. I'm just someone who can't stay sober, so why keep trying?' What should the social worker do **FIRST**?

A. Help the client see the slip as a lapse to learn from
B. Recommend moving the client to a more intensive level of care
C. Review the wedding situation to plan for future high-risk events

Why each option is right or wrong

A **Correct.** In relapse prevention, the abstinence violation effect is the guilt and self-blame after a slip, when a person concludes they are a failure who cannot change. That belief can turn a single lapse into a full relapse (Marlatt & Gordon, 1985). The client's words, 'someone who can't stay sober,' show this effect, so the first task is to reframe the drinking as a lapse to learn from. It beats C because reviewing the high-risk situation is the right next step, but a client who has decided trying is pointless will not engage in planning until that belief eases.

B **Incorrect** (trap: Hands off too soon). One brief lapse after 4 months of abstinence, with no drinking since, does not show that outpatient care has failed. Stepping up the level of care would fit if drinking resumed and continued despite a revised plan.

C **Incorrect** (trap: Right action, wrong time). Analyzing the wedding as a high-risk situation and planning coping responses is a core relapse prevention step. It works best after the client's all-or-nothing self-blame eases; otherwise the client may see no point in planning.

Takeaway: After a slip, address the abstinence violation effect first: frame the lapse as a learning event, then plan for the high-risk situation.

Sources: Marlatt, G. A., & Gordon, J. R. (Eds.). (1985). Relapse Prevention: Maintenance Strategies in the Treatment of Addictive Behaviors. Guilford Press, abstinence violation effect; lapse versus relapse
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.17 Intervening with addiction · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

Section 2

62. INT-1188 (rev 2) · Best answer: B

A client with a trauma history yelled profanities at the front desk staff after a long wait. In the next session, the client explained that crowded waiting rooms feel unsafe, and the clinic began offering a quiet room for waiting. This week, after a 5-minute delay in the quiet room, the client swore at a receptionist again. What should the social worker do **NEXT**?

A. Explore again what waiting brings up for the client
B. Set clear conduct expectations and state what happens next time
C. Arrange for the client to skip the waiting area entirely

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Exploring the trigger was the right first response and was done after the first incident, leading to the quiet room. Repeating it without addressing the behavior signals that mistreating staff carries no expectation.

B **Correct.** Trauma-informed care pairs understanding with clear, predictable expectations, because physical and psychological safety must hold for staff as well as clients (SAMHSA, 2014). The clinic has already explored the trigger and made an accommodation, yet the client swore at staff again while waiting in the quiet room. Calmly stating the expected conduct and the specific consequence of another incident, while affirming the client's place in services, is the limit the situation now needs. It beats C because a further accommodation responds to the trigger again but leaves the behavior toward staff unaddressed.

C **Incorrect** (trap: Good but not best). More accommodation can be part of the plan, but on its own it treats the outburst only as a scheduling problem. Staff safety and a clear expectation for the client's conduct still need to be stated.

Takeaway: Trauma-informed limits combine understanding with clear expectations; after an accommodation fails, state the limit rather than only accommodating again.

Sources: SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), Key principles: safety; trustworthiness and transparency
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.4 Setting limits and boundaries with clients · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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63. ETH-1532 (rev 2) · Best answer: A

A client who entered an inpatient psychiatric unit voluntarily for depression with psychotic features asks to leave in 2 days, sooner than the team advises, to keep a new job. The psychosis has cleared, and the psychiatrist's assessment this morning found no suicidal thoughts or intent. The team asks the social worker to help. What is the **MOST** appropriate action?

A. Plan the client's discharge with follow-up care arranged
B. Repeat a full suicide risk assessment before planning
C. Ask the psychiatrist to consider a hold to finish treatment

Why each option is right or wrong

A **Correct.** A voluntary patient keeps the right to withdraw consent (NASW Code 1.03(a)), and the Code permits limiting self-determination when a client's actions pose a serious, foreseeable and imminent risk (1.02), which this morning's assessment did not find. The psychosis has cleared and this morning's assessment found no suicidal thoughts or intent, so the client's choice to protect the job stands. The social worker makes the earlier discharge as safe as possible: outpatient appointments, medication access and a crisis plan. It beats B because the psychiatrist just assessed risk; repeating it adds delay without new information, though new warning signs would call for reassessment.

B **Incorrect** (trap: Good but not best). Reassessment is essential when new warning signs appear or time has passed. Here a full assessment was done this morning with no concerning findings, so repeating it now delays planning without adding information.

C **Incorrect** (trap: Overrides the client's choice). A hold is meant for imminent danger, not for completing a recommended course of treatment. Using it to override a low-risk client's choice would violate self-determination and could damage the client's trust in future care.

Takeaway: A voluntary patient without imminent risk may leave earlier than advised; help make that discharge as safe as possible.

Sources: NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.03(a)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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64. ETH-1585 (rev 1) · Best answer: B

A social worker leads a support group for older adult caregivers at a senior center. In the third meeting, white members do most of the talking, and one member twice talks over a Black member to correct that member's story about a clinic visit. The Black member and two Latino members have gone quiet. What is the **MOST** appropriate action by the social worker?

A. Check in privately with the quieter members after the meeting
B. Name the pattern and set a norm that each member is heard
C. Let the group develop its own norms over the next sessions

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Private check-ins show care and may help later. Used alone, they ask the members being silenced to manage the problem and leave the dominant pattern in place for the next meeting.

B **Correct.** Group leaders shape norms, and early sessions are when norms take hold (Yalom & Leszcz, 2020). A pattern in which members of color are talked over and fall silent reproduces racial power differences inside the group. Naming the pattern and setting a norm that each member is heard addresses it openly and protects equal voice (NASW Code 1.05(b)). It beats A because private check-ins may comfort the quieter members but place the burden on them and leave the group's norm unchanged.

C **Incorrect** (trap: Wrong model or concept). Groups do develop norms, but leaders actively shape them, especially early. Waiting lets an unequal pattern harden and signals that talking over members of color is acceptable.

Takeaway: When group interaction reproduces racial power differences, the leader names the pattern and sets norms for equal voice.

Sources: Yalom & Leszcz (2020). The Theory and Practice of Group Psychotherapy (6th ed.), the therapist's basic tasks: culture building and norms; NASW Code of Ethics (2021), 1.05(b)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.5 Power differentials, racial privilege, internalized inferiority/superiority · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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65. INT-1538 (rev 1) · Best answer: B

During a home visit for a job placement program, a client chats about weekend plans and mentions that a sibling who lives in another state is undocumented. The client's service plan centers on finding warehouse work. Which way of handling the sibling's status in the case record is **BEST**?

A. Record the sibling's status to keep the social history complete
B. Leave the sibling's status out of the case record
C. Ask the client's permission before writing the status down

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Completeness is not the standard; relevance is. A relative's legal status has nothing to do with finding warehouse work, and recording it adds risk to a third party without improving services.

B **Correct.** Documentation should protect privacy and include only information directly relevant to the delivery of services (NASW Code 3.04(c)). A sibling's immigration status, mentioned in passing by a client whose plan centers on job placement, does not bear on the services, and recording it could expose a third party to harm if records are later released. It beats C because asking permission treats the question as one of consent, but relevance decides what goes into the record; even with permission, unrelated third-party details do not belong there.

C **Incorrect** (trap: Good but not best). Consent matters for disclosure, but it does not make unrelated information relevant. The record holds what services require, and this detail fails that test whether or not the client agrees.

Takeaway: Record only information relevant to services; consent does not make unrelated details, especially about third parties, belong in the record.

Sources: NASW Code of Ethics (2021), 3.04(c)
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.18 Case management: recording, documentation, case presentation · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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66. ASM-1080 (rev 2) · Best answer: C

A client with borderline personality disorder has had near-daily passive thoughts of death for years and attends weekly DBT. After a breakup this week, the client says the thoughts feel different. In the risk assessment that follows, the client says they bought two large bottles of acetaminophen yesterday and will 'probably' take them this weekend. The client cannot name anything that would keep them safe. What should the social worker do **NEXT**?

A. Continue DBT skills, noting the thoughts are long-standing
B. Complete a safety plan and add a midweek session
C. Have the client evaluated for hospitalization today
D. Explore what the breakup means to the client

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Chronic passive thoughts can be managed in ongoing DBT. Treating this as baseline ignores the new facts, bought pills, a time frame and no way to stay safe, which signal a sharp rise in acute risk.

B **Incorrect** (trap: Good but not best). A safety plan with more frequent contact suits intermediate acute risk, when a client can keep themselves safe. This client has a method in hand, a likely timeline and cannot name anything that would keep them safe, so outpatient steps are not enough today.

C **Correct.** Suicide risk is best judged on two dimensions: chronic baseline risk and current acute risk (Wortzel et al., 2014). Years of passive thoughts set a high baseline, but the new facts, bought pills, a likely time frame and inability to stay safe, mark high acute risk. VA's risk stratification guidance says high acute risk typically calls for hospitalization with observation until then (Rocky Mountain MIRECC, 2024). It beats B because a safety plan and extra session fit someone who can maintain safety alone; this client cannot.

D **Incorrect** (trap: Right action, wrong time). The breakup's meaning matters and will be part of treatment. With a stated plan, acquired means and a timeline already assessed, exploration now delays the protective step the risk requires.

Takeaway: In chronically suicidal clients, rate acute risk separately; a new plan, acquired means and inability to stay safe call for hospital-level evaluation.

Sources: Wortzel, H. S., Homaifar, B., Matarazzo, B., & Brenner, L. A. (2014). Therapeutic risk management of the suicidal patient: Stratifying risk in terms of severity and temporality. Journal of Psychiatric Practice, 20(1), 63-67, whole article; Rocky Mountain MIRECC for Suicide Prevention (2024). Therapeutic Risk Management: Risk Stratification Table. U.S. Department of Veterans Affairs, Acute risk: high
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.1 Assessing risk of harm to self or others · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

67. INT-0001 (rev 1) · Best answer: A

In the fifth session, a client who has been engaged suddenly says, 'You're like everyone else. You don't really listen.' The comment follows the social worker's suggestion to try a new coping exercise. What should the social worker do **FIRST** to repair the working relationship?

A. Invite the client to say more about feeling unheard
B. Explain the reasons for suggesting the coping exercise
C. Reassure the client that the social worker is on their side
D. Suggest a transfer to a social worker who is a better fit

Why each option is right or wrong

A **Correct.** A rupture in the alliance is best repaired by meeting it with curiosity rather than defense. Inviting the client to describe the experience of not being heard validates the feedback and gives the social worker information to adjust (Safran & Muran, 2000). The client's reaction to a suggestion is also useful clinical material. It beats C because reassurance tells the client how to feel about the relationship instead of exploring what went wrong, so the rupture stays unaddressed.

B **Incorrect** (trap: Too directive). Explaining the rationale defends the suggestion and implies the client misunderstood. It can confirm the client's sense of not being heard. The rationale can be revisited once the client's experience is understood.

C **Incorrect** (trap: Good but not best). Reassurance is well-meant but skips the client's actual complaint. Saying the social worker cares does not show listening; exploring the client's experience does.

D **Incorrect** (trap: Hands off too soon). Offering a transfer at the first sign of tension treats a normal, repairable rupture as a failed relationship. Working through ruptures often strengthens the alliance.

Takeaway: Meet an alliance rupture with nondefensive curiosity: invite the client's experience before explaining or reassuring.

Sources: Safran, J. D., & Muran, J. C. (2000). Negotiating the Therapeutic Alliance: A Relational Treatment Guide. Guilford Press, rupture resolution
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.2 Building and sustaining the helping relationship (empathy, rapport, acceptance) · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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68. ASM-1578 (rev 3) · Best answer: D

During a video intake for depression, the social worker reaches the trauma history section, which asks about sexual abuse and assault. The client is at home. When the client's partner comes up, the client glances off-screen, lowers their voice and begins answering in single words. What should the social worker do **FIRST**?

A. Ask about unwanted sexual experiences in specific terms
B. Skip the trauma section unless the client raises it later
C. Email the trauma questionnaire for the client to finish later
D. Ask whether the client is alone and able to talk freely

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Behaviorally specific wording is the right way to ask about sexual trauma, and it is the next step once privacy is confirmed. Asking now could expose the client to danger if the partner is listening.

B **Incorrect** (trap: Good but not best). Dropping the section leaves out information that shapes diagnosis and treatment, and it signals the topic is off-limits. Trauma questions should be postponed only until the client can speak safely, not abandoned.

C **Incorrect** (trap: Ignores risk). A form sent to a shared home email or device could be seen by the partner, adding risk rather than reducing it. Written screens can help, but only through a channel the client controls.

D **Correct.** Trauma-informed screening calls for a private, safe setting before sensitive questions, because safety comes first (SAMHSA TIP 57; SAMHSA, 2014). The client's glance off-screen and lowered voice at the partner's name suggest someone may be listening, possibly someone the client fears. A brief yes-or-no check lets the client signal danger without explaining. It beats A because specific questions are the right technique only after privacy is confirmed; asked now, they could put the client at risk.

Takeaway: Before asking about sexual trauma, confirm the client can speak privately and safely, especially by video from home.

Sources: SAMHSA TIP 57 (2014), Part 1, Ch. 4: Screening and Assessment; SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), Key principles: Safety
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.7 Eliciting sensitive or confidential information (substance use, sexual abuse) · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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69. ETH-1503 (rev 1) · Best answer: C

In an intensive outpatient program for alcohol use disorder, a client says nobody really stays sober and asks the social worker, 'Have you ever been through this?' The social worker has been in recovery for 12 years, and program policy allows staff to share their recovery status. What is the **BEST** response?

A. Describe the social worker's own path to sobriety in detail
B. Explain that the social worker's personal life stays private
C. Confirm it briefly, then turn back to the client's doubts

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). Some disclosure can help here, but a detailed story shifts the session to the worker and can burden the client or invite comparison. Disclosure should be short enough that the focus quickly returns to the client's own recovery.

B **Incorrect** (trap: Good but not best). Keeping personal information private is a sound default and is often correct. In this program, though, brief recovery disclosure is allowed, and a flat refusal misses a chance to answer the client's hopelessness directly.

C **Correct.** Self-disclosure is appropriate when it is brief, relevant and offered for the client's benefit, with the focus returned to the client (Hepworth et al.). The clinical issue is the client's hopelessness about recovery, the program permits recovery disclosure, and a short confirmation can show that sobriety is possible. Turning back to

the client's doubts keeps the session theirs. It beats B because privacy is a reasonable default, but here a brief, permitted disclosure speaks directly to the hopelessness the client raised (Reamer, 2020).

Takeaway: Self-disclose only briefly, for the client's benefit, and return the focus to the client right away.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Ch. 5: authenticity and self-disclosure; Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, self-disclosure

Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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70. INT-1071 (rev 2) · Best answer: C

A crisis stabilization program offers up to 6 sessions. A client seen after a sudden job loss and panic attacks is, by session 5, sleeping again, applying for jobs, and using a coping plan. The client now asks to work on feelings of worthlessness that go back to childhood. What is the **MOST** appropriate step?

A. Extend crisis sessions to address the childhood pattern

B. Close the case now that functioning has returned

C. Refer for ongoing therapy and set a follow-up contact

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Long-standing feelings of worthlessness call for ongoing therapy, not crisis sessions stretched past their purpose. Extending would also delay the client's link to a provider who can offer the longer work.

B **Incorrect** (trap: Good but not best). Functioning has returned, but closing without a referral ignores the client's request for help with a long-standing problem. Crisis work ends with linkage and follow-up, not simply discharge.

C **Correct.** Crisis intervention is brief and present-focused: it aims to restore at least pre-crisis functioning, then link the person to longer-term help for deeper issues and plan follow-up (Roberts, 2005). This client is sleeping, job hunting and coping, so the crisis goal is met, and the new request is long-standing work. It beats A because the client's wish is valid, but stretching a crisis service into open-ended therapy blurs its purpose; a referral gives the childhood pattern the time it needs, and follow-up closes the crisis episode (Roberts & Ottens, 2005).

Takeaway: Crisis intervention restores functioning, then links the client to longer-term services for underlying issues, with follow-up.

Sources: Roberts, A. R. (Ed.). (2005). Crisis Intervention Handbook: Assessment, Treatment, and Research (3rd ed.). Oxford University Press, goals of crisis intervention; referral after stabilization; Roberts, A. R., & Ottens, A. J. (2005). The seven-stage crisis intervention model: A road map to goal attainment, problem solving, and crisis resolution. Brief Treatment and Crisis Intervention, 5(4), 329-339, Stage 7: follow-up

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.5 Crisis intervention and crisis treatment approaches · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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71. ASM-1113 (rev 1) · Best answer: C

In a residential treatment program, a 14-year-old with years of abuse before foster placement has sudden outbursts, freezes and stares blankly at times, and sleeps poorly. Staff notice that outbursts follow loud voices and room checks at night. The team proposes treating the youth for bipolar disorder. What is the social worker's **BEST** contribution to the team?

A. Support the plan to treat the youth for bipolar disorder
B. Recommend a new placement to reduce the outbursts
C. Point out that outbursts track reminders of earlier trauma

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Mood disorders do occur in youth with trauma histories and deserve evaluation. Agreeing now, though, sets aside the trigger-linked pattern; bipolar disorder is defined by distinct mood episodes, which reactions to specific reminders do not show.

B **Incorrect** (trap: Good but not best). Another move would add a fresh loss for a youth whose history is already marked by disruption. Placement changes tend to worsen trauma reactions; adjusting triggers in the current setting is the better course.

C **Correct.** Complex trauma from repeated childhood abuse often appears as emotional dysregulation, dissociation and reactions to reminders, which can be mistaken for a mood disorder. Here the outbursts follow loud voices and night room checks, plausible reminders of past abuse, and the blank staring suggests dissociation. Bringing this pattern to the team supports a trauma-informed plan and changes to night routines (SAMHSA, 2014). It beats A because bipolar disorder requires distinct mood episodes, not reactions to specific triggers, and a mislabel could lead to unneeded medication (DSM-5-TR).

Takeaway: In youth with abuse histories, trace outbursts to trauma reminders before accepting a mood-disorder explanation.

Sources: SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), Four Rs: realize, recognize, respond, resist re-traumatization; DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Bipolar I Disorder

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.1 Trauma, stress, violence, crisis: signs and impact (historical, complex) · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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72. ETH-1113 (rev 2) · Best answer: A

For years, an employee assistance social worker has responded to workplace deaths at a large company with a required group meeting within 48 hours, in which each employee describes the event and their reactions. While updating training, the social worker reads a systematic review finding that single-session debriefing does not prevent PTSD and may increase symptoms for some people. What is the **BEST** response?

A. Switch to psychological first aid with follow-up
B. Keep the meetings but make attendance voluntary
C. Continue the meetings since employees rate them highly

Why each option is right or wrong

A **Correct.** Social workers keep up with emerging knowledge and base practice on recognized, empirically based knowledge (NASW Code 4.01(b), 4.01(c)). A Cochrane review found single-session individual debriefing did not prevent PTSD and concluded that compulsory debriefing should stop (Rose et al., 2002). Psychological first aid guidance says not to debrief by asking for details and offers practical support instead (Brymer et al., 2006), with follow-up for those who need more. A beats B because voluntary attendance still leaves in place the core element the evidence questions: early, detailed recounting of the event.

B **Incorrect** (trap: Good but not best). Voluntary attendance removes the compulsion the review warned against, but those who attend still recount the event in detail soon after it. The evidence questions the technique itself, not only its required use.

C **Incorrect** (trap: Ignores risk). Satisfaction ratings do not show that a practice prevents harm. When research suggests a practice is ineffective and may increase symptoms, popularity is not a reason to continue it.

Takeaway: When research shows a familiar practice is ineffective or harmful, update to evidence-supported approaches rather than relying on habit or satisfaction.

Sources: NASW Code of Ethics (2021), 4.01(b); NASW Code of Ethics (2021), 4.01(c); Rose, S., Bisson, J., Churchill, R., & Wessely, S. (2002). Psychological debriefing for preventing post traumatic stress disorder (PTSD). Cochrane Database of Systematic Reviews, (2), CD000560, Authors' conclusions; Brymer, M., et al. (2006). Psychological First Aid: Field Operations Guide (2nd ed.). National Child Traumatic Stress Network and National Center for PTSD, Introduction and Overview: Behaviors to Avoid

Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.8 Keeping knowledge current: training, licensing CE, supervision, consultation · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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73. ASM-1085 (rev 1) · Best answer: C

In a nursing facility, a 79-year-old resident with heart failure has refused most meals and several medications since their spouse died 6 weeks ago. The resident says, 'I'm just done. Let me be.' The resident has no advance directive about refusing care. Staff want to honor the refusal as the resident's right. What is the **MOST** appropriate action for the social worker?

A. Support the refusal and document it as an informed choice

B. Request a hospice referral to focus on comfort care

C. Evaluate the refusal for depression and a wish to die

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). A capable resident may refuse care, and staff are right to value that. A refusal that began with a recent loss and sounds hopeless may come from treatable depression, so it is not yet a settled, informed choice.

B **Incorrect** (trap: Skips assessment). Hospice can be right for advanced heart failure when comfort is the resident's considered goal. Referring now treats grief-driven refusal as an end-of-life decision before anyone has assessed mood or intent.

C **Correct.** SAMHSA's toolkit for senior living communities lists refusing food, medicine or other treatment among signs that a person needs help right away (SAMHSA, 2011). Six weeks after a spouse's death, 'I'm just done' with refusal of meals and medicine

may reflect depression and a passive wish to die. Assessing mood, hopelessness and intent comes before deciding whether the refusal is an informed choice. It beats A because honoring the refusal now skips the assessment that tells treatable despair from a considered decision.

Takeaway: In older adults, refusing food or medicine after a loss can signal suicidal intent; assess depression before treating it as an informed choice.

Sources: SAMHSA (2011). Promoting Emotional Health and Preventing Suicide: A Toolkit for Senior Living Communities (SMA10-4515), Fact Sheets for Residents: signs that a person needs help now
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.1 Assessing risk of harm to self or others · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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74. INT-1530 (rev 1) · Best answer: C

A social worker co-leads an anger-management group for adolescents at a residential treatment center. The colleague often contradicts the social worker in front of members, and members have begun taking sides. The two discussed the pattern privately after two sessions, but nothing changed. What should the social worker do **NEXT**?

A. Name the disagreement openly with members during the next session
B. Ask the program director to assign a different co-leader
C. Take the conflict to a joint supervision meeting

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Co-leaders can model respectful disagreement in session, but only when their relationship is sound. Raising a live rivalry with teens who are already taking sides risks deepening the split. This fits after the pair has worked the conflict through.

B **Incorrect** (trap: Hands off too soon). Replacing the partner skips the in-house step of joint supervision and leaves the members' side-taking unexamined. Reassignment fits if supervised work fails or the pairing proves unworkable.

C **Correct.** Co-leaders need a working alliance; unresolved rivalry between them splits a group and models poor conflict handling for members (Yalom & Leszcz, 2020). The pair already tried a private talk, and the pattern continues while members take sides, so the next step is outside help: a joint supervision or consultation meeting where both leaders examine the dynamic together. It beats A because airing a still-unresolved conflict in group invites members to widen the split; open disagreement helps members only when the co-leaders are working well together.

Takeaway: Co-leader conflict that persists after direct talk and splits the group goes to joint supervision, not to members or a reassignment.

Sources: Yalom & Leszcz (2020). The Theory and Practice of Group Psychotherapy (6th ed.), The therapist: co-therapy
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.3 Inter- and intradisciplinary teamwork (co-therapy, care conferences, IEPs) · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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75. ASM-1104 (rev 2) · Best answer: D

A 3-year-old has had 8 hospital stays for breathing pauses that only the parent has witnessed. The parent, a former nursing assistant, is warm with staff and presses for more invasive tests. During one monitored stay, the parent was away for 4 days and no pauses occurred. Soon after the parent returned, a pause requiring resuscitation happened while the parent was alone with the child. Which explanation is **MOST** likely?

A. An undiagnosed breathing disorder
B. Parental anxiety causing overreporting
C. Breath-holding spells triggered by distress
D. Illness induced by the caregiver

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). A medical cause is reasonable to pursue early in a workup. It would not explain why events stopped during 4 monitored days without the parent and returned when the parent was alone with the child.

B **Incorrect** (trap: Near-miss diagnosis). An anxious parent may overreport or misread normal breathing. That explanation fails here because a monitored event required resuscitation, which overreporting cannot create.

C **Incorrect** (trap: Near-miss diagnosis). Breath-holding spells in toddlers follow crying, pain or frustration and are seen by many different caregivers. They would not cluster around one caregiver or vanish when that caregiver is away.

D **Correct.** Caregiver-fabricated illness is maltreatment in which a caregiver falsifies or causes a child's symptoms; it should be considered when illness is recurrent, unexplained and inconsistent with testing (Flaherty et al., 2013). The decisive details are timing and observation: no events across 4 monitored days without the parent, then a severe event while the parent was alone with the child. A medically knowledgeable, highly engaged caregiver who seeks more procedures fits the pattern. It beats B because overreporting cannot produce a confirmed event needing resuscitation. In DSM-5-TR, the caregiver receives the factitious disorder diagnosis.

Takeaway: Recurrent unexplained illness that occurs with one caregiver and stops in their absence suggests caregiver-fabricated or induced illness.

Sources: Flaherty, E. G., MacMillan, H. L., & AAP Committee on Child Abuse and Neglect (2013). Caregiver-fabricated illness in a child: A manifestation of child maltreatment. Pediatrics, 132(3), 590-597, Recognition and assessment; DSM-5-TR (APA, 2022), Somatic Symptom and Related Disorders: Factitious Disorder
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.12 Traits of perpetrators of abuse, neglect, exploitation · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

76. ASM-1533 (rev 2) · Best answer: D

A 38-year-old client moved across the country at age 19 and has had no contact with parents or siblings since. The client says, 'They have no hold on me anymore.' The client becomes furious and cannot sleep after the partner's parents offer any advice, and has ended two long friendships over minor disagreements. Which concept **MOST** likely describes this pattern?

A. Differentiation of self
B. Disengaged boundaries
C. Family projection process
D. Emotional cutoff

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Differentiation of self is the ability to stay emotionally connected to family while thinking and acting independently. The client's claim of independence can look like differentiation, but intense reactivity to in-laws and abrupt endings show low, not high, differentiation.

B **Incorrect** (trap: Wrong model or concept). Disengaged boundaries is a structural term for a household whose members are distant and underinvolved with one another. It describes current family organization, not an adult's long-term avoidance of the family of origin with reactivity carried into new relationships.

C **Incorrect** (trap: Wrong model or concept). The family projection process describes parents transmitting their anxiety to a child who becomes the family's focus. The stem says nothing about how the parents focused on this client, and the pattern described is the client's own distancing.

D **Correct.** Emotional cutoff is managing unresolved attachment to the family of origin through distance or silence. Bowen theory holds that cutoff reduces contact but not the underlying reactivity, which then shows up in other close relationships (Kerr & Bowen, 1988). The client claims freedom from family, yet reacts intensely to in-laws' advice and abruptly ends friendships. It beats A because a well-differentiated person stays connected while keeping a separate self and responds calmly; this client's distance coexists with high emotional reactivity.

Takeaway: Emotional cutoff reduces contact with family but not reactivity; unresolved attachment resurfaces as intense reactions in other close relationships.

Sources: Kerr, M. E., & Bowen, M. (1988). Family Evaluation: An Approach Based on Bowen Theory. W. W. Norton, emotional cutoff; differentiation of self
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.2 Family functioning and dynamics and their effects · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

77. ASM-1600 (rev 2) · Best answer: C

A hospital social worker meets with a 72-year-old spouse who has cared at home for 3 years for a partner with major neurocognitive disorder. The spouse describes losing sleep, giving up friendships and feeling trapped. The social worker wants a standardized measure of how the caregiving role itself is affecting the spouse. Which instrument is the **BEST** choice?

A. Geriatric Depression Scale
B. Lawton IADL Scale
C. Zarit Burden Interview
D. Ecomap

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). The Geriatric Depression Scale screens for depression in older adults. The spouse may well be depressed, but a mood score does not show how much of the distress comes from the caregiving role, which is what the social worker wants to measure.

B **Incorrect** (trap: Wrong model or concept). The Lawton scale rates instrumental activities of daily living such as shopping, finances and medications. It describes the care recipient's functional needs, not the strain the caregiver feels.

C **Correct.** The Zarit Burden Interview is a self-report measure built to capture the burden family caregivers feel, including effects on their health, social life and emotional well-being (Zarit, Reever & Bach-Peterson, 1980). Lost sleep, dropped friendships and feeling trapped are exactly the caregiving strain it measures. It beats A because a depression scale measures mood in general; it cannot show how much of the spouse's distress is tied to the caregiving role, which is the target here.

D **Incorrect** (trap: Wrong model or concept). An ecomap is a drawing of a family's links to outside systems such as friends, agencies and church. It could show the lost friendships, but it is not a standardized measure of how burdened the caregiver feels.

Takeaway: Match the tool to the target: burden scales measure caregiver strain, depression scales measure mood, and IADL scales rate the care recipient's function.

Sources: Zarit, S. H., Reever, K. E., & Bach-Peterson, J. (1980). Relatives of the impaired elderly: Correlates of feelings of burden. The Gerontologist, 20(6), 649-655, whole article
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.6 Tools for assessing strengths, resources, needs, challenges · **Qualifier** BEST · **Cognitive** recall · **Difficulty** 2 of 3

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78. ETH-1587 (rev 1) · Best answer: A

A social worker runs a reentry group at a county jail using a curriculum built around personal choices. In one session, members say that heavy policing of their neighborhoods and lack of jobs also led to their arrests. Several members stop taking part when the social worker returns to the workbook. What is the **BEST** response by the social worker?

A. Discuss structural factors alongside personal choices in the session
B. Validate the members' points, then return to the planned module
C. Record the concerns to share with curriculum developers later
D. Keep the focus on personal choices members control after release

Why each option is right or wrong

A **Correct.** Anti-oppressive practice links personal troubles to structural conditions while still supporting individual agency (Dominelli, 2002). Members have named policing and job loss, and they disengage when these are set aside. Bringing structural factors into the discussion alongside personal choices takes their experience seriously and keeps them engaged in the work. It beats B because a brief validation followed by a return to the workbook repeats the very move that led members to withdraw (NASW Code 1.05(b)).

B **Incorrect** (trap: Good but not best). Validation is a good start, but returning straight to the workbook signals that structural concerns don't belong in the group. Members already withdrew when that happened.

C **Incorrect** (trap: Right action, wrong time). Feedback to curriculum developers is worth giving and can follow. It does nothing for the members who are disengaging in the session now.

D **Incorrect** (trap: Wrong model or concept). Personal agency matters for reentry, and the curriculum stresses it. Focusing solely on individual choices dismisses the structural factors members named and can make their experience sound like excuse-making.

Takeaway: Anti-oppressive group work connects members' personal choices to the structural conditions they name rather than individualizing every problem.

Sources: Dominelli, L. (2002). Anti-Oppressive Social Work Theory and Practice. Palgrave Macmillan, structural and personal dimensions of oppression; NASW Code of Ethics (2021), 1.05(b)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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79. INT-1630 (rev 1) · Best answer: A

A client who uses a wheelchair has been turned down after four job interviews. Two of the employers had offices reached only by stairs, so those interviews took place in a lobby. The client says, 'Maybe I'm just not good enough to hire.' Which response from the social worker is **MOST** helpful?

A. Help the client look at how barriers shaped these outcomes
B. Rehearse interview answers to build the client's confidence
C. Contact the employers to raise the access problem
D. Refer the client for counseling on self-esteem

Why each option is right or wrong

A **Correct.** Empowerment practice links personal troubles to structural conditions, so that clients stop blaming themselves for the effects of exclusion (Gutierrez, 1990). This consciousness raising is a first step toward action: a client who sees the barrier can decide how to respond (Lee, 2001). The client blames personal inadequacy even though two interviews were moved to a lobby because the offices were inaccessible. It beats B because rehearsal treats the problem as a skills gap and can confirm the self-blame; skills work may help later, once the client sees the structural part of the picture.

B **Incorrect** (trap: Good but not best). Interview practice is a reasonable support, but starting there accepts the client's view that the rejections reflect personal shortcomings. It ignores the access barriers that shaped at least two of the interviews.

C **Incorrect** (trap: Overrides the client's choice). Contacting employers may become part of a plan, but doing so now acts for the client before the client has made sense of what happened or chosen a response. Empowerment builds the client's own capacity to act.

D **Incorrect** (trap: Hands off too soon). Referral for self-esteem work frames the problem as an internal deficit and sends away a concern the social worker can address directly. The client's self-doubt follows real exclusion, which belongs in this conversation.

Takeaway: Empowerment practice helps clients connect personal troubles to structural barriers before choosing action, reducing self-blame for the effects of exclusion.

Sources: Gutierrez, L. M. (1990). Working with women of color: An empowerment perspective. Social Work, 35(2), 149-153, Empowerment techniques: raising critical consciousness; Lee, J. A. B. (2001). The Empowerment Approach to Social Work Practice (2nd ed.). Columbia University Press, Consciousness raising
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.1 Strengths-based and empowerment interventions · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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80. INT-1676 (rev 1) · Best answer: C

A county parenting program is 5 months into its first year. Process data show that parents who speak Spanish come to the first session but rarely come back. Exit calls reveal that sessions are held in English with English handouts. The funder's summative outcome evaluation is scheduled for month 12. Which action is **BEST**?

A. Keep the model unchanged until the outcome evaluation is done

B. Interview the Spanish-speaking parents about why they stop attending

C. Add Spanish-language sessions and track retention by language

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Holding a model steady can protect the comparison in a tightly controlled trial, but this is a first-year program whose process data exist to improve delivery. Waiting 7 months leaves a whole group of parents without usable services; the summative report can describe the change.

B **Incorrect** (trap: Right action, wrong time). Asking parents why they leave is the right step when the reason is unknown. Exit calls have already shown that language is the barrier, so another round of interviews delays a fix the data already support.

C **Correct.** Formative evaluation exists to improve a program while it is running (Scriven, 1967). Process data have identified a delivery barrier and exit calls explain it, so the program should change delivery now and keep monitoring whether the change works. It beats A because freezing a component that is failing a group of families trades their access for a cleaner year-end comparison; the summative evaluation can account for a documented mid-course change. Fixing implementation problems first also makes the later outcome findings more meaningful (Rossi, Lipsey & Henry, 2019).

Takeaway: Use formative findings to correct delivery during implementation; don't hold a failing component unchanged to protect a later summative evaluation.

Sources: Scriven, M. (1967). The methodology of evaluation. In R. W. Tyler, R. M. Gagne, & M. Scriven (Eds.), Perspectives of Curriculum Evaluation (pp. 39-83). Rand McNally, formative versus summative roles of evaluation; Rossi, P. H., Lipsey, M. W., & Henry, G. T. (2019). Evaluation: A Systematic Approach (8th ed.). SAGE, Ch. 4: Assessing Program Process and Implementation
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.4 Agency program evaluation: needs assessment, formative/summative, cost analyses · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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81. ETH-1550 (rev 1) · Best answer: B

A client receiving depression treatment at a community mental health clinic has missed 3 sessions in a row. The social worker has called twice and left voicemails, but the client has not replied. The treatment goals are not yet met. What should the social worker do **NEXT**?

A. Close the case and record the client's lack of contact
B. Send a letter offering a closing session and referrals
C. Keep the case open and hold the client's weekly time

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Closing the file after two voicemails, with no final outreach and no referrals, leaves a client with unmet goals without a path back to care. Closing becomes appropriate after a final offer of closure or referral has gone unanswered.

B **Correct.** Social workers avoid abandoning clients who still need services and help arrange continued care when services end (NASW Code 1.17(b)). After two unanswered calls, a letter that offers a closing session and lists referrals gives this client, whose goals are unmet, a clear chance to return or move to other care. It also shows a reasonable final effort before closure (NASW Code 1.17(e)). It beats C because holding a slot indefinitely leaves the client's status unresolved and offers no route to other help if the client does not come back.

C **Incorrect** (trap: Good but not best). Holding the time slot is well-meant, but it leaves the client's status unclear, delays services for others and offers no closure or referral. It fits only when the client has said they plan to return soon.

Takeaway: When a client stops attending, make a final outreach that offers closure and referrals, and document it before closing the case.

Sources: NASW Code of Ethics (2021), 1.17(b); NASW Code of Ethics (2021), 1.17(e)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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82. INT-0119 (rev 1) · Best answer: B

A family service agency learns its funding will drop 15% next year. It runs a crisis response program, a parenting education class and a youth recreation program. The director must decide how to allocate the reduced budget. Which approach is **BEST**?

A. Cut each program's budget by the same 15%
B. Rank programs openly by client need and outcome data
C. Close the program that serves the fewest clients
D. Protect the programs that board members value most

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Across-the-board cuts are simple and look even-handed, but they treat unequal needs and results as equal. A 15% cut to crisis services may cause more harm than the same cut elsewhere.

B **Correct.** When resources cannot meet every need, the NASW Code calls for allocation procedures that are open and fair, nondiscriminatory and based on appropriate, consistently applied principles (NASW Code 3.07(b)). Ranking programs by client need, including the safety stakes of crisis services, and by evidence of outcomes applies clear principles in a transparent way. It beats A because equal cuts look fair but ignore differences in need and effectiveness, and could weaken an essential crisis service as much as an optional one.

C **Incorrect** (trap: Skips assessment). Participant counts alone are a narrow measure. A small program can serve the highest-need clients, such as people in crisis, so need and outcomes should be weighed before anything is closed.

D **Incorrect** (trap: Misreads law or ethics). Basing cuts on what board members prefer substitutes personal preference for open, consistently applied principles. Board input matters, but allocation should rest on client need and evidence.

Takeaway: When cutting budgets, allocate through an open process using consistent principles such as client need and outcome evidence.

Sources: NASW Code of Ethics (2021), 3.07(b)
Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.6 Budgeting, fiscal oversight, and allocating resources · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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83. ASM-1554 (rev 1) · Best answer: A

At a refugee resettlement youth program, a 16-year-old who arrived with family 4 years ago now refuses to speak the family's language or attend community celebrations, calling them 'embarrassing.' Classmates mock the teen's accent, and the teen has stopped trying to join school activities. The teen says they do not fit in anywhere. Which acculturation strategy is the teen **MOST** likely showing?

A. Marginalization
B. Assimilation
C. Separation
D. Integration

Why each option is right or wrong

A **Correct.** Berry's (1997) model crosses two questions: is the heritage culture kept, and is contact with the larger society sought? Marginalization is low on both and often arises from enforced cultural loss combined with exclusion. This teen rejects the family's language and celebrations and, after being mocked, has stopped trying to join school activities. It beats B because assimilation means giving up the heritage culture while actively taking part in the host society; this teen has withdrawn from both.

B **Incorrect** (trap: Wrong model or concept). Assimilation fits the rejection of the family's language, but it also requires seeking daily participation in the new culture. The teen has stopped joining school activities after exclusion, so the host-culture side is missing.

C **Incorrect** (trap: Wrong model or concept). Separation means holding on to the heritage culture while avoiding the larger society. This teen is pulling away from the heritage culture too, so the first half of the pattern does not fit.

D **Incorrect** (trap: Wrong model or concept). Integration combines keeping the heritage culture with taking part in the larger society and is linked to the best adaptation. The teen is engaged in neither culture, the opposite of this pattern.

Takeaway: Berry's acculturation strategies rest on two dimensions, keeping heritage culture and joining the larger society; losing both is marginalization.

Sources: Berry, J. W. (1997). Immigration, acculturation, and adaptation. Applied Psychology: An International Review, 46(1), 5-34, acculturation strategies

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.20 Displacement effects: homelessness, immigration, refugee status · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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84. ETH-1664 (rev 1) · Best answer: A

PRETEST — unscored on this form; do not count it

A tenant in supportive housing keeps stacks of newspapers and boxes that block the apartment's only exit path. After an inspection, the fire marshal gives the tenant 14 days to clear the path or the lease will be ended. The tenant tells the social worker, 'It's my home. I decide what stays.' What should the social worker do **FIRST**?

A. Explain the deadline and what is at stake if it passes

B. Arrange a crew to clear the path while the tenant is out

C. Ask the property manager to respect the tenant's choice

D. Refer the tenant to a treatment group for hoarding

Why each option is right or wrong

A **Correct.** Self-determination may be limited when a choice poses a serious, foreseeable and imminent risk to the client or others (NASW Code 1.02), and a blocked exit endangers the tenant, neighbors and firefighters. When legal obligations override a client's wishes, the client should be told (NASW Code 1.01). Explaining plainly what the order requires and what is at stake sets up a joint plan the tenant can shape, such as what to move and in what order. It beats D because hoarding treatment may help over time, but the 14-day order comes first.

B **Incorrect** (trap: Overrides the client's choice). Clearing the path without the tenant removes the hazard but overrides the tenant in their own home, and forced cleanouts

often damage trust and lead to quick reaccumulation. The tenant should lead the work within the safety limit.

C **Incorrect** (trap: Ignores risk). A tenant's control over their home matters, but a blocked exit threatens neighbors and emergency responders too. Asking the manager to drop the requirement ignores a real fire risk and a legal order.

D **Incorrect** (trap: Right action, wrong time). Hoarding treatment can help the tenant keep the path clear over the long run and may be part of the plan. Starting there leaves the 14-day deadline and the immediate fire hazard unaddressed.

Takeaway: When safety law limits a client's choice, tell the client plainly, then let them shape how the requirement is met.

Sources: NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.01
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 3 of 3

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85. INT-1552 (rev 1) · Best answer: B

A community mental health agency sees almost no clients from a recently resettled refugee community, although resettlement staff report high stress among its families. Translated flyers mailed last spring produced no calls. The community has an active mutual assistance association whose elected leaders families consult on many decisions. What should the social worker do **FIRST**?

A. Ask community leaders to hand out the translated flyers
B. Plan the outreach jointly with the community's leaders
C. Host an open house at the agency with interpreters

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Involving leaders is a step in the right direction, but it asks them only to deliver a message the agency wrote, and that message already failed. This fits after the content and approach have been shaped with the community.

B **Correct.** Building a service network with a community means treating its own organizations as partners in designing services, not only as audiences (NASW Cultural Competence Standard 5). The association is a trusted local asset, and the failed flyers suggest the message or the service may not fit. Co-planning lets leaders shape how help is described, where it is offered and who delivers it (Kretzmann & McKnight, 1993). It beats A because handing leaders a flyer that already failed uses their trust without using their knowledge.

C **Incorrect** (trap: Right action, wrong time). An open house is welcoming, but it asks families to come to an unfamiliar agency before any trust exists. It fits later, once community partners have helped plan and promote it.

Takeaway: Engage underserved communities through their trusted organizations as co-planners of outreach and services, not just as distribution channels.

Sources: NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 5: Service Delivery; Kretzmann, J. P., & McKnight, J. L. (1993). Building Communities from the Inside Out: A Path Toward Finding and Mobilizing a Community's Assets. ACTA Publications, asset mapping: inventory of individual, associational and institutional assets
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.24 Building service networks and community resources · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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86. ETH-1105 (rev 1) · Best answer: B

A social worker in a group practice completes a 2-day workshop in a trauma treatment model. The practice's marketing coordinator then updates the website to call the social worker a 'certified trauma specialist.' Certification in the model requires supervised cases that the social worker has not yet begun. What should the social worker do?

A. Keep the description while starting the certification cases
B. Have the profile describe the training actually received
C. Tell new clients at intake that certification is pending

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Planning to finish certification is good, and the Code allows accepting work while acquiring competence. Advertising the credential before earning it, though, misrepresents qualifications to the public (4.06(c)).

B **Correct.** Social workers make sure representations of their credentials and competence are accurate, claim only credentials they hold, and correct others' inaccurate statements about them (NASW Code 4.06(c)). A workshop is a start, but competence in a new technique also takes training, consultation and supervision (1.04(b)), which the NASW Competence principle reinforces. The coordinator made the error, yet correcting it is the social worker's duty. B beats A because intending to earn a credential does not make the claim accurate now, and the public reads the website today.

C **Incorrect** (trap: Good but not best). Telling clients at intake helps those who arrive, but the website keeps misleading everyone who reads it, including referral sources. The inaccurate public statement itself must be corrected.

Takeaway: Claim only credentials actually held, and correct others' inaccurate descriptions of one's qualifications.

Sources: NASW Code of Ethics (2021), 4.06(c); NASW Code of Ethics (2021), 1.04(b); NASW Code of Ethics (2021), Ethical Principles: Competence
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.5 Core values: competence, social justice, integrity, dignity and worth · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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87. ASM-1613 (rev 1) · Best answer: C

PRETEST — unscored on this form; do not count it

A 24-year-old client is in treatment for depression at a college counseling center. The client lives with parents and grandparents, as the family's culture expects until marriage, and describes the arrangement as supportive. While reviewing a draft plan, the social worker notes that the center's template lists 'move to independent housing' as a standard goal for clients this age. How should the social worker handle this goal?

A. Keep the goal, since leaving home fits this life stage
B. Ask the client privately whether the family applies pressure
C. Replace it with goals the client defines within the family

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Developmental views that center early adulthood on leaving home reflect individualistic norms. Applied to a client whose culture expects multigenerational living, the goal treats a supportive arrangement as a problem and is unlikely to be owned by the client.

B **Incorrect** (trap: Skips assessment). Checking for family pressure makes sense when a client seems to defer or seems distressed. This client has already described the arrangement as supportive, so probing for coercion treats the family's custom as a likely problem.

C **Correct.** Plans reflect the client's own goals within the client's culture, and social workers recognize the strengths in all cultures (NASW Code 1.05(a), 1.02). This client describes multigenerational living as expected and supportive, so a goal of moving out imposes a norm the client does not share and could add family strain. Goals such as more social contact or steadier study routines can be built within the household. It beats A because independent living is one cultural pattern of adulthood, not a developmental requirement.

Takeaway: Build plan goals from the client's cultural definition of healthy functioning, not from one culture's developmental norm.

Sources: NASW Code of Ethics (2021), 1.05(a); NASW Code of Ethics (2021), 1.02
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.4 Culture in intervention planning · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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88. ASM-1536 (rev 1) · Best answer: A

A 24-year-old client with obsessive-compulsive disorder lives with a parent. In a joint session, the parent says they rewash the client's clothes on request, answer the same reassurance questions for up to 2 hours each night, and no longer invite guests to the home. The client's rituals have increased over the past year. Which response is **MOST** appropriate?

A. Plan with both how the parent will reduce these accommodations
B. Begin exposure exercises with the client in individual sessions
C. Refer the parent to a caregiver support group for stress

Why each option is right or wrong

A **Correct.** Family accommodation, such as excessive reassurance or washing on a relative's behalf, is linked to more severe obsessive-compulsive symptoms and can predict poorer treatment outcomes, so reducing it is a key treatment goal (Lebowitz, Panza & Bloch, 2016). This parent's rewashing, nightly reassurance and avoidance of guests are feeding the client's rituals. It beats B because exposure work is a core treatment, but if the household keeps supplying reassurance and rewashing, gains are undercut; planning the reduction with both client and parent addresses the maintaining pattern directly.

B **Incorrect** (trap: Good but not best). Exposure with response prevention is a first-line treatment and will likely be part of care. Doing it only in individual sessions leaves the parent's nightly reassurance and rewashing in place, which keeps reinforcing the rituals between sessions.

C **Incorrect** (trap: Hands off too soon). The parent's strain deserves attention, and support may be added. A support group alone does not change the accommodating behaviors that are maintaining the client's symptoms.

Takeaway: Family accommodation can maintain OCD and anxiety symptoms; plan its reduction with the client and family rather than treating the client in isolation.

Sources: Lebowitz, E. R., Panza, K. E., & Bloch, M. H. (2016). Family accommodation in obsessive-compulsive and anxiety disorders: A five-year update. Expert Review of Neurotherapeutics, 16(1), 45-53, whole article
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.2 Family functioning and dynamics and their effects · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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89. ASM-1680 (rev 2) · Best answer: A

At a primary care visit, a parent of two completes an adverse childhood experiences (ACE) questionnaire and scores 7. The parent describes fears that the score means they will become seriously ill or harm their children, then asks the social worker what the number means. What is the **MOST** appropriate response?

A. Explain that the score estimates risk for groups rather than individuals

B. Recommend trauma-focused therapy because of the high score

C. Encourage the parent to keep talking about these parenting fears

Why each option is right or wrong

A **Correct.** An ACE score counts types of childhood adversity and is linked to risk across populations; its developers stress that it is neither diagnostic nor predictive for an individual (Anda, Porter & Brown, 2020). The parent has voiced specific fears and asked a direct question, so a plain, accurate answer helps most now. Explaining that the score describes group risk, not what will happen to this parent, corrects the misreading and opens talk about the supports that shape outcomes. It beats C because the parent has already described the fears; more exploration would sidestep the question.

B **Incorrect** (trap: Skips assessment). Treatment should follow an assessment of current symptoms and functioning, not a questionnaire score. Recommending therapy on the score alone treats the number as a diagnosis, which the measure's developers caution against.

C **Incorrect** (trap: Good but not best). Inviting more detail is useful early, but the parent has already shared these fears and is now asking for information. Deflecting the direct question could add to the worry the number has caused.

Takeaway: An ACE score shows population-level risk; it does not diagnose or predict an individual's outcome, so explain it accurately to clients.

Sources: Anda, R. F., Porter, L. E., & Brown, D. W. (2020). Inside the adverse childhood experience score: Strengths, limitations, and misapplications. American Journal of Preventive Medicine, 59(2), 293-295, whole article
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.6 Tools for assessing strengths, resources, needs, challenges · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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90. INT-1061 (rev 1) · Best answer: B

In week 4 of a closed support group for adults with chronic pain, one member talks at length each session about medical appointments. Others have started checking their phones, and two arrived late last week. No one has interrupted the talkative member. What should the social worker do **FIRST**?

A. Meet privately with the talkative member after group
B. Ask the group how it has been sharing session time
C. Set a fixed time limit for each member's turn
D. Ask the member to save medical updates for later

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Meeting privately is a useful second step if group-level work does not change the pattern. Done first, it singles out the member and misses the group's part in letting one person hold the floor.

B **Correct.** A monopolizing member is a group-level problem: the group allows it, often to avoid its own material. The leader first works through the group, inviting members to look at how time is shared, which brings in quieter members and spares the talkative one public blame (Yalom & Leszcz, 2020; SAMHSA TIP 41). It beats A because a private meeting is a sound step if the group-level approach fails, but used first it treats the problem as one person's and leaves the group's silence unaddressed.

C **Incorrect** (trap: Good but not best). A uniform time limit treats the symptom with a rule and skips the process the group needs to look at. It can feel punitive and rarely helps members understand why the pattern developed.

D **Incorrect** (trap: Too directive). Publicly redirecting the member's content may embarrass them and casts the issue as one person's fault. It also leaves the other members' silence and withdrawal unexplored.

Takeaway: Address a monopolizing member first at the group level, exploring how members share time, before meeting with the member privately.

Sources: Yalom, I. D., & Leszcz, M. (2020). The Theory and Practice of Group Psychotherapy (6th ed.). Basic Books, problem group members: the monopolist; SAMHSA TIP 41 (2005), Ch. 6: Group Leadership, Concepts, and Techniques
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.23 Group work: managing process and building cohesion · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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91. INT-1139 (rev 1) · Best answer: C

PRETEST — unscored on this form; do not count it

In a parenting group for families working toward reunification, the social worker models giving a 4-year-old a calm, specific instruction. The next week, a parent accurately lists every step of the demonstration and says they want to use it. The parent reports that at visits the words come out rushed and harsh. What should the social worker do **NEXT**?

A. Model the instruction again, more slowly this time
B. Offer the parent a reward for using the skill at visits
C. Have the parent practice the instruction with coaching
D. Explain the reasons the calm instruction works

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). A slower demonstration helps when an observer missed or forgot steps. This parent recalls every step, so watching again adds little; the difficulty lies in performing the skill.

B **Incorrect** (trap: Wrong model or concept). Incentives target the motivation process, which matters when someone can perform a skill but chooses not to. This parent wants to use it and is trying, so a reward would not fix the rushed delivery.

C **Correct.** Bandura's model of observational learning has four processes: attention, retention, reproduction, and motivation (Bandura, 1977). The parent lists every step (retention) and wants to use the skill (motivation), but the words come out rushed and harsh, so the gap lies in turning the memory into skilled action. Guided practice with feedback builds that reproduction. It beats A because another demonstration feeds retention, which is already intact; the parent needs to perform the skill, not watch it again.

D **Incorrect** (trap: Good but not best). Explaining why a skill works supports attention and buy-in, which this parent already shows by recalling every step and wanting to use it. The gap is in performance, which needs practice.

Takeaway: If a client remembers a modeled skill and wants to use it but performs it poorly, provide guided practice to build reproduction.

Sources: Bandura, A. (1977). Social Learning Theory. Prentice-Hall, observational learning: attention, retention, reproduction, motivation
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.26 Using role modeling to teach behavior · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

92. ASM-1516 (rev 1) · Best answer: D

PRETEST — unscored on this form; do not count it

In a telehealth session, a 22-year-old who moved away for graduate school says the faith absorbed from family and congregation no longer feels like their own. The client is reading widely, questioning teachings once accepted without thought and working out which beliefs to keep by personal choice. According to Fowler, which stage of faith does this **MOST** likely describe?

A. Synthetic-conventional
B. Conjunctive
C. Mythic-literal
D. Individuative-reflective

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Synthetic-conventional faith, common in adolescence, rests on beliefs absorbed from family and community and seldom examined. It describes where the client started, not the current critical questioning.

B **Incorrect** (trap: Wrong model or concept). Conjunctive faith, usually not before midlife, accepts paradox and draws meaning from several traditions after long self-examination. This client is in the earlier work of critically sorting inherited beliefs, not integrating paradox.

C **Incorrect** (trap: Wrong model or concept). Mythic-literal faith belongs to school-age children, who take religious stories and rules literally. A graduate student critically weighing inherited teachings is far beyond this stage.

D **Correct.** Fowler's individuative-reflective stage, often beginning in young adulthood when a person leaves home, involves critically examining inherited beliefs and taking personal responsibility for one's own commitments (Fowler, 1981). This client is questioning teachings once taken for granted and choosing which beliefs to keep. It beats A because synthetic-conventional faith, typical of adolescence, holds beliefs absorbed from the community without examining them, the stage the client is now leaving. Spiritual questioning at this age is a normal developmental transition, not a crisis by itself.

Takeaway: Fowler's individuative-reflective stage is the young-adult move from unexamined inherited faith to critically chosen, personally owned beliefs.

Sources: Fowler, J. W. (1981). Stages of Faith: The Psychology of Human Development and the Quest for Meaning. Harper & Row, Stage 4: individuative-reflective faith

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.19 Racial, ethnic, cultural, spiritual development across the lifespan · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

93. ETH-1514 (rev 3) · Best answer: B

A social worker provided couples therapy to two partners for 8 months. The partners have since separated and are disputing custody of their children. One former partner asks the social worker to write a letter to the court describing that partner as the better parent. What is the **MOST** appropriate response?

A. Write a letter describing that partner's parenting strengths
B. Explain why the past role rules out taking a side
C. Write the letter once the other partner also consents

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). Writing for one partner abandons the neutral role the worker held with both and offers a custody opinion the worker never formed through an evaluation. It could also disclose couples material without the other partner's consent.

B **Correct.** When serving couples or families, social workers clarify their obligations to each person and anticipate conflicts, such as being asked to testify in a custody dispute (NASW Code 1.06(d)). As the couple's therapist, the worker owed duties to both partners and never evaluated either as a parent, so advocating for one would betray the other and exceed what the worker knows. Explaining this role limit answers the request honestly. It beats C because consent addresses confidentiality but not the role conflict or the missing custody evaluation (Reamer, 2020).

C **Incorrect** (trap: Good but not best). Getting both partners' consent solves the confidentiality problem and may seem fair. It leaves the role problem in place: the worker would still be taking one side and giving a parenting opinion without having done a custody evaluation.

Takeaway: In couples or family work, clarify duties to each person; a former couples therapist does not advocate for one partner in custody disputes.

Sources: NASW Code of Ethics (2021), 1.06(d); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, conflicts of interest: forensic and custody roles
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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94. ASM-1039 (rev 2) · Best answer: C

Which mood can count toward a depressive episode in children and adolescents in place of sad or depressed mood?

A. Anxious mood
B. Labile mood
C. Irritable mood
D. Detached mood

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Anxiety often accompanies depression in young people and can be noted with the anxious distress specifier. It does not replace depressed mood as a core symptom of a depressive episode.

B **Incorrect** (trap: Wrong model or concept). Labile mood, quick shifts between emotions, can appear in bipolar disorder, personality disorders and brain injury. DSM-5-TR does not name it as a substitute for depressed mood in young people; irritability is the recognized variant.

C **Correct.** DSM-5-TR allows irritable mood to stand in for depressed mood in children and adolescents, both for a major depressive episode and for persistent depressive disorder. Young people often show depression as crankiness, anger or low frustration tolerance rather than reported sadness, so an irritable child should be screened for depression, not only for behavior problems. It beats A because anxiety commonly co-occurs with youth depression but does not substitute for the core mood criterion.

D **Incorrect** (trap: Wrong model or concept). Detachment can accompany depression, trauma or dissociation, but DSM-5-TR does not name it as a substitute for depressed mood in youth. Irritability is the recognized developmental variant.

Takeaway: In youth, irritable mood can replace sad mood in depression criteria; irritable children need depression screening, not just behavior plans.

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Major Depressive Disorder
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.5 Signs of mental and emotional illness across the lifespan · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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95. ETH-1006 (rev 2) · Best answer: B

A social worker in private practice bills a client's health plan with the client's written authorization. During a utilization review, the plan's reviewer asks for copies of all session notes to decide whether to approve more sessions. What is the **BEST** way for the social worker to respond?

A. Send copies of every session note the reviewer has requested
B. Send the treatment plan and progress the decision requires
C. Send the notes with identifying details removed

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Sending every note exceeds what the reviewer needs to approve sessions and exposes details the payment decision does not require. Full notes would be appropriate only if the client specifically authorized them for a stated reason.

B **Correct.** Disclosures to a payer require the client's authorization (NASW Code 1.07(h)), and even an authorized disclosure is limited to what its purpose requires (NASW Code 1.07(c)). HIPAA likewise applies a minimum-necessary standard to disclosures for payment (45 CFR 164.502(b)). The reviewer needs the diagnosis, treatment plan and progress toward goals, not session-by-session detail. B beats A because full notes go beyond what the approval decision requires.

C **Incorrect** (trap: Misuses the technique). Removing identifiers does not work for a coverage decision, because the plan must know whose care it is approving. It also still sends far more clinical detail than the decision needs. De-identification suits consultation or teaching, not billing.

Takeaway: Share with a payer just the information its decision needs, even when the client has authorized disclosure.

Sources: NASW Code of Ethics (2021), 1.07(h); NASW Code of Ethics (2021), 1.07(c); HIPAA Privacy Rule, 45 CFR 164.502(b)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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96. ETH-1025 (rev 2) · Best answer: A

An adult client at a group private practice asks to use their employer health plan to pay for counseling. The plan requires a diagnosis on every claim and reviews progress after 12 sessions before approving more. The client is about to sign the consent form. What is **MOST** important for the social worker to explain before services begin?

A. What the plan will receive and how it may limit sessions
B. That session content will stay between the client and worker
C. That the client may drop the plan and self-pay later

Why each option is right or wrong

A **Correct.** Informed consent includes the limits that third-party payers place on services (NASW Code 1.03(a)). The decisive facts are that the plan will see a diagnosis on every claim and can stop approving sessions after review. Both affect the client's privacy and the length of care, so the client needs them before agreeing. A beats C because self-pay is a useful alternative, but the client can weigh it only after learning what using insurance involves. Any disclosure to the plan also needs the client's authorization (NASW Code 1.07(h)).

B **Incorrect** (trap: Misreads law or ethics). Promising that session content stays private is inaccurate here, because the plan will receive a diagnosis and may review progress. A false assurance undermines consent and damages trust once the client learns what the insurer sees.

C **Incorrect** (trap: Good but not best). Self-pay is a reasonable alternative worth mentioning, and it may become the client's choice. It helps only after the client understands what insurance involves; offered first, it skips the payer disclosure the client needs to decide.

Takeaway: Before services begin, tell clients what a third-party payer will receive and how it may limit their care.

Sources: NASW Code of Ethics (2021), 1.03(a); NASW Code of Ethics (2021), 1.07(h)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.4 Informed consent: principles and how it is obtained · **Qualifier** MOST · **Cognitive** application · **Difficulty** 1 of 3

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97. ETH-1054 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

Under the HIPAA Privacy Rule, which records must a covered entity keep for 6 years?

A. Clinical progress notes for each client
B. Written policies and signed authorizations
C. Psychotherapy notes kept apart from the record
D. Records of clients who have died

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Retention of clinical progress notes is set by state law, agency policy and contracts (3.04(d)), and the periods vary. The HIPAA 6-year rule covers compliance documentation, not the clinical record.

B **Correct.** The Privacy Rule requires covered entities to keep required compliance documentation, such as policies and procedures, signed authorizations and other required written communications, for 6 years from creation or last effective date (45 CFR 164.530(j)). It does not set a retention period for the clinical record itself; state law, agency policy and contracts do (NASW Code 3.04(d)). B beats A because confusing these two rules is a common error: the 6-year figure applies to HIPAA paperwork, not to progress notes.

C **Incorrect** (trap: Wrong model or concept). Psychotherapy notes get extra protection from disclosure under HIPAA, but the Privacy Rule sets no 6-year retention period for them. Their retention follows state law and agency policy.

D **Incorrect** (trap: Wrong model or concept). HIPAA continues to protect a deceased client's information, but it sets no special retention period for those records. State law, agency policy and contracts govern how long they are kept.

Takeaway: HIPAA's 6-year rule covers compliance documents like policies and authorizations; clinical record retention follows state law, policy and contracts.

Sources: HIPAA Privacy Rule, 45 CFR 164.530(j); NASW Code of Ethics (2021), 3.04(d)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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98. ASM-0119 (rev 1) · Best answer: B

A client began services 5 months ago for social anxiety, with goals of joining a weekly activity and speaking up in work meetings. At a progress review, the social worker considers several findings. Which finding **MOST** clearly indicates that the client is ready to end services?

A. Says sessions have become the best part of the week
B. Has reached both targets and copes with setbacks on their own
C. Has attended every session and completed each assignment

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Valuing sessions highly can reflect a strong alliance, but it can also signal reliance on the relationship. It says nothing about whether goals are met and may need to be explored as ending approaches.

B **Correct.** Readiness for termination rests on goal attainment and the client's ability to maintain gains independently (Hepworth et al.). This client has reached both treatment goals and manages setbacks without the social worker, which shows the change can last. Services should end when they are no longer needed (NASW Code 1.17(a)). It beats C because perfect attendance and completed homework show strong engagement in the work, not that the work is finished.

C **Incorrect** (trap: Good but not best). Attendance and homework completion show engagement and predict good outcomes, but they describe the process, not the result. A highly engaged client may still have goals left to reach.

Takeaway: Readiness to end services is shown by goals met and gains kept independently, not by attendance or attachment to sessions.

Sources: NASW Code of Ethics (2021), 1.17(a); Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, termination: determining readiness
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.6 Signs a client is ready for termination · **Qualifier** MOST · **Cognitive** application · **Difficulty** 1 of 3

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99. ETH-1052 (rev 1) · Best answer: A

A supervisor reviews a supervisee's unsigned draft progress note for a hospital patient awaiting discharge. The note calls the patient 'manipulative and drug-seeking.' It also records that the patient asked for pain medication 4 times in an hour and refused a walk with physical therapy. What should the supervisor do **FIRST**?

A. Have the supervisee revise the draft to describe observed behavior
B. Co-sign the draft, then discuss wording in the next supervision
C. Have the supervisee add an addendum explaining the terms
D. Revise the note personally so it is accurate before co-signing

Why each option is right or wrong

A **Correct.** Records must be accurate, and social workers use accurate, respectful language rather than derogatory labels when writing about clients (NASW Code 1.12, 3.04(a)). Because this note is an unsigned draft, it can still be revised directly; addenda are for completed entries. The note already contains the observable facts, so the fix is to replace the labels with those behaviors. A beats B because co-signing first would make the labels part of the permanent record, where they could then be corrected only by an addendum.

B **Incorrect** (trap: Right action, wrong time). Discussing wording is right, but signing first makes the labels part of the permanent record. The revision belongs before the note is finalized, with the teaching conversation alongside it.

C **Incorrect** (trap: Misuses the technique). An addendum is the correction method for a signed, completed note. This draft is unsigned, so an addendum would leave the derogatory labels in place when they can simply be revised.

D **Incorrect** (trap: Good but not best). Rewriting it personally fixes this note but takes over the supervisee's authorship of their own documentation and skips the learning that supervision exists to provide. The supervisee should make the revision.

Takeaway: Replace labels with observed behavior in records; revise drafts directly, and correct finalized notes only by addendum.

Sources: NASW Code of Ethics (2021), 1.12; NASW Code of Ethics (2021), 3.04(a)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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100. INT-1034 (rev 3) · Best answer: C

A client at a family services agency wants to ask their parent-in-law, who lives with the family, to stop giving the client's 6-year-old candy at bedtime. When the social worker offers a handout with an I-statement script, the client frowns and says, 'That's not how we talk to elders in my family.' What should the social worker do **FIRST**?

A. Coach the client to deliver the handout script as written
B. Help the client plan a respectful request made in private
C. Ask how the client would usually raise such a concern at home

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). I-statements are a standard assertiveness tool, but pressing the script after the client has said it clashes with family norms imposes one cultural style of directness and makes success less likely.

B **Incorrect** (trap: Right action, wrong time). A private, respectful request is a common way to adapt assertiveness where elders are owed deference, and it may be where this conversation lands. Proposing it now assumes the cultural rule instead of asking; in some families the right route is through a spouse or another relative.

C **Correct.** Assertiveness means stating one's needs while respecting others, and its form should fit the client's cultural values, which the client knows best (NASW Cultural Competence Standard 4). The client has signaled that the script clashes with family expectations but has not said what would fit: a private talk, going through a spouse, or different wording. Asking how concerns are usually raised with elders lets the worker shape the skill around the client's own knowledge. It beats B because a private request may be exactly right, but choosing it for the client guesses at a norm the client is ready to explain.

Takeaway: When a client signals that a skill clashes with cultural norms, ask how things are done in their family before adapting the skill.

Sources: NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 4: Cross-Cultural Skills
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.25 Teaching assertiveness skills · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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101. ETH-1030 (rev 2) · Best answer: A

A client comes to an outpatient substance use program's walk-in hours asking to start treatment today. The client smells of alcohol, slurs some words, says they had 'a few drinks' an hour ago, and walks steadily. The intake packet includes the consent to treatment. What is the **BEST** way for the social worker to proceed?

A. Offer brief support now and hold consent until sober
B. Complete the consent forms while motivation is high
C. Ask the client to return another day when sober

Why each option is right or wrong

A **Correct.** Valid consent requires that the client understand the services, risks and rights and choose freely (NASW Code 1.03(a)). Recent drinking with slurred speech can impair that understanding for now. The best response keeps the motivated client engaged with brief support and a set return time while postponing formal consent until the client is sober. A beats B because a signature given while impaired may not reflect informed consent. It beats C because sending the client away entirely risks losing a moment of readiness.

B **Incorrect** (trap: Misreads law or ethics). Motivation matters, but a signature given while intoxicated may not reflect real understanding of services, risks and rights. Consent obtained now could need redoing and weakens trust in the process.

C **Incorrect** (trap: Good but not best). Waiting for sobriety is right for the formal consent, but turning the client away entirely risks losing a motivated person. Brief support and a set return time keep engagement while protecting consent.

Takeaway: Postpone formal consent when intoxication impairs understanding, but keep a motivated client engaged until consent can be valid.

Sources: NASW Code of Ethics (2021), 1.03(a)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.4 Informed consent: principles and how it is obtained · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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102. ETH-1053 (rev 1) · Best answer: C

A social worker is leaving a community agency in 3 weeks to open a private practice. Several clients will move to colleagues at the agency, and two have said they may follow the social worker later. What is the **BEST** way for the social worker to handle the client records before leaving?

A. Take a list of client contact details for future follow-up
B. Leave the records at the agency without adding notes
C. Leave the records with the agency and write transfer summaries

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Removing client contact information takes agency records without authorization and invites soliciting clients. Clients who want to follow can contact the social worker themselves or authorize the agency to release their records.

B **Incorrect** (trap: Good but not best). Leaving the records in place meets the custody duty, but without transfer summaries the colleagues inherit cases without a clear picture of goals and progress, which undermines continuity (3.04(b)).

C **Correct.** Agency records stay with the agency, stored securely with access limited to authorized people (NASW Code 1.07(l)). Documentation should also be sufficient to support continuity of future services (3.04(b)), and transfer summaries support an orderly transfer of responsibility (1.16(b)). C beats B because simply leaving the records meets the custody duty but not the continuity duty. Clients who later choose to follow can ask the agency, in writing, to send their records to the new provider.

Takeaway: When leaving an agency, records stay with the agency; add transfer summaries so care continues smoothly.

Sources: NASW Code of Ethics (2021), 1.07(l); NASW Code of Ethics (2021), 3.04(b); NASW Code of Ethics (2021), 1.16(b)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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103. INT-1057 (rev 1) · Best answer: C

At intake for couples therapy, one partner says they have nearly decided to divorce and came only because the other partner pleaded. The other partner wants to save the marriage and asks to start work on communication right away. Both agree to a few sessions. Which approach is **BEST**?

A. Begin couples therapy focused on communication skills
B. Refer the partner leaning toward divorce for individual therapy
C. Help both partners reach clarity about their next step

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Communication training is the right work once both partners commit to improving the relationship. Here one partner has nearly decided to leave, so skills work would rest on a goal only one partner shares and is likely to stall.

B **Incorrect** (trap: Hands off too soon). Sending the leaning-out partner elsewhere hands off the couple's central question instead of addressing it. Both partners agreed to a few sessions together, so the work can begin with the couple.

C **Correct.** Couples therapy needs both partners willing to work on the relationship. When one partner leans toward divorce and the other toward repair, this mixed agenda tends to stall standard couples work. Discernment counseling responds by helping the couple reach clarity and confidence about their next step, using individual conversations with each partner, before any decision to begin couples therapy (Doherty et al., 2016). It beats A because the hopeful partner's request is understandable, but skills training assumes a shared commitment that the facts show is missing.

Takeaway: When one partner leans toward divorce and the other toward repair, clarify the relationship's direction before beginning couples therapy.

Sources: Doherty, W. J., Harris, S. M., & Wilde, J. L. (2016). Discernment counseling for "mixed-agenda" couples. Journal of Marital and Family Therapy, 42(2), 246-255, whole article
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.27 Couples treatment approaches · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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104. ASM-1105 (rev 2) · Best answer: B

At a community health clinic, a client who works at a nail salon seeks care for back pain. Privately, the client says the owner keeps the client's passport, deducts a growing 'travel debt' from each paycheck, and warns that complaining will bring immigration agents. The client wants to keep working for now but asks what help exists. What is the social worker's **BEST** response?

A. Report the salon owner's conduct to local police today
B. With consent, link the client to anti-trafficking services
C. Explain how the client can apply for a T visa

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Law enforcement may become involved later, and calling is right if there is imminent danger or the client chooses it. Reporting today overrides an adult's stated wishes and may deepen the client's fear of deportation.

B **Correct.** A held passport, a debt that keeps growing, and threats about immigration enforcement are markers of labor trafficking; federal law names debt bondage as a form of labor trafficking and counts threatened abuse of the legal process as coercion (22 U.S.C. 7102). The client is an adult who wants to keep working for now but asked about help, so a consent-based link to specialized anti-trafficking services respects self-determination (NASW Code 1.02) and opens legal, safety and immigration options. It beats A because a police report against the client's wishes can feed the very fear the owner exploits.

C **Incorrect** (trap: Outside the social work role). Immigration relief for trafficking survivors exists, but advising on eligibility and filing is legal work for an immigration attorney or accredited representative. The social worker's role is to connect the client to that expertise.

Takeaway: Adult trafficking indicators call for a consent-based link to specialized services, not overriding the client or giving legal advice.

Sources: Trafficking Victims Protection Act, 22 U.S.C. 7102(3), (7), (11); NASW Code of Ethics (2021), 1.02
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.15 Exploitation across the lifespan: financial, immigration-related, trafficking · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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105. ETH-1720 (rev 2) · Best answer: C

A 70-year-old client with macular degeneration receives telehealth counseling from an agency. The client uses a screen reader but cannot read the agency's scanned consent forms and treatment plans, so an adult child reads them aloud. The client says they want to review these papers privately and at their own pace, and that Word documents work well with the screen reader. What is the **MOST** appropriate response?

A. Read each document aloud to the client during sessions
B. Email the scanned forms so the client can enlarge them
C. Send the documents as accessible electronic files

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Reading documents aloud is a reasonable short-term step and keeps family out of private matters. It still ties review to session time and the worker's pace, when the client asked to read independently.

B **Incorrect** (trap: Misuses the technique). Emailing seems to give private, self-paced access, but scanned pages are images that a screen reader cannot read, and enlarging them does not help a client who relies on audio output. The format, not the delivery route, is the barrier the client named.

C **Correct.** Auxiliary aids must be effective, timely and provided in a way that protects the privacy and independence of the person with a disability, and the provider should consult the person about what works (28 CFR 36.303(c)(1)(ii)). The client named both the goal, private review at their own pace, and a format that works. Sending accessible electronic files meets both. It beats A because reading aloud in session keeps family out but still prevents the client from reviewing the papers independently and on their own schedule.

Takeaway: Provide documents in the accessible format the client names, protecting privacy and independence, rather than relying on others to read for them.

Sources: ADA Title III regulations, 28 CFR 36.303(c)(1)(ii)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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106. INT-1579 (rev 1) · Best answer: B

A community clinic begins serving many adults who recently resettled from Afghanistan. Staff want to track depression with a scale developed and normed with English-speaking U.S. adults, and a staff member has translated it into Dari. Early scores look low, even for clients who describe poor sleep, hopelessness and withdrawal. What is the **MOST** appropriate response?

A. Compare the clients' scores with the scale's published norms
B. Seek a version with validity evidence for this population
C. Have a second translator check the Dari wording
D. Stop using standardized measures with this group

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Published norms describe the group the scale was built with. Comparing these clients with English-speaking U.S. norms assumes the scale works the same way across language and culture, which has not been shown.

B **Correct.** Validity belongs to the interpretation of scores for a particular population and language, so a translated scale needs evidence that it measures the same construct in the new group (AERA, APA & NCME, 2014). Low scores alongside clear signs of distress suggest the tool may miss how depression is expressed in this group, a known cultural issue (DSM-5-TR). This beats C because an accurate translation preserves the words but not necessarily their meaning, the relevance of items, or the norms.

C **Incorrect** (trap: Misuses the technique). Checking translation accuracy is a useful step. It confirms the wording, but it does not show that the items capture how distress is expressed in this community or that the scores mean the same thing.

D **Incorrect** (trap: Good but not best). Dropping measurement gives up useful tracking. A measure with validity evidence for this group, used alongside clinical interviews, serves clients better than no measure at all.

Takeaway: A scale's validity is population-specific; before relying on scores from a new cultural or language group, seek evidence it works for them.

Sources: AERA, APA & NCME (2014). Standards for Educational and Psychological Testing. American Educational Research Association, Ch. 3: Fairness in Testing; DSM-5-TR (APA, 2022), Culture and Psychiatric Diagnosis: Cultural Concepts of Distress
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.5 Research design, data collection/analysis, reliability, validity · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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107. ETH-1024 (rev 2) · Best answer: C

A parent brings an 8-year-old to an outpatient clinic for anxiety treatment. The parent says the child's other parent lives across town after a recent divorce and 'doesn't need to know about this.' The parent is unsure what the custody order says about medical decisions. What should the social worker do **FIRST**?

A. Begin treatment with the consent of the parent who is present
B. Call the other parent to ask for permission to treat
C. Review the custody order to confirm who may consent
D. Require both parents' signatures before any sessions

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). Starting treatment assumes the present parent has authority to consent alone. That may be true, but it is unknown until the custody order is checked; treatment can begin once authority is confirmed.

B **Incorrect** (trap: Right action, wrong time). The other parent may need to consent, depending on the custody order. Calling before checking is premature and may disclose the child's treatment unnecessarily.

C **Correct.** Treating a child requires consent from a person with legal authority to make the child's health care decisions (NASW Code 1.03(c); 45 CFR 164.502(g)(3)). After a divorce, that authority depends on the custody order, which may give decision-making to one parent or require both. The social worker first reviews the order. C beats B because contacting the other parent may be needed, but only after the order shows whether that parent's consent is required.

D **Incorrect** (trap: Good but not best). Requiring both signatures may be more than the law requires if one parent holds sole decision-making authority, and it can delay care. The custody order shows what consent is actually needed.

Takeaway: Before treating a child of separated parents, check the custody order to confirm who has authority to consent.

Sources: NASW Code of Ethics (2021), 1.03(c); HIPAA Privacy Rule, 45 CFR 164.502(g)(3)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.4 Informed consent: principles and how it is obtained · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 3 of 3

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108. INT-1181 (rev 1) · Best answer: C

In Landreth's ACT model for setting limits in child-centered play therapy, what does the social worker do **FIRST**?

A. State the limit on the behavior
B. Offer an acceptable alternative
C. Acknowledge the feeling behind the behavior
D. Name the consequence if it continues

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Communicating the limit is the second step. Leading with it treats the behavior as the whole story and often escalates a child whose feelings have not yet been recognized.

B **Incorrect** (trap: Right action, wrong time). Targeting an alternative is the third step, giving the child an acceptable way to express the feeling. Offered first, it can sound like a distraction rather than a response to what the child feels.

C **Correct.** Landreth's ACT sequence begins by acknowledging the child's feeling or wish ('You're really mad at me'), then communicating the limit ('but I'm not for hitting'), then targeting an acceptable alternative ('you can hit the bop bag'). Starting with the feeling tells the child the emotion is accepted even though the act is not (Landreth, 2012). It beats A because stating the limit first is the most common error; it skips the empathy that helps the child accept the limit.

D **Incorrect** (trap: Wrong model or concept). Consequences enter only if a child keeps breaking a limit after the ACT steps, usually framed as a choice the child can make. Naming one first makes the response punitive and skips the model's three steps.

Takeaway: Limit setting with children: acknowledge the feeling, communicate the limit, then target an acceptable alternative.

Sources: Landreth, G. L. (2012). Play Therapy: The Art of the Relationship (3rd ed.). Routledge, Therapeutic limit setting: the ACT model

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.4 Setting limits and boundaries with clients · **Qualifier** FIRST · **Cognitive** recall · **Difficulty** 1 of 3

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109. ASM-1089 (rev 1) · Best answer: A

A 66-year-old hospital patient with advanced kidney disease tells the social worker, 'I've turned into nothing but a cost to my kids. They'd manage fine without me.' Which construct from the interpersonal theory of suicide does this statement **MOST** likely reflect?

A. Perceived burdensomeness
B. Thwarted belongingness
C. Acquired capability

Why each option is right or wrong

A **Correct.** Joiner's interpersonal theory holds that the desire for suicide grows from perceived burdensomeness, the belief that one's existence costs others more than it gives, together with thwarted belongingness; acquired capability explains who can act on that desire (Van Orden et al., 2010). Calling oneself a cost the family would be better without is the core burdensomeness belief. It beats B because thwarted belongingness centers on feeling alone or disconnected, not on being a drain on others.

B **Incorrect** (trap: Wrong model or concept). Thwarted belongingness is the painful sense of not belonging or of being cut off from others. The patient's statement is about costing the family, not about feeling alone.

C **Incorrect** (trap: Wrong model or concept). Acquired capability is a reduced fear of death and higher pain tolerance built through painful experiences. It concerns the ability to act on suicidal desire, not the belief expressed here.

Takeaway: Statements that others would be better off without oneself reflect perceived burdensomeness, a core driver of suicidal desire.

Sources: Van Orden, K. A., Witte, T. K., Cukrowicz, K. C., Braithwaite, S. R., Selby, E. A., & Joiner, T. E. (2010). The interpersonal theory of suicide. Psychological Review, 117(2), 575-600, Perceived burdensomeness; thwarted belongingness; acquired capability

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.1 Assessing risk of harm to self or others · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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110. ETH-1508 (rev 2) · Best answer: A

A social worker in private practice often refers clients with eating disorders to residential care. A new residential program's marketing director offers the social worker $500 for each referred client who enrolls. The social worker would provide no services to those clients after the referral. What should the social worker do **FIRST**?

A. Decline the payment and base choices on clinical fit

B. Ask the program to donate the fees to a client scholarship fund

C. Ask for the program's outcome data before referring

Why each option is right or wrong

A **Correct.** Social workers do not give or receive payment for a referral when they provide no professional service to the referred client (NASW Code 1.16(c)). A per-enrollment fee creates a financial incentive that could bend referral decisions away from clients' needs, a conflict of interest (NASW Code 1.06(a)). Declining the offer now, and referring only when the program fits a client clinically, settles the issue. It beats B because sending the fees to a scholarship fund removes personal gain but keeps a per-enrollment payment that can still bend referral decisions.

B **Incorrect** (trap: Misreads law or ethics). Redirecting the money avoids personal gain, which makes this tempting. The payment would still be tied to each client who enrolls, so the pull toward this program remains, and the Code's bar on referral payments when no service is provided (1.16(c)) is not avoided by changing where the money goes.

C **Incorrect** (trap: Right action, wrong time). Checking a program's outcomes is good practice before referring anyone, and the social worker may do it later. It does not

address the fee, which is improper whatever the data show, so the offer should be declined first.

Takeaway: Refuse payment for referrals when no service is provided; base every referral on clinical fit.

Sources: NASW Code of Ethics (2021), 1.16(c); NASW Code of Ethics (2021), 1.06(a)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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111. INT-1010 (rev 1) · Best answer: A

In EMDR therapy, the social worker explains what to expect and teaches a calm-place exercise. This happens before any target memory is chosen. Which phase is this?

A. Preparation
B. Assessment
C. Installation
D. Closure

Why each option is right or wrong

A **Correct.** EMDR has eight phases: history taking, preparation, assessment, desensitization, installation, body scan, closure, and reevaluation (Shapiro, 2018). Preparation builds the alliance, explains what to expect, and equips the client with calming resources, such as a calm-place exercise, before any reprocessing begins. It beats D because closure also uses calming skills, but it comes at the end of a session in which reprocessing has occurred, to return the client to balance.

B **Incorrect** (trap: Wrong model or concept). Assessment, phase 3, comes after preparation. The client selects a target memory and rates its image, negative belief, preferred belief, and level of distress, setting up reprocessing.

C **Incorrect** (trap: Wrong model or concept). Installation, phase 5, follows desensitization. It strengthens the client's chosen positive belief about themselves once distress about the memory has dropped.

D **Incorrect** (trap: Wrong model or concept). Closure, phase 7, ends each reprocessing session by returning the client to calm, often with the same skills. The question describes groundwork laid before any reprocessing, which is preparation.

Takeaway: EMDR preparation (phase 2) explains the process and builds calming resources before any trauma memory is reprocessed.

Sources: Shapiro, F. (2018). Eye Movement Desensitization and Reprocessing (EMDR) Therapy: Basic Principles, Protocols, and Procedures (3rd ed.). Guilford Press, the eight phases
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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112. INT-1037 (rev 2) · Best answer: C

In an outpatient substance use group, a member who is 3 weeks sober dreads a coworker's retirement party where alcohol will be served. When the social worker offers to demonstrate refusing a drink, the member says, 'Easy for you, you've never been in my shoes.' Another member, sober for 2 years, has handled many similar work events. Which approach to teaching drink refusal is **MOST** effective?

A. Have the social worker demonstrate the refusal anyway
B. Give the member a list of refusal phrases to rehearse
C. Invite the member in longer recovery to demonstrate it

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). A demonstration by the leader is a common, useful way to model refusal skills. Here the member has already discounted the worker as someone who has never faced this, which weakens the model's influence.

B **Incorrect** (trap: Good but not best). Refusal phrases supply useful words, but a list skips seeing the skill performed, including tone and body language. Modeling followed by rehearsal builds skill and confidence more than written phrases alone.

C **Correct.** People learn more readily from a model they see as similar to themselves, especially when they doubt their own ability (Bandura, 1977; Schunk & Hanson, 1989). The newer member has rejected the worker as unlike them, while a peer who has faced the same work events offers a credible, similar model. A peer demonstration also draws on imitative behavior, one of the therapeutic factors of groups (Yalom & Leszcz, 2020). It beats A because the worker's demonstration may be technically sound, but the member has just said it won't feel relevant.

Takeaway: Choose models the client sees as similar and credible; in groups, a peer who has coped with the same situation can model best.

Sources: Bandura, A. (1977). Social Learning Theory. Prentice-Hall, observational learning: attention, retention, reproduction, motivation; Schunk, D. H., & Hanson, A. R. (1989). Influence of peer-model attributes on children's beliefs and learning. Journal of Educational Psychology, 81(3), 431-434, coping versus mastery models; Yalom & Leszcz (2020). The Theory and Practice of Group Psychotherapy (6th ed.). Basic Books, therapeutic factors: imitative behavior

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.26 Using role modeling to teach behavior · **Qualifier** MOST · **Cognitive** reasoning · **Difficulty** 3 of 3

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113. INT-1684 (rev 2) · Best answer: C

To cut costs, a nonprofit's director proposes ending individual supervision for 4 associate social workers and replacing it with a monthly peer consultation group made up of the associates. The associates carry caseloads with frequent suicide risk. The director asks the clinical manager for a recommendation. What is the **MOST** appropriate recommendation?

A. Adopt the peer group, since associates can advise one another
B. Adopt the peer group with the manager reviewing notes monthly
C. Keep designated supervisors and add the peer group as a supplement
D. Keep individual supervision but reduce it to monthly sessions

Why each option is right or wrong

- **A** **Incorrect** (trap: Wrong model or concept). Peer groups offer support and fresh ideas, but no member has authority over another's work or shares accountability for client outcomes. Treating peer consultation as supervision leaves associates without anyone who can direct their practice.
- **B** **Incorrect** (trap: Good but not best). Note review adds some oversight, but a monthly look at records is not supervision. It lacks the regular relationship, skill development and timely guidance on risk that caseloads with frequent suicide risk need.
- **C** **Correct.** Supervision is an ongoing relationship in which the supervisor holds authority and shares responsibility for the supervisee's practice; peer consultation is advisory, and no member carries that accountability (NASW & ASWB, 2013; Kadushin & Harkness, 2014). Associates managing frequent suicide risk need someone who can direct their work and act on safety concerns. Keeping designated supervision while adding the peer group preserves accountability and adds low-cost peer learning. It beats B because occasional note review is oversight in name, without the regular contact supervision provides.
- **D** **Incorrect** (trap: Good but not best). Monthly supervision keeps a designated supervisor and saves money, so it sounds like a fair compromise. With frequent suicide risk on the associates' caseloads, though, a month between contacts leaves risk questions waiting; keeping regular supervision and adding the peer group as a supplement meets both aims.

Takeaway: Peer consultation can supplement supervision but cannot replace it, because no peer holds supervisory authority or shared accountability.

Sources: NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 3 (Legal and Regulatory Issues): supervisors share responsibility for services to clients; Kadushin & Harkness (2014). Supervision in Social Work (5th ed.). Columbia University Press, Ch. 9: Group Supervision
Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.2 Supervision and consultation models: individual, peer, group · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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114. ASM-1182 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

At a community health center, a 24-year-old client tells the social worker that their partner has flushed the client's birth control pills twice and says a baby would 'keep us together.' In answer to safety questions, the client describes no physical violence. The client does not want to become pregnant now and does not want to end the relationship. What is the **MOST** appropriate response?

- **A.** Encourage the client to leave the relationship for their safety
- **B.** Connect the client with the provider about less detectable methods
- **C.** Suggest couples counseling so both partners can discuss family plans

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Leaving is the client's decision, and the client has said they want to stay. Pressing a person to leave a controlling partner can damage trust, and the period around separation can bring added risk. Harm reduction respects the client's choice.

B **Correct.** Sabotaging a partner's contraception to cause pregnancy is reproductive coercion, a form of intimate partner violence. ACOG (2013, No. 554) recommends harm-reduction counseling and offering long-acting methods that are less detectable to partners. The client's goals are to avoid pregnancy and keep the relationship, so linking the client with the provider for a discreet method protects those goals. It beats A because pressing a client to leave overrides the stated choice and does not prevent pregnancy now.

C **Incorrect** (trap: Ignores risk). Couples counseling treats coercion as a communication problem and can expose the client to retaliation for disclosing. Couples work is generally avoided when one partner is controlling the other's reproductive choices.

Takeaway: Reproductive coercion is partner abuse; respond with harm reduction, such as discreet contraception, while respecting the client's relationship choices.

Sources: American College of Obstetricians and Gynecologists (2013). Reproductive and Sexual Coercion. Committee Opinion No. 554, Recommendations

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.4 Physical, sexual, psychological abuse and neglect: signs, dynamics, impact · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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115. INT-1681 (rev 2) · Best answer: C

An accreditation survey of an outpatient behavioral health clinic results in three cited deficiencies. The director asks the quality manager to draft a plan of correction and to decide which deficiency to correct before the others. Which finding should the plan address **FIRST**?

A. Client signatures missing on 40% of treatment plans

B. Consent forms omitting current confidentiality limits

C. Six positive suicide screens lacking reassessment

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Unsigned plans are the most frequent finding and need correction, since client participation in planning matters. Frequency alone does not set priority, though; a safety gap affecting fewer clients comes first.

B **Incorrect** (trap: Good but not best). Consent forms that omit current confidentiality limits affect every new client's informed consent, and the fix is quick, so it should follow soon. The gap is real but carries less immediate danger than six clients at known suicide risk going unreassessed.

C **Correct.** Corrective action after an external review is prioritized by potential for harm, not by how often a deficiency appears (Reamer, 2015). Six clients who screened positive for suicide risk have no documented reassessment, so the clinic must first reach and reassess them and then fix the process that let the gap occur; accreditors treat suicide risk reduction as a national patient safety goal (The Joint Commission,

NPSG.15.01.01). It beats A because unsigned plans are more widespread but pose a documentation and engagement problem rather than a threat to life.

Takeaway: Address external review findings in order of potential client harm; a safety gap outranks more frequent documentation deficiencies.

Sources: Reamer, F. G. (2015). Risk Management in Social Work: Preventing Professional Malpractice, Liability, and Disciplinary Action. Columbia University Press, Ch. 1: Professional Risk Management: An Overview; The Joint Commission (2018). R3 Report Issue 18: National Patient Safety Goal for Suicide Prevention, NPSG.15.01.01 (hospitals and behavioral health care organizations)

Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.6 Quality assurance: external program reviews and audits · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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116. ASM-1691 (rev 1) · Best answer: B

After 10 weeks of in-home family services, a parent has met the plan's goals during visits by using a reward chart and calm time-outs with a 6-year-old. The parent's daily log shows the chart is used only on visit days, and the parent calls the social worker during most conflicts between visits. The supervisor suggests closing the case. What is the **BEST** response?

A. Close services, since the plan's goals were met during visits

B. Continue services while fading support to build independent use

C. Refer the parent to a community parenting class and close the case

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). The goals were met in the worker's presence, but readiness means the parent can use the skills alone. Closure is the right step once the log shows steady use between visits.

B **Correct.** A central sign of readiness to end is that the client uses new skills without the worker and keeps gains between contacts (Hepworth et al.). The log shows the chart is used only when the worker visits, and the parent still relies on calls to manage conflicts, so the gains are not yet the parent's own. Continuing while deliberately fading support tests and builds independent use. It beats A because closing now would end services for a family still in need of them, which the Code asks workers to avoid (NASW Code 1.17(b)).

C **Incorrect** (trap: Hands off too soon). A parenting class may help later, but it hands off a family whose gains still depend on the current worker. Closing now with a class referral risks losing progress before it becomes independent.

Takeaway: Close a case once skills appear without the worker present; skills seen only during visits call for fading support first.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Termination: assessing readiness to end; NASW Code of Ethics (2021), 1.17(b)

Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.6 Signs a client is ready for termination · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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117. ETH-1526 (rev 1) · Best answer: A

During a home visit, a hospice patient with advanced lung disease tells the social worker, 'I've been saving my pain pills. I'm going to take them all tonight so my family can stop taking care of me.' The patient shows the social worker a full bottle hidden in a drawer. What should the social worker do **FIRST**?

A. Stay with the patient and get the care team's help now
B. Explore the patient's feeling of being a burden to the family
C. Explain how legal aid in dying differs from a private overdose

Why each option is right or wrong

A **Correct.** Self-determination and confidentiality yield when a client's plan poses a serious, foreseeable and imminent risk of harm (NASW Code 1.02, 1.07(c)). This patient has a plan for tonight, the means in hand and a reason rooted in feeling like a burden, which is different from a lawful end-of-life choice. Staying with the patient and bringing in the hospice team lets the team secure the medication and assess the patient now. It beats B because the sense of burden must be explored, but only after the immediate danger is contained.

B **Incorrect** (trap: Right action, wrong time). Feeling like a burden is a key driver to explore, and the team should address it soon. With a plan for tonight and pills within reach, though, talking first leaves the means available. It belongs after the safety steps.

C **Incorrect** (trap: Ignores risk). Information about lawful options may matter later if the patient raises them in a calmer moment. Right now the patient has a hidden overdose plan for tonight, so a discussion of legal processes ignores the immediate danger.

Takeaway: A terminally ill client with a plan, means and a timeline needs immediate protective action; terminal illness does not lower the response to suicide risk.

Sources: NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.07(c)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.6 Law and ethics around death and dying · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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118. ETH-1603 (rev 1) · Best answer: C

At a family shelter, a client who fled political violence and arrived in the country 10 months ago tells the social worker, 'A friend said there is a time limit to ask for protection here. Should I apply for asylum?' The client has not met with any lawyer. What should the social worker do **FIRST**?

A. Explore the violence the client faced before leaving
B. Help the client write a statement for the asylum form
C. Arrange a prompt meeting with an immigration legal provider

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Understanding the client's experiences will matter for support and later for any legal claim. Opening a detailed trauma history now delays a time-sensitive legal step and may push the client to retell painful events before a lawyer can guide what is needed.

B **Incorrect** (trap: Outside the social work role). Drafting the asylum statement is legal work that shapes a claim and its credibility. Social workers may later support a client

through the process, but preparing filings belongs with an immigration attorney or accredited representative.

C **Correct.** Whether and how to seek asylum is a legal question with strict filing rules, including a general one-year deadline after arrival (8 U.S.C. 1158(a)(2)(B)). Social workers do not give legal advice; they refer when another professional's specialized knowledge is needed (NASW Code 1.16(a)). With about 2 months left and no lawyer involved, a prompt link to an immigration attorney or accredited representative protects the client's options. It beats A because trauma-informed support matters but can continue alongside legal help and should not delay a time-limited decision.

Takeaway: Time-sensitive immigration questions go promptly to an immigration attorney or accredited representative; social workers support but do not give legal advice.

Sources: Immigration and Nationality Act, 8 U.S.C. 1158(a)(2)(B); NASW Code of Ethics (2021), 1.16(a)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.9 Globalization, immigration, refugee status effects on clients and services · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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119. ETH-1691 (rev 1) · Best answer: C

A Black engineer in CBT for depression is the only Black member of a work team. The client has documented being passed over for two promotions and hearing a manager use a racial slur. The client's thought record reads: 'They'll never promote someone like me, so I'm worthless.' How should the social worker **BEST** approach this thought?

A. Challenge the promotion prediction as a fortune-telling distortion
B. Shift the sessions toward filing a formal discrimination complaint
C. Accept the bias as real and examine the self-worth conclusion
D. Set the thought record aside until the workplace problem resolves

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Fortune-telling is a real cognitive distortion, but this prediction rests on documented events. Treating it as faulty thinking invalidates the client's experience of racism and can rupture the alliance. Challenging predictions fits when the evidence does not support them.

B **Incorrect** (trap: Overrides the client's choice). Supporting a complaint may become part of the work if the client wants it, and the worker can raise the option. Steering sessions there now sets the depression aside and makes the decision for the client.

C **Correct.** Culturally responsive CBT separates accurate perceptions of oppression from the unhelpful conclusions a client draws from them, validating the first and working on the second (Hays, 2009). The promotions and the slur are documented, so the first half of the thought reflects real bias; the self-judgment is where cognitive work helps. This keeps the problem located in workplace racism, not the client's thinking (NASW Code 1.05(b)). It beats A because labeling a well-founded expectation a distortion tells the client that noticing racism is the problem.

D **Incorrect** (trap: Good but not best). Waiting for the workplace to change leaves the self-blame untreated, even though it can be worked on now. The client's depression needs attention whether or not the bias stops.

Takeaway: In CBT with clients facing discrimination, validate accurate perceptions of bias and target the self-blaming conclusions drawn from them.

Sources: Hays, P. A. (2009). Integrating evidence-based practice, cognitive-behavior therapy, and multicultural therapy: Ten steps for culturally competent practice. Professional Psychology: Research and Practice, 40(4), 354-360, whole article; NASW Code of Ethics (2021), 1.05(b)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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120. ASM-1528 (rev 1) · Best answer: D

The spouse of a nursing home resident with advanced major neurocognitive disorder visits every day. The resident no longer recognizes the spouse. The spouse tells the social worker, 'I don't know whether to think of myself as married anymore.' The spouse has put off decisions about selling their house and feels guilty about accepting a friend's dinner invitation. Which concept **MOST** likely describes the spouse's experience?

- **A.** Anticipatory grief
- **B.** Disenfranchised grief
- **C.** Caregiver burnout
- **D.** Ambiguous loss

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Anticipatory grief is mourning before an expected death, and dementia caregivers often feel it. It fits part of this picture, but the spouse's central struggle is not knowing whether they are still married, a sign of a loss that has no clear ending.

B **Incorrect** (trap: Wrong model or concept). Disenfranchised grief is grief that others do not recognize or support, such as grief for an ex-partner. Nothing here shows the spouse's loss being dismissed by others; the difficulty comes from the nature of the loss itself.

C **Incorrect** (trap: Wrong model or concept). Burnout is exhaustion from prolonged caregiving demands. This spouse visits daily but describes confusion about their marital status and frozen decisions, not depletion from care tasks, which nursing home staff now provide.

D **Correct.** Ambiguous loss is a loss without clarity or closure. In one type, a person is physically present but psychologically absent, as in advanced dementia (Boss, 1999). The resident is alive but no longer knows the spouse, so the spouse cannot tell whether the marriage continues, puts off decisions about the house and feels guilty about living a normal life. It beats A because anticipatory grief is mourning a death that is expected; this spouse's distress centers on the unclear status of a person who is still here.

Takeaway: Ambiguous loss occurs when a loved one is physically present but psychologically gone, as in dementia, leaving no clear ending to grieve.

Sources: Boss, P. (1999). Ambiguous Loss: Learning to Live with Unresolved Grief. Harvard University Press, loss when a person is physically present but psychologically absent
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.14 How caregiving affects families · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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121. ETH-1712 (rev 2) · Best answer: A

At a community health center, an undocumented parent says a neighbor was just detained by immigration agents. The parent's children, ages 6 and 9, are U.S. citizens. After spending most of the session on the arrest, the sleepless nights since and a call that morning to the family's immigration attorney, the parent asks, 'If they take me, who will pick up my kids?' What should the social worker do **NEXT**?

A. Help the parent make a written plan naming a caregiver
B. Invite the parent to say more about the fear of detention
C. Reassure the parent that parents of citizen children are rarely detained

Why each option is right or wrong

A **Correct.** Immigration enforcement fear weighs on children in mixed-status families whether or not a parent is detained (Dreby, 2012), and a concrete plan restores some control. The parent has already voiced the fear and has legal help for the immigration case; the question now is practical. Helping the parent name a trusted caregiver, arrange school pickup permission and gather key documents answers the request, while the attorney advises on any legal forms. This supports the parent's own goal (NASW Code 1.02). It beats B because exploring a fear the parent has already described delays the planning the parent asked for.

B **Incorrect** (trap: Right action, wrong time). Exploring fear is a good opening when a client first raises enforcement worries. This parent has spent most of the session on the fear and now asks a specific practical question, so more exploration repeats ground already covered.

C **Incorrect** (trap: Ignores risk). No social worker can predict enforcement actions, and parents of citizen children are detained. False reassurance may leave the children without a plan and undermines the parent's trust.

Takeaway: When mixed-status families fear enforcement, help parents make concrete child-care plans and leave legal questions to their attorney.

Sources: Dreby, J. (2012). The burden of deportation on children in Mexican immigrant families. Journal of Marriage and Family, 74(4), 829-845, whole article; NASW Code of Ethics (2021), 1.02
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.9 Globalization, immigration, refugee status effects on clients and services · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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122. ASM-0004 (rev 1) · Best answer: D

A parent tells the social worker that their 20-month-old, who used to explore the playground confidently, now clings to the parent and protests at separations, then pushes the parent away moments later. The child is otherwise healthy, and there have been no recent changes at home. What **MOST** likely explains the behavior?

A. Separation anxiety disorder
B. Insecure-avoidant attachment
C. Regression after a stressful event
D. Rapprochement subphase

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Distress at separation is developmentally expected in toddlers. The disorder requires fear beyond what is typical for the child's developmental level, with impairment, which the stem does not describe (DSM-5-TR).

B **Incorrect** (trap: Wrong model or concept). Avoidant attachment shows up as a child who seems indifferent to the caregiver and does not seek comfort. This toddler actively seeks the parent, which is the opposite pattern.

C **Incorrect** (trap: Skips assessment). Regression is a reasonable idea when a stressor is present, but the stem rules out recent changes at home. Choosing it adds facts that the stem does not give.

D **Correct.** In Mahler's separation-individuation theory, the rapprochement subphase (about 16 to 24 months) follows the confident practicing period. The toddler, newly aware of being separate, alternates between clinging and pushing away (Mahler, Pine & Bergman, 1975). The age, the earlier confident exploring and the cling-then-reject pattern all fit. It beats A because separation anxiety disorder requires fear that is excessive for the child's developmental level, and protest at separation is expected at 20 months.

Takeaway: Clinging that alternates with pushing away at 16 to 24 months is Mahler's rapprochement subphase, a normal phase.

Sources: Mahler, M. S., Pine, F., & Bergman, A. (1975). The Psychological Birth of the Human Infant. Basic Books, rapprochement subphase; DSM-5-TR (APA, 2022), Anxiety Disorders: Separation Anxiety Disorder
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.16 Typical vs. atypical development across the lifespan · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3